椰子翻譯 一鍵生成雙語對照・保留原始排版 升級 Pro / Max，暢享無浮水印 PDF
免費翻譯 PDF→ cocotranslate.com/zh-TW/doc

Constructing Professional Identity: The Role of Work and Identity Learning Cycles in the Customization of Identity among Medical Residents
Author(s): Michael G. Pratt, Kevin W. Rockmann and Jeffrey B. Kaufmann
Source: *The Academy of Management Journal*, Apr., 2006, Vol. 49, No. 2 (Apr., 2006), pp. 235-262
Stable URL: https://www.jstor.org/stable/20159762

Accessibility support:
If you experience accessibility issues with this file, report them here

JSTOR is a not-for-profit service that helps scholars, researchers, and students discover, use, and build upon a wide range of content in a trusted digital archive. We use information technology and tools to increase productivity and .facilitate new forms of scholarship. For more information about JSTOR, please contact support@jstor.org

Your use of the JSTOR archive indicates your acceptance of the Terms & Conditions of Use, available at https://about.jstor.org/terms

建構專業認同：工作與認同學習循環在住院醫師認同調適中的角色 作者：Michael G. Pratt、Kevin W. Rockmann 與 Jeffrey B. Kaufmann 來源：《管理學會期刊》，2006 年 4 月，第 49 卷，第 2 期（2006 年 4 月），第 235–262 頁 穩定網址：https://www.jstor.org/stable/20159762

無障礙支援：如果您在使用此檔案時遇到無障礙問題，請在此回報

JSTOR 是一項非營利服務，協助學者、研究人員與學生在值得信賴的數位典藏中探索、使用並運用各類內容。我們運用資訊科技與工具提升生產力，並促進新型態的學術研究。如需更多 JSTOR 相關資訊，請聯絡 support@jstor.org。您使用 JSTOR 典藏，即表示您接受使用條款，詳見 https://about.jstor.org/terms

JSTOR is working to digitize, preserve and extend access to *The Academy of Management Journal*

JSTOR 致力於將《管理學會期刊》數位化、保存並擴大其存取範圍

This content downloaded from
120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從120.126.194.245 下載
所有使用均須遵守 https://about.jstor.org/terms

Academy of Management Journal
2006, Vol. 49, No. 2, 235–262.

CONSTRUCTING PROFESSIONAL IDENTITY: THE ROLE OF WORK AND IDENTITY LEARNING CYCLES IN THE CUSTOMIZATION OF IDENTITY AMONG MEDICAL RESIDENTS

MICHAEL G. PRATT
University of Illinois at Urbana-Champaign

KEVIN W. ROCKMANN
George Mason University

JEFFREY B. KAUFMANN
Iowa State University

Through a six-year qualitative study of medical residents, we build theory about professional identity construction. We found that identity construction was triggered by work-identity integrity violations: an experienced mismatch between what physicians did and who they were. These violations were resolved through identity customization processes (enriching, patching, or splinting), which were part of interrelated identity and work learning cycles. Implications of our findings (e.g., for member identification) for both theory and practice are discussed.

Q: What does it mean to you to be a surgeon?

A: The reason I like surgery, there's the active nature of it . . . I love being in the operating room. I love doing operations. . . . [Surgery] is not a technical discipline . . . you're thinking about these things all the time . . . then you bring in the art aspect of it as well . . . The last time you talked to me, I hadn't done too many operations. Now I'm doing very complicated operations routinely. Like everyday, that's a major difference, so now instead of talking about "well, you know it's really fun to play chopsticks. . . ." Now it's like talking about the nuances of playing Beethoven piano sonatas.

-Interview with a surgical resident

Despite a growing interest in matters of identity in organizational studies, researchers know relatively little about how identities are formed among those who carry out some highly critical organizational functions: professionals (Ibarra, 1999). Professions, such as medicine, law, and accounting, arise when an organized group possesses esoteric knowledge that has economic value when applied to problems (e.g., sickness) faced by people in a society (Carr-Saunders & Wilson, 1933; MacDonald, 1995). Because of their unique knowledge and skill sets, society grants professionals higher levels of prestige and autonomy than it grants nonprofessionals (Larson, 1977). Given the historical and potentially increasing importance of professionals in all types of organizations (Wallace, 1995), and given the centrality of identity in how individuals make sense of and "enact" their environments (see Weick, 1995), addressing issues of professional identity construction is timely.

Unfortunately, it is not clear whether current understanding of identity formation among organizational members helps to address this issue. To begin, there is a paucity of literature on the topic. As Ibarra noted: "Despite consensus in the socialization literature that identity changes accompany work role changes, the process by which identity evolves remains under explained" (1999: 765). A similar plea for more dynamic and nuanced models of identity construction was made by Pratt, who suggested that identification sometimes involves

We would like to thank Peter Foreman and Nora Madjar for their assistance during earlier portions of this project. We would also like to thank Susan Ashford, Blake Ashforth, Caroline Bartel, Kevin Corley, Jane Dutton, Marlena Fiol, CV Harquail, Greg Oldham, Violina Rindova, Sonali Shah, the participants at the Boston Identity Conference, as well as various scholars from the University of Michigan I.C.O.S. group, the Washington University Olin School of Business, the New York University Stern School of Business, and the University of Illinois Organizational Behavior Group and Psychology Department, for their insights and comments on drafts of this paper. Finally, we would like to thank Tom Lee and the three anonymous reviewers whose comments greatly improved the quality of our arguments.

This content downloaded from
120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

《管理學會期刊》2006 年，第 49 卷，
第 2 期，235–262 頁。

建構專業認同：……的角色
醫療住院醫師身分調整中的工作與認同學習循環

MICHAEL G. PRATT 伊利諾大學厄巴納－
香檳分校

KEVIN W.
ROCKMANN 喬治梅森大學

JEFFREY B. KAUFMANN
荷華州立大學

我們透過一項為期六年的住院醫師質性研究，建構有關專業身分建構的理論。我們發現，身分建構是由工作身分完整性遭到違背所觸發：也就是醫師感受到自己所做的事與自身身分不相符。

這些違背情形透過身分客製化歷程（充實、修補或支撐）獲得化解，而這些歷程是彼此相連的身分學習與工作學習循環的一部分。本文也探討研究發現對理論與實務的啟示（例如對成員認同的影響）。

問：對你而言，成為一名外科醫師代表什麼？

答：我喜歡外科的原因之一，是它積極投入的特性……我喜歡待在手術室，也喜歡執行手術。……[外科] 並非一門技術學科……你時時刻刻都在思考這些事情……此外，還要融入其中的藝術面向。……上次和你談話時，我做的手術還不多。現在我已經能定期執行非常複雜的手術了，幾乎每天都在做那種……

這是很大的差異，因此現在談的不再是「嗯，你知道，玩《筷子》真的很有趣……」，而是談演奏貝多芬鋼琴奏鳴曲的細微之處。

-與一位外科住院醫師的訪談

儘管人們對身分認同議題的興趣日益增加

在組織研究中，研究人員對於某些執行關鍵組織職能的人員——專業人士——如何形成認同，所知仍相當有限（Ibarra, 1999）。當一個有組織的群體掌握了專門知識，而這些知識應用於解決人們面臨的問題（例如疾病）時具有經濟價值，便會形成專業領域，例如醫學、法律和會計（

社會（Carr-Saunders & Wilson, 1933; Mac Donald, 1995）。由於專業人士擁有獨特的知識與技能，社會賦予他們較高

的聲望與自主權，高於非專業人士（Larson, 1977）。鑑於專業人士在各類組織中的重要性深植於歷史且可能日益提升（Wallace, 1995），以及認同在人們理解並「實踐」其環境的過程中居於核心地位（見 Weick, 1995），探討

專業認同　　的建構正逢其時

遺憾的是，目前對組織成員身分形成的理解是否有助於處理這項議題，仍不清楚。首先，相關主題的文獻十分匱乏。正如 Ibarra 所指出：「儘管社會化文獻普遍認同，身分的變化會伴隨工作角色的變化，但身分演變的過程仍缺乏充分解釋」（1999：765）。Pratt 也呼籲建立更具動態性與細膩度的身分建構模型；他指出，認同有時涉及

我們要感謝 Peter Foreman 和 Nora Mad jar 在本研究前期提供協助。我們也要感謝 Susan Ashford、Blake Ashforth、Caroline Bartel、Kevin Corley、lane Dut ton、Marlena Fiol、CV Harquail、Greg Oldham、Violina Rindova、Sonali Shah、波士頓身分認同研討會的與會者，以及密西根大學 I.C.O.S. 團隊的多位學者，

奧林商學院、紐約大學

史登商學院，以及伊利諾大學組織行為研究團隊和心理學系，感謝他們對本文草稿提出見解與意見。最後，我們要感謝 Tom Lee 和

三位匿名審查者，他們的意見大幅提升了我們論述的品質。

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從 120.126.194.245 下
載 所有使用均受 https://about.jstor.org/terms 條款約束

identity change, but that of "all of the central questions of organizational identification, the one that has probably received the least attention by organizational scholars has been, 'How does organizational identification occur?'" (1998: 192). Sveningsson and Alvesson echoed this sentiment a half decade later, writing: "Despite the espoused interest in the issue of becoming identified, most authors do not go very far in that direction" (2003: 1164).

In addition, even if there were a substantive body of literature on identity construction of organizational *members,* it is not clear whether such insights could be transferred to the identity construction of *professionals* (Van Maanen & Barley, 1984). To illustrate, because of its roots in social identity theory, most research in organizational identification has examined the importance of organizational membership in employee's self-concepts (e.g., Ashforth & Mael, 1989; Dutton, Dukerich & Harquail, 1994; Pratt, 1998). However, it is not clear whether membership serves a similar role in the self-concepts of professionals (Gouldner, 1957; Wallace, 1995). Organizational membership is an indicator of *where you work* (i.e., an organization). Professionals, by contrast, are often defined by *what they do.* It is not surprising, therefore, that the surgical resident in our opening quote immediately launched into "what he did" when asked to describe what it meant to be a surgeon.

The purpose of this article is to build and enrich theory around how professionals construct their own professional identities. We examined this process in the context of medicine, as it is thought to be a prototypical profession (Hughes, 1956). Specifically, we examined physicians during residency training, which is thought to "contribute to the development of commitment to the occupation as a life career and to a shared identity, a feeling of community or solidarity among all those who have passed through it" (Freidson, 2001: 84). Thus, unlike sociological treatments exploring how professions as an *occupational group* establish legitimacy, status, and boundaries (cf. Abbott, 1988; Larson, 1977), the present treatment focused on *individual* actors.

To accomplish our goal, we performed a six-year, longitudinal, primarily qualitative study that tracked medical residents throughout their entire residency programs. Because our interest was in building and enriching theory, the bulk of our theory lies at the end of the paper. However, to orient the reader toward our eventual findings, we begin by providing a brief theoretical review of identity construction.[1]

IDENTITY CONSTRUCTION: A BRIEF REVIEW

Several theories implicitly or explicitly address the social "fact" that how an individual self-defines him- or herself changes in the context of organizational life. We identified three literatures as especially helpful in understanding the identity construction process, broadly defined: careers and role transitions, socialization, and identity work. Though we ultimately found that each perspective held important pieces of the puzzle of identity construction, none adequately accounted for our findings. This gap was due, in part, to the fact that identity construction is often implied by theories, rather than being their main focus (Ibarra, 1999).

Careers/Role Transitions

Research taking a careers/role transitions perspective suggests that identities may change in organizational settings as individuals' careers progress. For example, Hall (1968, 1971, 1995) argued that throughout their careers, individuals must move or transition into new roles, and that these transitions facilitate changes in what he referred to as "subidentities." In particular, he viewed such transitions as involving alignment or fit between a subidentity, defined as "the aspects of identity which are relevant to particular social roles" (Hall, 1968: 447), and role expectations. In developing this perspective, Hall (1971, 1995) argued for the importance of perceived competence in making the transition but did not elucidate how the content of identity changes in this process.

In another treatment, Nicholson and colleagues (Nicholson, 1984; West, Nicholson, & Arnold, 1987) argued that, depending on the job conditions, four outcomes can occur as employees transition into new work roles: absorption, determination, exploration, and replication. These outcomes mark a change in person (absorption), role (determination), both (exploration), or neither (replication). Although this body of work focuses on *when* identities are likely to change (e.g., when individuals

[1] We should note that some of the theory presented here was unknown to the authors before the start of the study. Our initial interest was in how professionals became socialized into multiple identities. However, several astute reviewers of our paper pointed out that our data actually said more about professional identity construction than multiple identity socialization. We thank these reviewers for this insight.

This content downloaded from
20.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

身分變化，但「在組織認同的所有核心問題中，唯一那個 t

組織研究學者最少關注的問題，可能是：「組織認同是如何形成的？」（1998:192）。Sveningsson 與 Alvesson 半 f

十年後寫道：「儘管人們宣稱對形成認同這項議題感興趣，但大多數作者在這方面並未深入探討」（2003:1164）。此外，即使已有大量關於組織成員身分建構的文獻，也不清楚這些見解是否能轉用於專業人士的身分建構（Van Maanen & Barley, 1984）。舉例而言，由於組織認同研究源自社會認同理論，多數研究都探討組織成員身分對員工自我概念的重要性（例如 Ashforth & Mael, 1989；Dutton, Dukerich & Harquail, 1994；Pratt, 1998）。然而，成員身分是否在專業人士的自我概念中扮演類似角色，尚不明確（Gouldner, 1957；Wallace, 1995）。組織成員身分代表你在哪裡工作（也就是某個組織）。相較之下，專業人士通常是由其所從事的工作來界定。因此，開頭引文中的外科住院醫師在被問及成為外科醫師對他而言意味著什麼時，立即開始談論「他做了什麼」也就不足為奇了

geon.

本文旨在建構並豐富有關專業人士如何形塑自身專業認同的理論。我們以醫學為背景探討這個過程，因為醫學被視為典型的專業（Hughes, 1956）。具體而言，我們研究了接受住院醫師訓練的醫師；一般認為，這段訓練「有助於培養對此職業作為終身志業的投入，以及共同認同感，也就是所有曾經歷這段訓練的人所共享的社群感或團結感」（Freidson, 2001: 84）。因此，有別於探討專業如何以職業群體的身分建立正當性、地位與界線的社會學研究（參見 Abbott, 1988; Larson, 1977），本文聚焦於個別行動者。

為了達成目標，我們進行了一項為期六年的縱貫研究，以質性方法為主，追蹤醫學住院醫師完整的住院醫師訓練歷程。由於我們旨在建構並豐富理論，本文的大部分理論內容安排在文末。不過，為了讓讀者先了解我們最後得出的研究發現

，我們首先

簡要回顧身分建構的理論。1

身分建構：簡要回顧

若干理論以隱含或明確的方式，探討一項社會「事實」：個人在組織生活中的自我定義會如何改變。我們認為，以下三類文獻特別有助於廣泛理解身分建構的過程：職涯與角色轉換、社會化，以及身分工作。雖然我們最後發現，每種觀點都提供了理解身分建構的重要線索，但沒有一種能充分解釋我們的研究發現。造成這項缺口的部分原因是，身分建構往往只是理論隱含的議題，而非理論的主要焦點（Ibarra, 1999）。

職涯／角色轉換

採取職涯／角色轉換觀點的研究指出，隨著個人職涯發展，其身分可能在組織環境中有所改變。例如，Hall（1968、1971、1995）主張，個人在整個職涯歷程中必須轉換或過渡至新的角色，而這些轉換會促使他所稱的「次身分」發生變化。具體而言，他認為，這類轉換涉及次身分與角色期望之間的調適或契合。次身分的定義是「與特定社會角色相關的身分層面」（Hall, 1968: 447）。在發展這項觀點時，Hall（1971、1995）主張，個人對自身能力的認知對於完成轉換至關重要，但並未闡明身分內容如何

y chang其中的 p過程。

在另一種論述中，Nicholson 與同事（Nicholson，1984；West、Nicholson 與 Arnold，1987）主張，員工轉換至新的工作角色時，依據工作條件，可能出現四種結果：吸納、定型、探索與複製。這些結果分別代表個人（吸納）、角色（定型）、兩者（探索）或兩者皆非（複製）發生改變。儘管這一系列研究著重探討身分認同何時可能改變（例如，個人對工作有自主權時）以及可能如何改變（例如，吸納），但對於連結這些前因與結果的過程，也就是身分認同建構，著墨甚少。

1 我們應該指出，本文所呈現的部分理論，在研究開始之前作者群其實並不知悉。我們最初關注的是專業人士如何社會化為多重身分。然而，幾位敏銳的論文審查者指出，我們的資料實際上更能說明專業身分的建構，而非多重身分的社會化。我們感謝這些審查者提供這項洞見。

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從20.126.194.245 下
載 所有使用均受 https://about.jstor.org/terms 的條款規範

have discretion over their jobs) and *how* they might change (e.g., absorption), it says little about the process that bridges these antecedents and outcomes: identity construction.

Socialization

A second literature that either implicitly or explicitly deals with identity change in organizations is research on socialization. Research on socialization tactics (e.g., Van Maanen & Schein, 1979), conversion (e.g., Kanter, 1972; Lofland & Stark, 1965; Snow & Machalek, 1984; Snow & Phillips, 1980), and "identity transforming organizations" (e.g., Greil & Rudy, 1983) have all examined how groups and organizations actively engage in shaping members' identity. These literatures describe different levels of identity change—from the moderate change brought on by organizational socialization to the extreme change induced by identity conversion—but they all focus on techniques that organizations use to break down an individual's existing sense of self (e.g., divestiture or "sensebreaking" tactics) and on techniques that provide meaning (e.g., "sensegiving") and shape identity in an organization's image (see Pratt, 2000). And although sensegiving implies what identity content is desired by the organization (i.e., an adoption of organizational values), little is said about how members actively use identity-related information to construct their own identities.

Similar omissions can be found in the extensive literature on medical socialization. Research on medical socialization tactics (e.g., Shulman, Wilkerson, & Goldman, 1992; Weinholtz, 1991), for example, has tended to focus on what organizations do when they train medical professionals, and less on identity construction by the medical professionals themselves. By contrast, some nontheoretical treatments of medical socialization have depicted physicians' identity change but have not systematically described how such a process unfolds (e.g., Marion, 1989, 1991). There is a notable exception to these trends, however. Central to the work of Hughes and his colleagues (Becker, Geer, Hughes, & Strauss, 1961; Hughes, 1956) is the interplay between learning about one's identity and learning about the work one needs to accomplish. For example, Hughes argued that during medical school, a student "may be expected to get not merely a better notion of the skills required, of the tasks to be performed, of the roles to be played . . . but also to adjust his conception of his own mental, physical, and personal aptitudes, his tastes and distastes" (1956: 24). Thus, as one learns the work of a doctor, one also constructs new self-conceptions.

The work of Hughes and his colleagues resembles that on careers/role transitions in that both bring the issue of identity change to the fore. However, the former's focus was more indirect, focusing on students' understanding of professional culture and values (e.g., "pragmatic idealism") rather than on professional identity construction, per se. Additionally, this work also discusses the importance of identity construction without describing how it occurs. However, one key departure from the careers/roles research is that these scholars balked at viewing identity changes in terms of roles. As Becker and coauthors argued, "Students do not take on a professional role while they are students" (1961: 420).[2]

Identity Work

An emerging set of studies examines identity construction processes more directly. This literature, which we refer to generally as "identity work," focuses on individuals' active construction of identity in social contexts (e.g., Dickie, 2003; Ibarra, 1999; Snow & Anderson, 1987; Sveningsson & Alvesson, 2003; Van Maanen, 1997). In this research, identity construction is viewed as more interactive and more problematic than the relatively straightforward adoption of a role or category. Moreover, although this work focuses on individual agency, it also construes social groups' influence on identity construction to be significant. For example, some research has discussed how role models provide professional identities that one can "try on" to see if they fit (Ibarra, 1999). Other research has focused on the role of stories and rhet-

[2] Like Becker and colleagues (1961), we also had difficulty applying a role perspective to our study. To begin with, application of roles was unwieldy, and it was difficult to differentiate professional role from professional identity. Medical residents are initially interns (first-year residents), and then junior and senior residents. Each of these could be considered a "role." Moreover, residents are also "physicians," which can also be seen as a role—and as a professional identity—that is related to, but is sometimes distinct from, both their roles as interns and residents and their roles and identities as medical specialists (e.g., as "surgeon"). More importantly, we found taking a role perspective did not fit well with our data. The story that emerged about the interplay of work and identity learning cycles appeared more similar to what Becker and colleagues (1961) labeled "pre-role" activities. Although research on role theories, especially role transitions, provided some important cues to understanding our data (e.g., the role of social validation described by Ashforth [2001]), we nonetheless found the role perspective to be a poor fit with our data.

This content downloaded from
120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

對工作有自主權時）以及可能如何改變（例如，吸納），但對於連結這些前因與結果的過程，也就是身分認同建構，著墨甚少。

社會化

第二類隱含或明確探討組織中身分認同變化的文獻，是關於社會化的研究。社會化策略（例如 Van Maanen 與 Schein，1979）、皈依（例如 Kanter，1972；Lofland 與 Stark，1965；Snow 與 Machalek，1984；Snow 與 Phillips，1980），以及「身分轉變型組織」（例如 Greil 與 Rudy，1983）的研究，都探討了群體和組織如何積極塑造成員的身分認同。這些文獻描述了不同程度的身分認同變化——從組織社會化帶來的適度改變，到身分皈依引發的劇烈改變——但它們都聚焦於組織用來瓦解個人既有自我認知的技巧（例如剝離或「瓦解自我認知」策略），以及提供意義（例如「賦予自我認知」）並依照組織形象塑造身分認同的技巧（參見 Pratt，2000）。儘管賦予自我認知的概念暗示了組織所期望的身分認同內容（也就是採納組織價值觀），但鮮少論及成員如何積極運用與身分認同相關的資訊來建構自己的身分認同。

醫學社會化的豐富文獻中也有類似的遺漏。例如，醫學社會化策略的研究（如 Shulman、Wilkerson 與 Goldman，1992；Weinholtz，1991）往往著重於組織如何訓練醫療專業人員，較少探討醫療專業人員自身如何建構身分認同。相較之下，有些非理論性的醫學社會化研究描繪了醫師的身分認同變化，卻未有系統地說明這個過程如何展開（如 Marion，1989、1991）。然而，這些趨勢有一個值得注意的例外。Hughes 及其同事的研究（Becker、Geer、Hughes 與 Strauss，1961；Hughes，1956）核心在於：人們對自身身分認同的認識，與對所需完成工作的認識，兩者如何相互交織。例如，Hughes 指出，在醫學院就讀期間，學生「可能不僅會更了解所需的技能、必須執行的任務、必須扮演的角色……也會調整對自身心智能力、體能、個人特質、喜好與厭惡的看法」（1956：24）。因此，學習醫師的工作時，人們也會建構新的自我認知。

Hughes 及其同事的研究與職涯／角色轉換研究相似，兩者都凸顯了身分認同變化的議題。然而，前者的探討較為間接，著重於學生對專業文化與價值觀（如「務實的理想主義」）的理解，而非專業身分認同的建構本身。此外，這些研究也談到身分認同建構的重要性，卻未說明其發生過程。不過，這些學者與職涯／角色研究的一項關鍵差異，在於他們不認同以角色變化來看待身分認同的改變。Becker 與共同作者指出：「學生在就學期間並未承擔專業角色」（1961：420）。2

身分認同建構

一批新興研究更直接地探討身分建構過程。我們一般將這類文獻稱為「身分工作」，其著重於個人在社會脈絡中主動建構身分的過程（例如 Dickie, 2003；Ibarra, 1999；Snow & Anderson, 1987；Sveningsson & Alvesson, 2003；Van Maanen, 1997）。在這項研究中，身分建構被視為比相對直接地採納某種角色或類別更具互動性，也更為複雜。此外，儘管這類研究著重個人的能動性，也認為社會群體對身分建構的影響相當重要。例如，有些研究探討榜樣如何提供專業身分，讓人可以「試穿」這些身分，以了解它們是否適合自己（Ibarra, 1999）。其他研究則聚焦於故事與修辭在創造自我身分方面所扮演的角色，例如 Alvesson, 1994

2 如同 Becker 及其同事（1961）一樣，我們也難以將角色觀點應用於本研究。首先，角色的應用並不容易，而且很難區分專業角色與專業身分認同。醫療住院醫師起初是實習醫師（第一年住院醫師），接著成為初級住院醫師與資深住院醫師。這些身分都可以被視為一種「角色」。此外，住院醫師也是「醫師」，這同樣可以被視為一種角色？也是一種專業身分認同？這種身分認同與他們作為實習醫師和住院醫師的角色，以及作為醫學專科醫師（例如「外科醫師」）的角色與身分認同有所關聯，但有時又彼此不同。更重要的是，我們發現，採取角色觀點並不符合我們的資料。關於工作與身分認同學習循環相互作用的故事，似乎更接近 Becker 及其同事（1961）所稱的「前角色」活動。儘管角色理論的研究，尤其是角色轉換的研究，提供了一些重要線索，幫助我們理解資料（例如 Ashforth [2001] 所描述的社會驗證作用），但我們仍發現角色觀點與資料的契合度很低。

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從 120.126.194.245 下載。所有使用均受 https://about.jstor.org/terms 規範

oric in creating self-identity (e.g., Alvesson, 1994; Sveningsson & Alvesson, 2003). Still others (echoing the work of Hughes and colleagues) have emphasized the role of "working and doing" in the creation of self (e.g., Van Maanen, 1997; Wrzesniewski & Dutton, 2001).

Such identity work, as well as the cited work in other areas, is promising, yet it remains a loosely affiliated body of research that has yet to systematically tackle issues of identity construction. We sought to redress this shortcoming by building theory on how a professional creates a sense of professional identity.

METHODS

Sample and Context

Our aim was to build theory in the area of professional identity construction and to extend existing theory by making it "more dense by filling in what has been left out—that is by extending and refining its existing categories and relationships" (Locke, 2001: 103; see also Lee, Mitchell, & Sablynski, 1999). Here, we use a grounded theory approach to better understand unexplored dynamics regarding identity construction among professionals.

In order to build and elaborate theory, we searched for a context that could serve as an "extreme case" (Eisenhardt, 1989; Pettigrew, 1990). Extreme cases facilitate theory building because the dynamics being examined tend to be more visible than they might be in other contexts. Applying this criterion, we chose to study medical residents in different specialties at a highly ranked graduate medical education center. We chose the medical profession because its segmentation let us explore professional identity differences among different groups within a general (medical) context (Wyatt, 1978).

We represent the segmentation of this profession by examining medical residents in three specialties: primary care, surgery, and radiology. The choice of these three physician groups maximized differences along two dimensions thought to be particularly relevant to the work of physicians: generalist versus specialist, and high to low degree of patient contact. The generalists, represented by primary care physicians, are primarily interested in the prevention and treatment of organ-based diseases through the use of medicines and other nonsurgical techniques. In contrast, surgeons are specialists who focus on acute problems that can be helped through direct and invasive intervention. Finally, radiologists are primarily diagnosticians who specialize in figuring out what is wrong with patients through the use of various types of imaging equipment. The amounts of training these three physician types require after medical school reflect these differences in their degree of specialization, with the primary care residents requiring three years of training; radiology residents, four; and surgical residents, from five to six. Regarding patient contact, primary care doctors tend to have the most frequent interactions with conscious patients; surgeons have less contact; and radiologists have the least contact. Given the differences among these types of physicians in focus of training, length of socialization, and kinds of tasks required at work, we felt that examining this combination of physician types would give us three distinct windows through which to view identity construction.

To better examine differences in identity dynamics across these specialties, we chose to study physicians during their residency, because professional identity formation and change are thought to be the most pronounced during that time (Blackwell, Gutmann, & Jewell, 1984). In the United States, residency occurs between the completion of medical school, where a physician obtains an M.D., and the beginning of independent practice. It also serves as an important boundary crossing from student to practitioner, as specialty and subspecialty training and experience both enhance and refine general medical knowledge. Research suggests that all doctors who have completed medical school have some values and beliefs in common as a result of their semistandardized training—and, in essence, have a generalized medical professional identity (e.g., Conrad, 1988; Knight, 1973). Residency represents the initial formation of more permanent, differentiated (i.e., specialty) professional identities. As such, medical residency is a relatively clear context for viewing how professional identities develop (Becker et al., 1961; Hughes, 1956; Marion, 1989, 1991).

Finally, we wanted to study identity dynamics within the same medical education training center. By controlling for organization, we were better able to see differences in identity construction that were attributable to residency type (e.g., surgery versus primary care) as opposed to those caused by differences between medical centers. Further following the logic of choosing extreme cases, we wanted to find a medical center that was known for its intensity in training residents. When initially screening organizations, we saw that the training practices of this organization were well known (Kaufmann & Pratt, 2005). In particular, both members of and recruits to this organization characterized it as demanding, high quality, and somewhat militant. Be-

This content downloaded from
[illegible]20.126.194.245 on at, 31 ct 2026 06:54:44 UTC [illegible]
All use subject to https://about.jstor.org/terms

論述在自我認同建構中的作用（例如，Alvesson，1994；Sveningsson 與 Alvesson，2003）。其他研究者（呼應 Hughes 及其同事的研究）則強調「工作與實踐」在自我建構中的作用（例如 Van Maanen，1997；Wrzeniewski 與 Dutton，2001）。

這類身分認同研究，以及其他領域中所引用的相關研究，雖然頗具潛力，卻仍是一批鬆散聯繫的研究，尚未系統性地處理身分認同建構的問題。我們試圖透過建立有關專業人士如何形成專業身分認同感的理論，來彌補這項不足。

方法

樣本與背景

我們的目標是在專業身分認同建構領域建立理論，並延伸現有理論，使其透過填補其中的空缺，使理論「更加周延」遺漏了什麼？也就是透過延伸精煉其既有的類別與關係（Locke, 2001:103；另見 Lee, Mitchell, & Sabski, 1999）。在此，我們採用紮根理論方法，以更深入了解尚未探索的動態[illegible]g 認同[illegible]y 建構[illegible]g p專業人士為了建立並闡述理論尋找一個可作為極端案例」（Eisenhardt，1989；Pettigrew，1990有助於建構理論，因為所探討的動態在其中往往比其他情境更為明顯。應用此項標準，我們選擇研究不同專科的住院醫師，他們就讀於一所排名極高的研究所醫學教育中心。我們選擇了醫專業，因為其分化使我們得以探不同群體之間的專業認同差異在一般（醫學）脈絡中（Wya 1978）。我們透過考察三個專科的住院醫師，來呈現這個專業的分化：基層醫療、外科與放射科。之所以選擇這三類醫師，是因為它們在兩個被認為與醫師工作尤其相關的面向上，呈現最大的差異：全科醫師與專科醫師，以及病人接觸程度由高至低。以基層醫師為代表的全科醫師，主要關注透過藥物及其他非手術技術，預防和治療以器官為基礎的疾病。相較之下，外科醫師是專科醫師，專注於可透過直接且侵入性的介入加以處理的急性問題。最後，放射科醫師主要是診斷專家，專門運用各種影像設備來判斷病人究竟出了什麼問題

[illegible]g用以觀察認同的方式[illegible]y 建構。

為了更仔細地檢視不同專科之間的身分差異，我們選擇研究住院醫師，因為人們認為，專業身分的形成與變化在這段期間最為顯著（Blackwell、Gutmann 與 Jewell，1984）。在美國，住院醫師訓練發生於醫學院畢業（醫師取得醫學博士學位）之後，以及開始獨立執業之前。這段訓練也構成了從學生到執業者的重要跨界歷程，因為專科與次專科的訓練及經驗，會進一步增強並細化一般醫學知識。研究顯示，所有完成醫學院教育的醫師，由於接受了半標準化的訓練，因此都具有某些共同的價值觀與信念？換言之，他們都具備概括性的醫療專業身分（例如 Conrad，1988；Knight，1973）。住院醫師訓練代表了更為長久且具差異化（即專科化）之專業身分的初步形成。因此，醫療住院醫師訓練是觀察專業身分如何發展的相對明確情境（Becker 等人，1961；Hughes，1956；Marion，1989, 1991）。

最後，我們希望研究同一所醫學教育訓練中心內的身分動態。透過控制組織因素，我們得以更清楚地看出身分建構上的差異，究竟是源於住院醫師訓練類型（例如外科與初級照護），還是由不同醫學中心之間的差異所造成。進一步依循選擇極端案例的邏輯，我們希望找到一所因訓練住院醫師時的高強度而聞名的醫學中心。在最初篩選組織時，我們發現這個組織的訓練做法廣為人知（Kaufmann & Pratt, 2005）。特別是，該組織的成員與招募對象都形容它要求嚴格、品質卓越，而且帶有某種軍事化色彩。由於這些特徵，我們將這個組織稱為「新兵訓練營」。3 目前放射科主任的這段話傳達了新兵訓練營背後的訓練理念：

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從[illegible]20.126.194.245 下載 [illegible]所有使用均受 https://about.jstor.org/terms 條款約束

cause of these characteristics, we labeled the organization "Boot Camp."[3] This excerpt from the current chair of radiology conveys the training philosophy behind Boot Camp:

> I am from the old school so I like the Boot Camp "commando" image. . . . This place grew up to take care of patients. Taking care of patients is 24 hours a day, seven days a week. Sickness takes no holiday. Medicine does not lend itself to the modern theories of how you should conduct your personal life and run your family. You say, "Oh I owe my family this" and I'm like, "Great—who's gonna take care of the patients because they don't come in 9–5 and they don't skip weekends and holidays?" That's not their luxury and they are literally putting their lives in your hands so you've got to take care of them. Boot Camp, unlike most of the universities' practices, grew up taking care of patients so they developed this culture of the Boot Camp "commandos" and they were on [call] every other night at a time when most programs were on every fourth night.

Taken together, this sample and context provided an excellent opportunity to examine the construction of professional identity.

Data

The primary method of data collection involved semistructured interviews with residents ($n = 29$) and medical faculty members (known as "attendings"), administrators, and other physicians, such as chief residents, responsible for the design and implementation of the residency training programs at Boot Camp ($n = 11$). We chose as participants various types of physicians (primary care, surgery, and radiology)[4] who were each beginning their residency at Boot Camp. Resident interviews occurred at four points in time, the first three during their first year—at 0 months (wave 1), 6–8 months (wave 2), and 12 months (wave 3)—and the last at the end of residency; these exit interviews were in the third year for primary care, the fourth year for radiology, and the fifth or sixth year for surgery.

We concentrated our efforts on interviewing individuals who were new to Boot Camp because we wanted to focus on members who were simultaneously being exposed to their profession and their organization. Demographically, our resident sample was approximately 38 percent female, with most of the women being in primary care (6 of 11 in primary care, 3 of 8 in radiology, and 2 of 10 in surgery). Although we did not ask about age or race, it appeared that most informants were in their mid to late twenties, and most were white.

Interviews were performed at the hospital and lasted approximately one hour on average. The first and third authors recorded the interviews as well as took notes, and interviews were transcribed verbatim. Although we modified interview protocols during each wave of data collection to take advantage of emerging themes (Spradley, 1979), common to each set of protocols were questions about each member's perceptions of his or her (1) professional (physician) identity(-ies), (2) organization's identity, (3) daily activities and training, and (4) relationships (e.g., with attendings and with other residents). This common set of questions allowed us to see changes in members' responses to questions over time.

These data were supplemented with short surveys, archival analysis, and unobtrusive observations. Surveys investigated how and with whom residents spent their time as well as how residents perceived their specialty vis-à-vis other specialties. For example, we asked each resident about the amount of time he or she spent working, studying, sleeping, and being with his or her partner, and the percentage of time spent with various groups (e.g., patients, nurses, attendings, and his or her cohort). We also followed residents during their visits ("rounds") with patients and made general observations of how the hospital functioned. These were made to confirm residents' descriptions of both their work tasks and the general hospital environment. Finally, we gathered archival documents from various sources, including internal department files on resident selection, historical books about the organization, news articles about the organization, and hospital and departmental pamphlets. Taken together, these secondary sources of data provided a richer context for understanding members' responses and sparked new questions for interview protocols that could be addressed in subsequent interviews with informants.

Data Analysis

In an iterative fashion, we analyzed the qualitative data by traveling back and forth between the data and an emerging structure of theoretical argu-

[3] Names of the graduate medical education center, physicians, and other identifying information discussed here have been changed to protect the anonymity of our informants.

[4] The primary care group also included a few internal medicine residents. Similarly, the surgery group included mostly general surgical residents, but also a few orthopedic surgical residents. We did not find significant differences in identity customization patterns within these subgroups. Therefore, for the sake of parsimony, we refer to these groups as "primary care" and "surgery."

This content downloaded from
120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

造成這些特徵的原因是，我們將這個組織稱為「新兵訓練營」。3 目前放射科主任的這段話傳達了新兵訓練營背後的訓練理念：

> 我來自舊派，所以我喜歡 Boot Camp「突擊隊」的形象。……這個地方發展起來，就是為了照顧病患。照顧病患是一天 24 小時、一週七天的工作。疾病不會放假。醫學這個行業不適合套用現代人對個人生活和家庭經營方式的理論。你會說：「噢，我還欠家人這些
>
> "
>
> 而我會想：「很好？那誰來照顧病患？他們不會只在朝九晚五的時間來，也不會避開週末和假日啊？」這不是他們的
>
> r
>
> 奢侈，他們真的把自己的性命交到 n
>
> 你手上，所以你得照顧好他們。和多數大學的作法不同，Boot Camp 是為了照顧病患而發展起來的，因此形成了 Boot Camp「突擊隊」文化；在多數訓練計畫採取每四晚值班一次的時候，他們卻每隔一晚就得值班[call] 。

綜合而言，這個樣本與情境為探討專業認同的建構提供了絕佳機會。

Data

主要的資料蒐集方法包括 lved
與住院醫師進行半結構式訪談 [n = 29)
以及醫學院教職員（稱為「主治醫師 nd
ing^」）、行政人員及其他醫師，例如 uch
總住院醫師，負責設計 and
住院醫師訓練計畫的實施 ms
在訓練營（n = 11）中。我們選擇參與者 ants
各種類型的醫師（基層醫療、外科 y,
及放射科）4，他們當時都正開始 res
在訓練營進行住院醫師訓練。住院醫師訪談於 rred
四個時間點進行，前三次是在 eir
第一年？在第 0 個月（第 1 波）、第 6 至 8 個月（第 2 波）和第 12 個月（第 3 波）？最後一次是在 wave
住院醫師訓練結束時；這些離職訪談是在第 end
年進行，家庭醫學為第四年，放射科為 ird
而外科為第五或第六 y年。 gery. gy,
我們將努力集中於訪談 g in

剛接觸訓練營的個人，因為 we
我們希望聚焦於同時 ulta
接觸其專業領域與 heir
組織。在人口統計方面，我們的住院醫師 am
樣本約有 38% 為女性，其中 ith
大多數女性任職於初級照護領域（10 人中有 6 人 1 in
初級照護、放射科 8 人中有 3 人，以及 10 人中有 2 人 in
外科）。雖然我們沒有詢問年齡或種族 ace,
看起來大多數受訪者都處於三十多歲 id
二十多歲後期，而且大多數是白人。
訪談於醫院進行 and
平均約持續一小時。第一 rst
和第三作者也將訪談錄音 ell
並做筆記，訪談內容也被逐字謄錄 ver
字。儘管我們修改了訪談方案 cols
在每一波資料蒐集期間，以掌握新興主題的 van
優勢（Spradley, 1979），針對 mon
每組訪談方案所提出的問題涉及 each
成員對其（1）專業 onal
（醫師）身分的看法、（2）組織的 iden
身分、（3）日常活動與訓練，以及（4） rela
關係（例如與主治醫師及其他 res
住院醫師的關係）。這組共通問題讓我們得以 us
觀察成員對問題的回應隨時間 ons
如何改變。

我們也透過簡短問卷、檔案分析及非介入式觀察，補充這些資料。問卷探討住院醫師如何安排時間、與誰共度時光，以及他們如何看待自己的專科與其他專科的相對位置。例如，我們詢問每位住院醫師花多少時間工作、讀書、睡覺及陪伴伴侶，以及有多少時間與不同群體相處（例如病人、護理師、主治醫師及同儕）。我們也跟隨住院醫師探訪病人（「查房」），並普遍觀察醫院的運作情形。這些觀察旨在確認住院醫師對工作內容及醫院整體環境的描述。最後，我們從各種來源蒐集檔案文件，包括科內有關住院醫師甄選的內部資料、介紹該組織的歷史書籍、相關新聞報導，以及醫院和科部的宣傳手冊。綜合而言，這些次級資料提供了更豐富的脈絡，有助於理解成員的回應，也引發了新的問題，供後續訪談時向受訪者探詢。

3 研究所醫學教育的名稱 er,
醫師及討論的其他識別資訊 ssed
此處均已更改，以保護受訪者的匿名性 ur
受訪者。4 初級照護組也包括幾名內科 al
住院醫師。同樣地，外科組 p in
主要由一般外科住院醫師組成，但也包括一名 few
骨科外科住院醫師。我們並未發現顯著 ant
在身分認同調整模式上有差異 thin
這些子群體之間。因此，為求簡潔 , we
我們將這些群體稱為「基層照護」與「外科」。 "

資料分析

我們採用迭代方式分析質性資料，在資料與逐漸形成的理論論證架構之間反覆往返

此內容下載自120.126.194.245，時間為，31 ct 2026 06:54:44
UTC 0所有使用均受 https://about.jstor.org/terms 條款約束

ments (Locke, 2001; Miles & Huberman; 1994; Strauss & Corbin, 1990). This analysis utilized three major steps.

Step 1: Creating provisional categories and first-order codes. We began by identifying statements regarding our informants' views of the world via open coding (Locke, 2001) and then drew on common statements to form provisional categories and first-order codes. Following the procedures recommended by Miles and Huberman (1994), we used a contact summary form to record the provisional categories revealed in each interview at each point in time. For example, by the second wave of interviews, there were several data fragments that related to the general question raised by the residents, "Why are we doing this work?" After codes were named and categories were constructed, we reviewed the data again to see which, if any, fitted each category. Sometimes the revisited data did not fit well into a category, which led either to abandonment or revision of a category. As noted, we used categories recognized in a wave of data to modify and direct later interviews as data collection continued (Spradley, 1979).[5]

Step 2: Integrating first-order codes and creating theoretical categories. Codes from each wave were consolidated for each group. That is, we summarized the contact forms compiled from all the data collected from surgical, radiology, and primary care residents in the wave 1, 2, 3, and exit interviews into different sets of themes (e.g., surgical residents during wave 1). This stage of analysis allowed us to compare intraspecialty and interspecialty differences within and across waves and to detect developmental changes in our variables of interest (e.g., identity and work). As we consolidated categories, they became more theoretical and more abstract. That is, we moved from open to axial coding (Locke, 2001; Strauss & Corbin, 1998). To illustrate, coding statements about the questions residents had about the work that they did led us to see that physicians' professional identity beliefs were not being validated. We used the category "violations to identity" to capture these elements.

Step 3: Delimiting theory by aggregating theoretical dimensions. Once theoretical categories had been generated, we looked for dimensions underlying these categories in an attempt to understand how different categories fitted together into a coherent picture. For example, some categories seemed more like process (e.g., "identity customization"), but others were more like states or structures (e.g., "perceived competence"). We brainstormed alternative conceptual frameworks or models that described how these themes related to one another and to available organizational theories. Once we had identified a possible framework, we reexamined the data's fit/misfit with our emergent theoretical understanding (e.g., Becker, 1970; Glaser & Strauss, 1967; Locke, 2001).

Figure 1 summarizes the process that we followed, which shows our first-order categories, theoretical categories, and aggregate theoretical dimensions.[6] Specifically, the aggregate theoretical dimensions shown were the ones that best explained the identity construction processes for these physicians.

We continue with a brief overview of our findings, followed by a discussion of how the resident groups engaged in a specific type of identity construction: identity customization. We follow this discussion by proposing a more general theoretical model of identity customization among professionals. Our study concludes with a discussion of the implications of our model both for theory related to identity construction and, more practically, for the training and management of health care and other professionals.

FINDINGS

As we looked at the data from each group of residents over time, two main findings became apparent. First, systematic changes occurred in the respondents' professional identities. As can be seen in Table 1, which summarizes the respondents' accounts of their work and gives exemplary quotations about both work and professional identity, some of these identity changes were rather slight, involving a deepening understanding of a professional identity, and others were more dramatic,

[5] Each coder began by reading all transcripts. A specific discipline was assigned to a specific coder and contact summary sheets were generated. For the sake of consistency and integration, the lead coder coordinated coding across disciplines to identify any potential sources of conceptual overlap. Once contact summary sheets were complete, the coders met numerous times to create theoretical categories. As theoretical categories were created, coders went back and recoded data to see if the codes fitted the emerging abstractions. Where they did not, coders reviewed the "discrepant data" and revised categories accordingly. This process was continued until all coders agreed. A similar process was used to delimit theory. This process continued over the six years of data collection.

[6] We would like to thank our colleagues Kevin Corley and Denny Gioia for providing the template for this figure.

This content downloaded from
20.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

（Locke，2001；Miles 與 Huberman；1994 ；
Strauss 與 Corbin，1990）。本分析採用了三個主要步驟
ps.

步驟 1：建立暫定類別與 d
第一階段編碼。我們首先辨識出受訪者對世界的看法之陳述
d

透過開放式編碼（Locke，2001），接著根據共同陳述形成暫定類別與第一階段編碼。依循該程序
s

我們依據 Miles and Huberman（1994）的建議，使用接觸摘要表，記錄每個時間點的每次訪談所揭示的暫定類別。例如，到了第二輪訪談時，已有幾個與居民提出的普遍問題「我們為什麼要做這項工作？」相關的資料片段。
t

在進行編碼之後

s
我們為其命名並建構類別，接著 e
再次檢視資料，以了解哪些資料（如果有的話）符合 d
每個類別。有時，重新檢視的資料並不 t

適合歸入某個類別，因而導致放棄或修訂該類別。如前所述，我們採用了在某一輪資料收集期間所辨識出的類別
o

修改並引導後續訪談，因為資料蒐 c
集工作持續進行（S pradle y，1979）。5

步驟 2：整合第一階編碼並建立理論類別。將各波次的編碼按群組整合。也就是說，我們將從外科、放射科及基層醫療住院醫師收集的所有資料中，彙整第一、二、三波訪談及離職訪談所編製的聯絡表單，並整理成不同主題組（例如，第一波的外科住院醫師）。
i

住院醫師在第一、二、三波及離職訪談中的資料，整理成不同的主題組（例如，第一波的外科住院醫師）。此分析階段
s

讓我們得以比較各波次內與跨波次的專業內及專業間差異，並偵測我們所關注變項（例如身分認同與工作）的發展變化。隨著我們逐
i

漸整合類別，它們變得更具理論性且 d
更加抽象。也就是說，我們從開放式編碼轉向軸心編 l
碼（Locke，2001；Strauss & Corbin，1998）。T o
說明針對這些問題的編碼陳述

5 每位編碼者先閱讀所有逐字稿。每個特定學科均指派給一位特定編碼者，並製作接觸摘要表。為了
f

維持一致性並整合分析，首席編碼者協調各學科間的編碼工作，以辨識任何可能的概念重疊來源。完成接觸摘要表後，編碼者多次開會建立理論類別。建立理論類別後，編碼者回頭重新編碼資料，以確認各代碼是否符合逐漸形成的抽象概念。在
y

若沒有，編碼者便檢視「不一致的資料」，並據此修訂類別。這個過程持續進行 d

直到所有編碼者達成共識。我們也採用了類似的過程來界定理論。這個過程在六年的資料蒐集期間持續進行。

住院醫師對自身工作抱持的看法，讓我們發現醫師的專業認同信念並未獲得肯定。我們使用「認同違
y」以捕p捉這些元素。

步驟 3：透過彙整理論面向來界定理論。在形成理論類別後，我們尋找這些類別底層的面向，試圖了解不同類別如何彼此契合，形成一幅連貫的圖像。例如，有些類別
s

看起來較像過程（例如「身分客製化」），但其他類別則更像狀態或結構（例如「知覺到的能力」）。我們集思廣益，提出描述這些主題如何彼此關聯的替代性概念架構或模型，
o

彼此之間以及可用的組織理論進行比較。一旦確認了可能的架構，我們便重新檢視資料與當時逐步形成的理論理解是否契合或不相契合（例如 Becker，1970
;
Glaser 與 Strauss ，1967；Locke ，2001）。

圖 1 彙整了我們所遵循的過程，其中呈現了我們的一階類別、理論類別以及整合性理論構面。[6] 具體而言，圖中所示的整合性理論構面，是最能解釋這些

phy醫師。

我們接著簡要概述研究發現，隨後討論住院醫師群體如何參與一種特定類型的身分建構：身分客製化。我們在此討論之後，提出一個更具一般性的專業人士身分客製化理論模型。本研究最後討論該模型對身分建構相關理論的影響，以及更實務地說，對醫療照護及其他
r

專業人士 ·

研究發現

隨著我們檢視各組住院醫師長期以來的資料，逐漸浮現兩項主要發現。首先，受訪者的專業認同出現了系統性的變化。如表 1 所示，該表彙整了受訪者對其工作的敘述，並摘錄了關於工作與專業認同的代表性引言；其中一些認同變化幅度較小，僅涉及對專業認同理解的深化，另一些則較為顯著，

6 我們感謝同事 Kevin Corley 與 Denny Gioia 提供此圖的範本。

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從20.126.194.245 下載 所有使用均受 https://about.jstor.org/terms 約束

FIGURE 1
Overview of Data Structure[a]

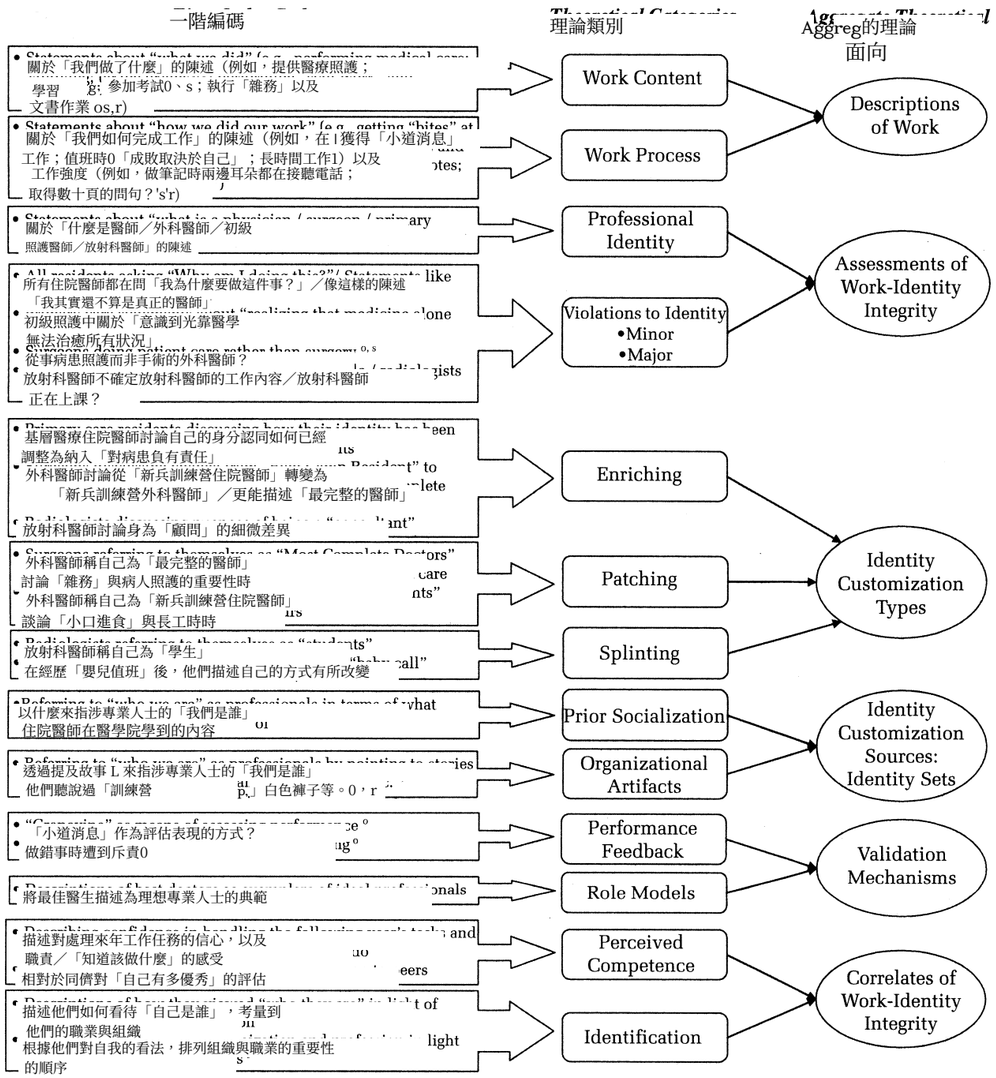


[a]All data were derived from semistructured interviews; "o" indicates "supplemented with observations"; "s" indicates "supplemented with survey data"; and "r" indicates "supplemented with archival data."

involving changes in professional identity. Second, we see in Table 1 that the *work* itself changed throughout residency. As we further analyzed the data, it became clearer that changes in identity were intertwined with changes in work. Specifically, we found that professional identity changes occurred when the residents' ideas about "who they were" as professionals (i.e., their *professional identity*) did not match the work that they did. We refer to these comparisons as *work-identity integrity assessments*. We use the term "integrity" not only because it implies consistency between who one is and what one does, but also because it implies sound structure, such as when one speaks of

This content downloaded from
120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

圖 1
資料結構概覽3



a所有資料皆來自半結構式訪談；「o」表示「以觀察資料補充」；「s」表示「以調查資料補充」；「r」表示「以檔案資料補充」。

涉及專業認同的改變。其次，從表 1 可見，住院醫師訓練期間，工作本身也有所改變。隨著我們進一步分析資料，身分認同的變化與工作變化交織在一起，這一點也愈發清楚。具體而言，我們發現，當住院醫師對「我是誰

他們身為專業人士的自我認知（亦即其專業身分認同）與實際從事的工作並不相符。我們將這些比較稱為工作與身分認同的一致性評估。我們使用「一致性」一詞，不僅因為它意味著一個人的身分認同與實際所為相符，也因為它意味著結構穩固，例如人們會談到

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從120.126.194.245 下載 所有使用均受 https://about.jstor.org/terms 約束

TABLE 1
Work and Identity during the Course of Medical Residency

Program	Beginning to Midpoint of First Year—Waves 1 and 2	End of First Year—Wave 3	Exit Interview
Primary care (two–three years) Work	*Specific tasks:* Writing orders, getting consults, admits (new patients), discharges, rounds, physical exams, histories. *Time breakdown:* Time working, 64.9% Time sleeping, 17.1% Time with non-work others, 17.1%. "[During the day I do] histories, physicals, and just talk to them [the patients] about what is going on. That also means like ordering their tests and following up on their tests and doing all the things I was telling you about, about them and taking care of them." (wave 2, interview)	*Specific tasks:* Same as wave 2. *Time breakdown:* Time working, 59.9% Time sleeping, 22.9% "You write all the orders and the nurses can't do anything. You can't give a diet tray or a meal tray without an order. . . . You write all the orders, you do all the grunt work. You're there overnight taking care of problems that come up."	*Specific tasks:* Same as waves 2 and 3 but with increased supervision of junior residents and medical students. *Time breakdown:* Time working, 51.2% Time sleeping, 25.7% "The [medical] students present the patients and it's helpful if you've gone over the presentation or at least the patient with the student before they present it because you kind of feel like the parent out in the audience when your kid's up to do the spelling bee."
Professional identity	"[A primary care physician] has an overall picture of that patient and can kind of coordinate care everywhere. When that patient gets admitted to the hospital . . . you can come in and see them and they'll see that familiar face and even if they're having GI people and cardiologists work on them they'll know that someone knows them—and hopefully [the primary care physician] has been taking care of them 5 to 10 years and can kind of coordinate everything and steer them in the right direction and make sure they're seeing the right specialist and that they're appropriately being taken care of." (wave 1 interview)	"For my patients, I try and make sure they get everything in a very timely fashion. I try and fight for my patients for what they need. In a way I feel like I'm not just their doctor but their advocate too." "In some ways, having patients is like having children. You kind of worry about them and wonder if you should nag them or they've got to figure out that their diabetes is their problem it's not mine. I think it's kind of the same thing. You have to worry about the whole patient and I worry about my patients."	"A primary care physician is somebody who really cares about these people, worries about these people, and it is kind of woven into the fabric of their life where 24 hours a day, they are available to them as if they are their children."

the integrity of a bridge. As noted in Figure 1, some violations of this work-identity integrity were relatively major and lead to a devaluation of the identity. Others were relatively minor. Furthermore, we found that differences in these work-identity integrity assessments resulted in different *identity customization* processes. We use the term "identity customization" to denote that identity is tailored to fit the work at hand, and not vice versa.

In describing this process, we begin with a discussion of the work that these residents did, which helps inform how they made work-identity integrity assessments—and, ultimately, how they customized their professional identities. We continue by addressing how these customized identities were socially validated and how identity construction correlated with perceived work competence and different patterns of identification with the

This content downloaded from
120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

表1 醫學住院醫師培訓期間的工作與身分認同

計畫	Beginning to Midpoint of 第一年？第1波與第2波	第一年結束 第 3 波	離職訪談
基層醫療（兩至三年）工作：具體任務：撰寫	g ord者、接 tti診諮 lts. 收治（新病患）、辦理出院、查房、身體檢查、問診。 時間分配： 工作時間，64.9% 睡眠時間，17.1% 與非工作相關他人相處的時間，17.1%。 "[白天我會對病人] 問診，進行身體檢查，並只是和他們聊聊 [病人] 目前的情況。這也包括像是安排 g 他們接受檢查，並追蹤 g 檢查結果 u 他們的檢查，並完成所有 e 我之前告訴你的那些事情， t, 了解他們並照顧 f 他們。」（第 2 波，訪談）	具體任務：與第 2 波相同。 時間分配： 工作時間，59.9% 睡眠時間，22.9% 「所有醫囑都是你開立，而護理師什麼也不能做。 Y 你不能提供飲食餐盤或餐盤 沒有醫囑……你 會寫下所有醫囑，你還要做所有繁重的工作。你會 整晚待在那裡處理 " 突然發生的問題。"	具體任務： 與第 2 波和第 3 波相同，但有所增加 指導初級 住院醫師與醫學學生。 時間分配： 工作時間，51.2% 睡眠時間，25.7% "這些[醫學] 學生 介紹病人，而且 如果你已經 已經談過的 報告，或至少 這位病患 學生，在他們 報告之前，因為你 會有點覺得 那位家長在外面 觀眾，當你的 孩子準備參加 拼字 g 比賽時。」
專業認同	"[基層照護醫師] 對該名患者有 d 整體的了解，並且 可以在各處協調照護 e 當那位病人 t 住院時…… 你可以進去探視他們 e 他們會看到那張熟悉的臉 即使他們出現腸胃道 n 病患與心臟科醫師共同努力 e 照顧他們，讓他們知道有人 e 認識他們？希望[這位家庭醫師] 已經 n 照顧他們五到十年 s 而且能協調各方 e 所有事情，並引導他們走向 e 正確的方向，並確保 t 他們看的是合適的專科醫師 而且他們是 適當的 y 有人照料。」（第一波訪談）	"對我的病人，我會盡力 確保他們能獲得 ly 所有所需的一切，並且非常及時地 y 提供。我會盡力為 r 病患爭取他們 ey 所需的照護。某種程度上，我覺得 y 我不只是他們的醫生，還 t 也要為他們發聲。" "在某些方面，照顧 病人就像照顧 孩子一樣。你會有點 擔心他們，並且 想知道自己是否該不該 g 他們，或是他們得 弄清楚那是他們的 糖尿病是他們自己的問題 不是我的。我想這是 差不多同一回事。你 得擔心 y 整體病人，而我擔心 y 我的病人。"	"一位基層醫療醫師 是個真正 關心這些人， 擔心這些 人，而且這有點 融入了 他們的生活，讓他們 24 小時 s a day, 隨時都能提供服務 y are 對他們而言，彷彿 他們的孩子。」

橋梁的結構完整性。如圖 1 所示，有些工作與身分認同的一致性落差相對較大，因而導致身分認同貶值；另一些則相對較小。此外，我們發現，工作與身分認同的一致性評估有所不同，也會導致不同的身分認同調整過程。我們使用「身分認同調整」一詞，表示調整身分認同以符合手邊的工作，而非反過來調整工作以符合身分認同。

在描述這個過程時，我們先討論這些住院醫師所從事的工作，藉此了解他們如何評估工作身分的完整性，並進一步探討他們如何調整自己的專業身分。接著，我們說明這些調整後的身分如何獲得社會認可，以及身分建構如何與他們感知到的工作能力及不同的認同模式相關。

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從 120.126.194.245 下載。所有使用均受 https://about.jstor.org/terms 條款約束

TABLE 1
Continued

Program	Beginning to Midpoint of First Year—Waves 1 and 2	End of First Year—Wave 3	Exit Interview
Surgery (five–six years) Work	*Specific tasks:* Rounds, conferences, writing orders, getting consults, physical exams, histories, discharges. *Time breakdown:* Time working, 65.5% Time sleeping, 16.9% Time with non-work others, 12.6% "An intern [first-year medical resident] is something like a *secretary* and a *social worker* and a *physical therapist* and an *errand* boy. You just take care of business as an intern. . . . That means you know—that means writing down hundreds of pages a day of text into different places—patients' charts and forms. It is just an unbelievable pile of paperwork that somebody has to do. . . . The interns get left to do all that sort of crap." (wave 2 interview)	*Specific tasks:* Same as wave 2. *Time breakdown:* Time working, 70.8% Time sleeping, 17.0% "So much time is spent doing menial labor . . . very little time is left to read."	*Specific tasks:* Supervision of residents, surgery. *Time breakdown:* Time working, 51.2% Time sleeping, 25.7% "This year I had more responsibility. You really have to know what's going on. You have a lot more teaching responsibilities. At the same time, a lot of the scut work falls on other people, so it was nice." "We do a lot less ICU [intensive care unit] work, less sort of floor work and more consultative work. And then the bulk of what you do—surgery."
Professional identity	"They [surgeons] don't like to sit and think too much. They are great actors instead of thinkers. They get up earlier. They get things done quicker. They do things with their hands." (wave 1 interview)	[We are] the most complete doctors in the hospital in terms of being able to address whatever ailment comes into the ER [emergency room]." "The Boot Camp way is trying to always look your best and everything like that and try not to let yourself show that you're 30 hours behind sleep for the week. Always look clean and pressed. You should try to be pleasant and try to carry yourself around in that manner."	"I think the main thing that surgeons have over most disciplines is that they have a therapeutic armory. If you are a medical doctor . . . their only arsenal is medicine . . . radiologists don't even treat patients. Anesthesiologists don't even treat patients. They treat them while the surgeon does what they need to do." "Well, being a Boot Camp surgeon is a little different than being a general surgeon. We tend to identify with each other and with the particular experience of being here more, I think, than we identify with the greater community of general surgeons."

This content downloaded from 120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

表 1
（續）

計畫	Beginning to Midpoint of 第一年？第 1 波與第 2 波	第一年結束時 第 3 波	離職面談
外科（五至六年）工作	具體工作： 查房；會議；寫作； 開立醫囑、安排會診、進行身體檢查、詢問病史、辦理出院。 時間分配： 工作時間，65.5% 睡眠時間，16.9% 與非工作相關人士相處的時間，12.6% 「實習醫師 [第一年醫學住院醫師] 有點像一位秘書、社工，還有物理治療師，以及一個跑腿的。你就是負責處理實習醫師的各種雜務……也就是說你懂嗎？意思就是每天寫下數百頁文字到不同的地方？病人的病歷和表格。這簡直是難以置信的一大堆某人必須處理的文書工作……實習生就這樣被留下來去做那種鳥事。」（波次第 2 次訪談）	具體任務：與第二波相同。 時間分配： 工作時間，70.8% 睡眠時間，17.0% 「有太多時間花在做繁瑣的勞務上……幾乎沒有時間可以閱讀。」	具體任務： 督導住院醫師 residents, 手術。 時間分配： 工作時間，51.2% 睡眠時間，25.7% "今年我有更多責任。你真的必須了解正在發生什麼事。你有更多教學責任。在同時，許多雜務由其他人員承擔，所以感覺很好。" "我們做的加護病房 [加護病房] 工作少了，某種程度上的病房工作也少了，而更多的是，會診工作。而接著，你大部分的工作都是手術嗎？"
專業認同	「他們 [外科醫師] 不喜歡坐著冥思苦想。他們是出色的演員，而不是思想家。他們起得比較早。他們會事情做得更快。他們會用雙手做事。」（第一波訪談）	[我們是] 最全面的醫院裡最全面的醫師，就能處理各種可能出現的病症，送進急診室的 [急診室]。」 「訓練營的方式是總是努力讓自己看起來處於最佳狀態，凡事都像那樣，並試著不要讓自己顯露出你已經有 30 個小時沒睡了」這一週。總是要看起來整潔且衣著平整。你應該試著待人和善並試著以那樣的方式舉止行事。"	"我認為外科醫師相較於大多數其他專科的醫師，最大的優勢是他們擁有一套治療武器庫。如果你是醫師……他們唯一的武器就是藥物……放射科醫師甚至不治療病人。麻醉科醫師甚至不治療病人。他們在手術期間為病人治療，而外科醫師則做需要做的事。" "嗯，參加新兵訓練營外科醫師有些不同於成為一名一般外科醫師。我們傾向於認同彼此以及共同擁有的特定經驗更多地參與其中，我想，而不是認同於更廣大的一般外科醫師社群。」

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從 120.126.194.245 下載。所有使用均受 https://about.jstor.org/terms 約束

TABLE 1
Continued

Program	Beginning to Midpoint of First Year—Waves 1 and 2	End of First Year—Wave 3	Exit Interview
Radiology (four years) Work	*Specific tasks:* Teaching conferences, studying, shadowing senior residents	*Specific tasks:* Same as wave 2 but with "baby call"	*Specific tasks:* Radiology consults, supervision of junior residents, intensive studying for specialty boards
	Time breakdown: Time working, 45.4% Time sleeping, 25.5% Time with non-work others, 37.7%	*Time breakdown:* Time working, 55.0% Time sleeping, 24.9%	*Time breakdown:* Time working, 39.1% Time sleeping, 27.2%
	"They usually get [you] out of here sometime between 5:00–6:00. Then I usually go home, cook dinner and maybe watch TV while I'm eating and then generally I read for a couple of hours a night." (wave 2 interview)	"Actually as we've started taking call more, you interact more with the physicians in other specialties." "Then when you do you take it [call] until 11:00 at night and it's pretty fun because now you're back in the hospital and you're in charge kind of."	"We have two pretty big board exams during the fourth year, so you spend some time preparing for those early in the year and late in the year."
	"Someone got mad at me for saying it, but I really think we're spoon-fed." (wave 2 interview)		"It [studying for boards] was stressful, it was a lot of work, it was a lot of long afternoons, we spent a lot of time after work as a class going over cases with a different attending everyday for like three or four months, so it was a lot more time, it was a lot more effort."
Professional identity	"Uh, to be a radiologist . . . uh . . . this . . . this is difficult. Cause I haven't been, it's easier for me to say what. . . its like to be a doctor. . . . Uh, to be a radiologist, you know, it will be, this will be good to change—my attitude in six months will be different. Um, to be a radiologist, you have to be. . . or the essence of being a radiologist is that basically you need to make the most important diagnosis [laugh]. That's, that's a given." (wave 1 interview)	"You just don't know enough to contribute a lot and when you start taking call, you get a lot more experience with just seeing a lot more films but you also feel like you are part of the program. You're involved." "I feel like a consultant."	"No matter how much the referring doctors think they know about radiology, you know more than them. You try to be helpful in seeing their point of view and realize how or what you say is going to affect that doctor and his/her patient. If you are able to put all that together and keep it all in mind, you will be a good radiologist. A radiologist is not just someone who can read films, but be a valuable consultant to the referring physicians."

This content downloaded from
120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

表1（續）

計畫	Beginning to Midpoint of 第一年？第1波與第2波	第一年結束 第三波	離職訪談
放射科（四年）工作	具體工作： 教學研討會、讀書、 跟隨資深住院醫師見習	具體任務：與第 2 波相同，但包括 「嬰兒電話」	具體任務： 放射科會診、 督導資淺 住院醫師、密集 studying 針對專科 董事會
	時間分配： 工作時間，45.4% 睡眠時間，25.5% 與非工作對象相處的時間，37.7%	時間分配： 工作時間，55.0% 睡眠時間，24.9%	時間分配： 工作時間，39.1% 睡眠時間，27.2%
	「他們通常會在[你] 離開這裡 大約在 5:00 至 6:00 之間。 然後我通常會回家煮 晚餐，可能邊吃飯邊看電視 ，然後 通常我每晚會閱讀大約幾 個小時。」（第二波 訪談）	「其實，隨著我們開始 更常值班，你 會更多地與 其他專科的 醫師互動。」 「然後當你值班時，你會接 一直[打] 電話到晚上 11:00 而且還滿有趣的，因為 現在你又回到了 醫院，而且你負責 某些事。"	"我們有兩個相當大的 在……期間參加資格考試 所以你 花一些時間 準備那些在年初 以及年底舉行的考試 。」
	「有人因為我這麼說而對我生氣， 但我真的覺得我們被人餵到嘴邊了 。」（第二波訪談）		「[準備國考] 壓力很大，事情很多 要做的事很多 午後常常很漫長，我們 下班後花了很多時間 全班一起進行 處理由不同主治醫師負責的病例 不同的主治醫師 或四個月 所以那段時間 多出不少時間，這需要 多花很多力氣。」 花了更多時間
專業認同	「呃，要成為一名放射科醫師…… 呃……這個……這很難。 因為我一直沒有，比較容易 說明……要成為一名 醫生是什麼感覺……呃，要成為一名 放射科醫師，你知道，這會是…… 這樣會不會有助於改變？我的 態度在六個月後會是…… 不同。嗯，要成為放射科醫師 你必須……或者說，放射科醫師的核心 身為放射科醫師的其中一項特質是 基本上，你需要做出 最重要的診斷[笑聲]。 這是、這是基本要求。」（第一波 訪談）	「你就是還不夠了解 所以能做的貢獻不多，而且 開始值班接聽電話時 你會累積更多 實際經驗，光是觀察就能 更多電影，但你 也會覺得自己是 計畫的一分子。你 參與其中。" "我覺得自己像是顧問。" "	"無論這 轉診醫師認為 他們對放射學的了解 你比他們懂得更多。 你試著提供協助， 設身處地理解他們的觀點 並了解如何或什麼 你所說的會影響 那位醫師及其 病患。如果你能 把這些都整合起來，並 牢記在心，你 就會成為一位優秀的放射科醫師 放射科醫師不只是 能夠判讀 電影，也能成為有價值的 轉介醫師的顧問 。

此內容於 2026 年 31 ct 06:54:44 UTC 從120.126.194.245 下載
所有使用均受 https://about.jstor.org/terms 的條款約束

profession and the organization. We wrap up this section by proposing a theoretical model that integrates the findings of this paper and builds new theory in the area of identity construction.

Descriptions of Work: Work Content and Process

Each group of residents was assigned work by senior members of the hospital (e.g., attendings). As shown in Figure 1, residents further distinguished their work along two dimensions: what they did (work content) and how they did it (work process). Regarding *work content,* primary care and surgical residents performed very similar tasks (see Table 1); both types of residents spent most of their time caring for patients (e.g., treating fevers), doing paper work, and completing a wide range of menial tasks (what they called "scut work"). Neither primary care nor surgical residents experienced many changes in their work during the first year. Surgical residents' work, however, changed dramatically later in their residency.

The initial work content of the radiology residents was quite different from that of the other residents. This difference stemmed from the lack of medical school training in radiology, which limited the ability of these residents to initially act as radiologists. As the chair of the Radiology Department explained:

> Medical schools are fundamentally designed to produce people to go into internal medicine, pediatrics, and surgery. . . .They don't train people to go into radiology. Our residents . . . when they come to radiology, they don't know any of the skills required in radiology.

As Table 1 shows, the radiology program required its residents to attend multiple teaching conferences each day, to learn assigned readings, and to take formal written examinations. Work rotations consisted of sitting side-by-side with a faculty radiologist going over images. Unlike their peers, each radiologist had a relatively large change in work content during the midpoint and end of the first year, with the introduction of "baby call": a truncated call schedule in which they assisted senior residents during some evening and weekend hours. It was here that these residents started to perform the duties associated with being radiologists.

With regard to *work process,* both primary care and surgical residents began with an almost immediate immersion into the work of a resident. For example, after a one-day orientation explaining how Boot Camp worked, surgical residents began working on the first of ten rotations (e.g., cardiothoracic, surgical intensive care) to which they were assigned over the first year. Both primary care and surgical residents felt their lives were consumed by their jobs and reported spending between 100 and 140 hours a week in the hospital. One primary care resident noted during the midpoint of the first year, "By the time I finish work next Friday, it will be my 26th day of work in a row without any days off." Radiology residents, by contrast, found their first year to be considerably less intense. They also reported spending significantly more time sleeping and less time working, logging about 50 hours a week in the hospital.[7] As noted above, however, the intensity of work increased in radiology at the midpoint of their first year, as residents began to be on call and actually perform the work of radiologists.

Work-Identity Integrity Assessments

By the midpoint of the first year, each group of residents struggled to reconcile their professional self-conceptualizations with the work that they did. For primary care residents, who saw themselves as "coordinators of care" for patients over the patients' lives (see Table 1), the integrity violation was relatively minor. While many reported struggling with the breadth of their work tasks, they did not find that it undermined their professional identities. To illustrate:

> I need to not only take care of their medical needs but also take care of any sort of social needs in order for them to achieve the health care that they need. Right now I am still struggling with that. (primary care resident, wave 2)

Integrity violations for surgical and radiology residents were much more severe. As noted in Table 1, surgical residents saw themselves as highly action-oriented and as professionals who effected "dramatic change in disease." Doing paperwork and other "scut work"—such as lowering a patient's toilet seat or deciding whether a patient could have a different flavor of vitamin shake—was at odds with this view of themselves as professionals. As a result, surgical residents felt that what they were

[7] To illustrate, in both the wave 2 and 3 data, *t*-tests showed that radiology residents, when compared with surgical residents, reported spending significantly more time studying ($p < .001$, both waves), sleeping ($p < .05$, both waves), and doing nonwork activities ($p < .07$, wave 2; $p < .05$, wave 1). They also spent significantly less time working ($p < .001$, both waves). Comparing radiology with primary care residents showed a similar pattern.

This content downloaded from
20.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

專業與組織。我們在本節最後提出一個理論模型，整合本文的研究發現，並為身分建構領域發展新的理論。

工作描述：工作內容與流程

每一組住院醫師都被分派工作 b y
醫院資深成員（例如主治醫師）。如圖 1 所示，住院醫師進一步區分 d
他們的工作可分為兩個面向：做什麼（工作內容）以及如何做（工作流程）。就工作內容而言，基層醫療與外科 l
住院醫師執行的工作非常相似（見表 e
1）；兩類住院醫師大部分時間都在照顧病患（例如治療發燒），處理文 a
書工作，以及完成各種繁瑣的 l
工作（他們稱之為「雜務」）。基層醫療與外科住院醫師在第一年工作內容的變化都不大。然而，外科 l
住院醫師的工作後來發生了劇烈變化 y
在他們住院醫師訓練的後期 y.
放射科住院醫師最初的工作內容與其他 r
住院醫師大不相同。這項差異源於醫學院缺乏放射學訓練，因此限制了 d
這些住院醫師最初擔任放射科醫師的能力。放射科主任 t
說明如下：

> 醫學院的基本設計，是培養人們進入內科、小兒科和外科……他們並不訓練人們從事放射學。我們的住院醫師……來到放射科時，並不了解放射科所需的任何技能。

如表 1 所示，放射科訓練計畫要求住院醫師每天參加多場教學會議、研讀指定讀物，並參加正式的筆試。工作輪調包括與一名放射科教師並肩查看影像。與同儕不同的是，每名放射科住院醫師在第一年中期和年底時，工作內容都出現相當大的變化，開始執行「嬰兒值班」：一種縮短的值班安排，在部分晚間和週末時段協助資深住院醫師。正是在這個階段，這些住院醫師開始逐

orm the duti是協 iated w同 b ei ng ra diologists.
就工作流程而言，基層醫療科與外科住院醫師一開始幾乎立即投入住院醫師的工作。例如，在接受一天的訓練，了解 Boot Camp 的運作方式後，外科住院醫師便開始進行第一年安排的十個輪訓科別中的第一個（例如心胸外科、外科加護病房），他們

在第一年期間將依序接受這些輪訓。基層醫療科與外科住院醫師都覺得自己的生活被工作完全佔據，並表示每週有 100 到 140 小時待在醫院。一名基層醫療科住院醫師在第一年中點時提到：「等我下週五結束工作時，我將連續工作第 26 天，中間一天休假都沒有。」相較之下，放射科住院醫師認為第一年輕鬆許多。他們也表示花在睡眠上的時間明顯較多，工作時間則較少，每週約有 50 小時待在醫院。7 然而，如上所述，放射科的工作強度在第一年中點時有所增加，因為住院醫師開始輪值待命，並實際執行放射科醫師的工作。

工作與職業認同完整性評估

到了第一年的年中，各組住院醫師都難以調和自己的職業自我概念與實際從事的工作。對基層照護住院醫師而言，他們認為自己是患者一生中的「照護協調者」（見表 1），因此所感受到的認同完整性受損相對輕微。許多人表示，工作內容涵蓋範圍廣，讓他們感到吃力，但並不認為這會動搖自己的職業認同。以下例子說明：

> 我不僅要照顧他們的醫療需求，也要照顧各種社會需求，才能讓他們獲得所需的醫療照護。我現在還在努力克服這個問題。（基層照護住院醫師，第 2 波）

外科與放射科住院醫師的誠信違規嚴重得多。如表 1 所示，外科住院醫師認為自己高度重視行動，也認為自己是能「徹底改變疾病狀況」的專業人士。處理文書工作和其他「雜務」——例如放下病人的馬桶座，或判斷病人能否換一種口味的維他命奶昔——都與他們對自身專業人士身分的看法相左。因此

，外科住院醫師覺得，他們所做的事

7 舉例來說，在第 2 波和第 3 波資料中，t 檢定顯示，相較於外科住院醫師，放射科住院醫師表示自己花更多時間讀書（兩波皆為 p < .001）、睡覺（兩波皆為
[05, p
< 07,
< .），以及從事非工作活動（第 2 波為 p . ；第 1 波為 p < . 05）。他們花在工作的時間也顯著較少（兩波皆為 [p < .001）。比較放射科與基層照護住院醫師，也呈現類似的模式。

此內容下載自 20.126.194.245，時間為 2026 年 10 月 31 日
06:54:44 UTC 所有使用均受 https://about.jstor.org/terms 約束

doing was removed from their identities as surgeons. As one surgical resident noted:

> Mostly your workload is working on the floor—working with patients, doing paperwork—basically a social worker for the year. . . . All the patient care and all the paperwork is put on you basically. You get removed from thinking that you are here for surgery, you just have to try and block it out. (surgical resident, wave 2)

The end result of this identity violation was that surgical residents did not feel like surgeons. As one resident put it during the midpoint of the first year, "I am not really a surgeon yet." Another even came to devalue his/her role as a physician: "I'm an intern [first-year resident] barely and I don't know anything. I know very little. I understand the human body and can make some good guesses. It doesn't mean much to be an MD anymore—not to me."

Radiology residents reported a similar sense of identity violation. However, the identity quote for radiology in the first column of Table 1 shows less certainty than the identity claims made by the surgical and primary care residents. The quotation in the table—which resonates with the identity descriptions made by his/her peers—is peppered with qualifiers (e.g., "I think," "I guess," "supposedly"); these qualifiers suggest that the resident was not very sure what a radiologist did. However, even this vague understanding of who a radiologist was (e.g., a diagnostician) did not match what they did on a daily basis (e.g., studying, attending lectures). Consequently, they felt useless, stupid, and devalued at work:

> It kind of makes you feel pretty shitty in the middle of the year—you really don't feel very useful a lot of times and you know, here I am 28—I have been studying for however many years in a row and everyone looks at you like you're just a first year. (radiology resident, wave 2)

As one radiology resident reported, "I'm a resident—I'm not even a radiologist."

Identity Customization

Residents handled these minor and major violations of work-identity integrity by customizing their identities. As noted, identity customization by these professionals denotes changes in identity made to fit work demands. We further delineate three different *customization types*—enriching, patching, and splinting—and describe the *identity sets,* or "raw materials" used to customize these identities. Some of these "materials" involve internalized beliefs from prior socialization experiences, while others are more external and involve elements adopted from the organizational context. We discuss customization separately for each residency group.

Primary care: Customization type. As noted above, primary care residents experienced relatively minor violations to their work-identity integrity. Those that did arise came from a growing recognition regarding the scope of responsibility toward patients. This recognition occurred as they were increasingly faced with life and death decisions, as they were contacted by patients at all hours of the day and night, and as they realized how much they had to think about, fight for, and otherwise care for their patients. The result was that primary care residents deepened their understanding of their professional identity. We refer to this identity customization process as *identity enriching.* The basic tenets of the professional identity remained the same, but the understandings of this identity became deeper and more nuanced. Note the fine distinction the following resident makes between medical care and medical treatment when describing (in a wave 3 interview) what it means to be a physician:

> I really think you need to be concerned about people, someone who cares for the sick, not just treats them but cares about them. I guess this year I have been more focused on learning how to care for people better, than [on] learning medicine.

Similar changes are also noted in the exit interviews of the primary care residents (see Table 1). This sense of a primary care physician as someone who treats more than medical needs is fully embraced as they leave their residencies. As such, primary care residents take on a more holistic view as doctors who take responsibility for "caring" for patients:

> I think you play an incredibly important role in a patient's life if you're a good primary care physician. The ideal of what it means to be a primary care physician, it means to be all these to a patient—a friend, ally, advocate, and in some ways a parent figure that guides them through the options. (exit interview)

Primary care: Identity set. It appeared that the main identity beliefs that primary care residents drew upon in making sense of their work were the ones gained during medical school (Knight, 1973). These residents found that the work that they were doing was largely what they expected to be doing—and what they had done during medical school. In a wave 3 interview, one informant said:

> Through med school, you're working with physicians quite a bit so you actually have a few years of

This content downloaded from
120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

與他們作為外科醫師的身分有所脫節。正如一位外科住院醫師所指出：

> 你的工作大多是在病房第一線 ?
> 照顧病人、處理文書？基本上就是一整年的社工…
> …所有病患照護和文書工作基本上都落在你身上。你會
> 忘記自己是來做手術的，只能試著把這件事拋諸腦後。
> （外科住院醫師
> ，第二波）

這種身分認同受侵犯的結果是，外科住院醫師覺得自己不像外科醫師。正如 e
一位住院醫師在第一年中期時這麼說：「我還算不上是真正的外科醫師。」另一位甚至更…… e
……貶低自己身為醫師的角色：「我只是實習醫師 [第一年住院醫師] ，幾乎什麼都不懂，我也不知道…… w
……任何事。我所知甚少。我了解人體，也能做出一些合理的推測。但擁有醫學博士學位並不代表什麼，
y對我來說更是如此。」

放射科住院醫師也表示，他們有類似的身分認同受挫感。然而，表 1 第一欄中的放射科身分認同引言，顯示出的確定程度較低 s

於外科與基層照護住院醫師所提出的身分認同主張。表格中的引言——也呼應同儕對身分的描述——充斥著各種保留語（例如「我想」、「我猜」、「據說」）；這些用語顯示，這位住院醫師並不太確定放射科醫師的工作內容。然而，即使對放射科醫師身分的理解如此模糊（例如診斷者），也與

y

每天都在做的事（例如讀書、上課）。因此，他們在工作中感到自己毫無用處、愚蠢，且不受重視：

> 到了年中，這會讓你覺得很糟吧？很多時候，你真的
> 覺得自己派不上用場；你知道，我都 28 歲了，連續讀了
> 這麼多年書，大家看你的眼神卻像你只是個一年級生。
> （放射科住院醫師，第二波訪談）

一位放射科住院醫師表示：「我只是住院醫師， i
我甚至還稱不上放射科醫師。」

身分客製化 n

住院醫師透過客製化身分，因應工作身分完整性遭受的輕微與重大侵害。如前所述，這些專業人士所說的身分客製化，是指調整身分以符合工作需求。我們進一步區分出三種不同的客製化類型：充實

，

包紮與夾板固定？並描述用來自訂這些身分的身分組合，或「原始素材」 e

身分認同。其中一些「材料」涉及先前社會化經驗所內化的信念，另一些則較為外在，涉及

e

從組織情境中採納的元素。我們分別討論各項住

y group.

基層醫療：自訂類型。如上所述，基層醫療住院醫師的工作身分完整性受到的侵害相對輕微。確實發生的侵害，源自於日益增加的

g

他們對自身照護病患責任範圍的認知。隨著他們愈來愈常面臨生死攸關的決

i

定，以及病患在各種 l
日夜時段聯絡他們，也隨著他們逐漸意識到 d
自己有多少事情必須思考、爭取，以及 d

否則就無法照顧病患。結果是，基層醫療住院醫師對自身專業認同的理解更加深入。我們將這個認同調整的過程稱為認同豐富化。專業認同的基本原則維持不變，但對其理解

f

變得更加深入，也更加細膩 .
請留意以下這位住院醫師所做的細微區分 t
他在描述（於第三波訪談中）何謂

t

成為一名醫師時：

> 我真的認為，你需要關心他人，成為一個照顧病患的人；
> 不只是治療他們，也要關懷他們。我想，今年我更專注於學
> 習如何更好地照顧他人，而不是[on] 學習醫學。

在基層醫療住院醫師的離職訪談中，也發現了類似的變化（見表 1）。他們認為基層醫師不只是滿足患者醫療需求的人， e

這種觀點在他們完成住院醫師訓練時已完全確立。因此，基層醫療住院醫師對醫師的看法更加全面， w

認為醫師有責任「照顧」 r
患者：

> 我認為，如果你是一位稱職的基層醫師，你在
> 患者生命中扮演的角色極其重要。基層醫師的理想
> 形象，意味著要成為患者身邊的各種角色——朋友、
> 盟友、倡議者；在某些方面，也像是引導患者了解
> 各種選擇的家長。（離職訪談）

基層醫療：身分已定型。基層醫療住院醫師在理解自身工作時，所依據的主要身分信念似乎是在醫學院期間形成的（Knight，1973）。他們發現，自己所做的工作大致就是原本預期會做的事，也是他們在醫學院期間做過的事。在第三波訪談中，一位受訪者說：

> 在醫學院就讀期間，你會花不少時間與醫師共事，
> 所以其實已經累積了好幾年的

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從 120.126.194.245 下
載。所有使用均須遵守 https://about.jstor.org/terms

> being in the hospital and working with patients. The expectations that you have I think are pretty well ingrained by the time you get to your residency just because you've been exposed to it for so many years.

By contrast, although primary care residents were aware of Boot Camp's prestige and intense reputation, they did not see the organization as being an important source of identity meaning when resolving work-identity integrity violations. An exemplary exit interview comment regarding the role of Boot Camp in helping them become physicians is, "I think I was a physician and on my way to all that other stuff before I came to Boot Camp."

Surgery: Customization types. The mismatch between identity and work caused the surgical residents to use alternative identities to help make sense of what they were doing. Two types of identities were created: one in reaction to the type of work that they did, and the other in reaction to the intensity of their training. With regard to the content of their work, most of which involved various aspects of patient care, residents came to attach their surgery identities to their preexisting notions of what it meant to be a generalist doctor. This patching created a new, composite identity as residents referred to themselves as the "most complete doctors." Being "most complete" meant that they could do what a generalist (i.e., primary care) doctor could do—clinical treatment—in addition to performing surgery:

> [We are] the most complete doctors in the hospital in terms of being able to address whatever ailment comes into the ER. (wave 3 interview)

We refer to this process as *identity patching* because surgical residents drew upon one identity (medical generalist) to permanently patch up "holes," or deficiencies in their understandings of who they were as surgeons, understandings stemming from the tasks they performed at work. Over the course of their residencies, however, they were able to build from this conceptualization of themselves as the "most complete doctors" to create a deeper, more nuanced understanding of this identity. Thus, by the end of their tenures, surgery residents had moved from an identity-patching to an *identity-enriching* process. To illustrate, note how the description of the "most complete doctor" in the last column of Table 1 came to include sophisticated new metaphors (e.g., armory and arsenals) and how it communicates a growing sense of how incomplete other physicians are compared with surgeons.

A similar type of identity patching occurred in reaction to the intense work process at Boot Camp. Surgical residents felt that they were working harder than other residents in the hospital, and harder than surgical residents at other medical centers:

> I think that most people that train in medicine are overworked during their training. I think surgery is particularly bad. I think Boot Camp is probably particularly bad for surgery. . . . I think Boot Camp is pretty bad. For example my fiancé is a surgical intern at Prestigious Hospital and he works many fewer hours than I do.

To make sense of this situation, surgical residents changed their conceptualizations of who they were to fit *how* they did their jobs—that is, the "Boot Camp way." As noted in Table 1, the "Boot Camp way is trying to always look your best and everything like that and try not to let yourself show that you're 30 hours behind sleep for the week." In incorporating these notions, residents combined their surgeon identities with the Boot Camp identity to become Boot Camp interns (first-year residents). As one resident noted at the end of the first year, "I'd say within the first three months, you pretty much get to be a Boot Camp intern."

Like the most complete doctor identity, the identity of the Boot Camp *intern* grew richer over time, developing into the "Boot Camp *surgeon*." As noted in Table 1, Boot Camp surgeons saw themselves as distinct from other surgeons; moreover, they tended to identify with this group more than with "the greater community of general surgeons."

Although we treat these two examples of patching as relatively distinct responses to work type and work process intensity, it is interesting that descriptions of the "most complete doctor" in exit interviews included war imagery (e.g., "arsenal"; Table 1). This suggests that there may have been some overlap between the two identity construction processes.

Surgery: Identity set. Surgical residents drew upon a variety of sources when customizing their professional identities. Although their use and acceptance of the term "most complete doctor" appeared to grow during their tenure at Boot Camp, residents were first exposed to the concept in medical school. Moreover, the generalist part of this identity appeared to be internalized in medical school:

> Being a physician to me means putting the patient first which means my job is to help you if you are sick. To do what I have to do to make you better. That means I have to use the knowledge that I received in medical school and my training as a resident to put your priorities first regardless of my ego, regardless of what others suggest, regardless of what the HMO tells me to do, my job is to take care of you

This content downloaded from
[illegible]20.126.194.245 on at, 31 ct 2026 06:54:44 UTC [illegible]
All use subject to https://about.jstor.org/terms

> 待在醫院、照護病人的經驗。我認為，到了住院醫師階段，你對工作的期待已經根深蒂固，因為你多年來一直身處這樣的環境。

相較之下，雖然基層醫療住院醫師知道 Boot Camp 聲譽卓著、訓練強度高，但在處理工作認同完整性受損時，他們並不認為這個組織是認同意義的重要來源。一段具代表性的離職訪談談到 Boot Camp 在協助他們成為醫師方面的作用：「我想，在來 Boot Camp 之前，我已經是醫師，也正朝著其他那些目標前進了

p."

外科：認同客製化類型。認同與工作之間的不契合，使外科住院醫師借助其他認同來理解自己所做的事。他們形成了兩種認同：一種是因應工作性質而形成，另一種則是因應訓練強度而形成。就工作內容而言，工作大多涉及病患照護的各個面向，住院醫師逐漸將

h

外科認同與他們原先對全科醫師意涵的看法連結起來。這種拼接形成了一種新的複合認同，住院醫師稱自己為「最全面的醫師」。「最全面」意指他們能做全科醫師（亦即基層醫療醫師）能做的事——臨床診療——此外還能進行外科手術：

> [我們是] 醫院裡最全面的醫師 l
> 就處理任何送進急診室的病症而言。（第三波訪談）

我們將這個過程稱為身分修補，因為外科住院醫師會運用一種身分

y

（一般科醫師），永久填補他們對自己身為外科醫師的認知中「空缺」或不足之處；這些認知源自他們在工作中執行的任務。然而，在住院醫師訓練期間，他們得以從「最全面的醫師」這一自我概念出發，進一步建立更深入、細緻的身分認同。因此，任期結束時，外科住院醫師已從身分修補轉向

n

身分充實的過程。舉例來說，請留意表 1 最後一欄對「最全面的醫師」的描述，如何納入更精

s

巧的新隱喻（例如軍械庫和武器庫），以及如何傳達出一種日益增長的體認：與外科醫師相比，其他醫師有多麼不全面

h

surg萬古歲月。

針對訓練營高強度工作流程，也出現了類似的身分修補現象。外科住院醫師覺得自己工作的

比醫院裡其他住院醫師更為辛苦，並且 d
比其他醫療中心的外科住院醫師更辛苦 l
中心：

> 我認為大多數接受醫學訓練的人，在受訓期間都過度勞累。我覺得外科尤其糟糕。我想 Boot Camp 對外科而言可能特別糟糕。……我覺得 Boot Camp 相當糟糕。例如，我的未婚夫是 Prestigious Hospital 的外科實習醫師，而他工作的時數比我少得多。

為了理解這種情況，外科住院醫師改變了對自身身分的概念，以配合他們工作的方式，也就是「Boot Camp 方式」。如表 1 所述，「Boot Camp 方式就是總是設法讓自己看起來最好之類的，並且盡量不要讓別人看出你這一週少睡了 30 個小時。」住院醫師在融入這些觀念時，將外科醫師身分與 Boot Camp 身分結合，成為 Boot Camp 實習醫師（第一年住院醫師）。正如一名住院醫師在第一年結束時所說：「我想在最初三個月內，你

pretty much g成為一名訓練營 p 實習醫師。"

如同最完整的醫師身分認同，訓練營實習醫師的身分認同也隨著時間變得更加豐富，逐漸發展成為「訓練營外科醫師」。如表 1 所示，訓練營外科醫師認為自己有別於其他外科醫師；此外，與「

g更廣大的社群 y of g一般外科醫師g」認同。

雖然我們將這兩種「拼湊」視為對工作類型與工作流程強度的兩種截然不同的回應，但值得注意的是，離職訪談中對「最全面的醫師」的描述也包含戰爭意象（例如「兵工庫」；表 1）。這顯示，這兩種身分建構過程或許有所重疊。

外科：身分組合。外科住院醫師在打造自己的專業身分時，運用了各種不同的資源。儘管他們對「最全面的醫師」一詞的使用與認同似乎在訓練營任職期間逐漸增加，但他們最初是在醫學院接觸到這個概念。此外，這種身分中的全科醫師特質似乎在醫學院時期便已內化：

> 對我而言，成為一名醫師，意味著以病人為優先，也就是說，如果你生病了，我的工作就是幫助你，盡我所能讓你康復。這表示我必須運用在醫學院所學的知識，以及住院醫師訓練期間累積的經驗，把你的需求放在第一位，不受自我意識、他人建議或 HMO 指示的影響。我的工作就是照顧你。

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從 [illegible]20.126.194.245 下載 [illegible]所有使用均須遵守 https://about.jstor.org/terms

and that's what I promised to do when I graduated from medical school. That's what being a physician means. (wave 1 interview)

By contrast, to build the Boot Camp intern and surgeon identities, surgical residents drew upon organizational stories and artifacts. First, stories played a role in these surgical residents' understanding of what it meant to be in this organization and what it meant to be working so intensely. Even before they entered Boot Camp, residents reported stories revolving around the intensity of training and unique pressures faced by Boot Camp surgeons:

> I heard that there was a 100 percent divorce rate—that no marriage has ever survived this program. I heard that you were not allowed to drink Coke in the hallways because it was unprofessional. I heard just that it was brutal. It used to be every other night on-call for five years with no vacations. (wave 1 interview)

> General surgery is just malignant and there are some services where not only are you working hours and hours and never getting any sleep [but] you're treated like crap. (wave 1 interview)

Second, physical artifacts—especially white uniform pants—were important for distinguishing first-year surgical residents from all other interns and in constructing their professional identity. As James (1890) noted more than a century ago, one's clothes and other possessions are often an integral part of identity. Dress is an especially potent component of identity among physicians and other medical professionals (Becker et al., 1961; Pratt & Rafaeli, 1997). It used to be common for all medical residents to wear white pants and a white lab coat during the first year of residency. Although many graduate medical education centers have retained the lab coats, the bottom half of the "white suit" has been largely abandoned. The Boot Camp surgery department retains this tradition, however, and reminds residents how Boot Camp is different:

> Not only do I not mind dressing up in this white suit but I take it as a big source of pride . . . I know very well who has worn this uniform over time. . . .This is a great tradition and so I mean these giants were not too good for this you know and I am sure not. So, part of it is that I am a bit of a traditionalist in that I tend to think of our move toward being overly casual is being somewhat of an excessive move in that direction. In mean, in thinking and in personal discipline and all these kinds of things. (wave 1 interview)

Wearing the white pants further strengthened the "commando" image of Boot Camp, which helped contribute to the respondents' Boot Camp intern identities:

> The Boot Camp "commando" attitude . . . yes it's still that way . . . I have to put my whites back on to come outside the OR [operating room] for 10 minutes and go back . . . (wave 1 interview)

Radiology: Customization types. To deal with the lack of integrity between the content of the work (e.g., studying; see Table 1) and professional identity, the radiology resident went through a two-stage identity customization process. At the beginning of the program, work-identity integrity was lacking because radiology residents were not actually practicing radiology. Although radiologists were supposed to read films and other images for real patients, these first-year residents spent their time reading, studying, and attending daily teaching conferences. Unable to align their work with what it meant to be radiologists, residents adopted a prior identity: that of student. These quotations from two radiology residents' midpoint interviews are exemplary: "You really feel like you're in medical school all over again" and "This year in training it's almost like being a student again."

The prior student identity bolstered the weakly defined radiologist identity by providing residents with a temporary identity to use until the radiologist identity developed and became stronger. We refer to this identity customization process as *identity splinting*. Like a weakened bone, radiology residents' sense of self at the start of the year failed to support them; it lacked sufficient explanatory power to make sense of the work they were doing. As a result, the residents temporarily used the prior student identity as a splint, protecting the fragile radiology identity that they retained alongside the student identity. Further, as the radiologist identity became stronger, the student identity—like a real splint—was cast aside.

The radiologist identity was strengthened as the residents' work changed and as their knowledge bases grew. Strengthening began during their eighth month of training, when the residents started to be assigned to baby call (when, as previously noted, the residents began to assist senior residents during evenings and weekends):

> You just don't know enough to contribute a lot and when you start taking call, you get a lot more experience with just seeing a lot more films but you also feel like you are part of the program. You're involved. (wave 3)

As these residents began to carry out the tasks of their specialty, they started to flesh out their views of what it meant to be radiologists. That is, they

This content downloaded from
120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

而這正是我從醫學院畢業時承諾要做的事。這就是成為一名醫師的意義，（第一波訪談）

相較之下，為了建立新兵訓練營實習醫師與外科醫師的身分，外科住院醫師借助了 n
組織故事與文物。首先，故事在這些外科住院醫師理解身處這個組織的意義，以及如此高強度工作的意義上，發揮了作用。Eve
n
在進入訓練營之前，住院醫師表示 d
圍繞訓練強度及訓練營外科醫師所面臨獨特壓力的故事：

> 我聽說離婚率是百分之百？沒有一段婚姻能撐過這個計畫。我還聽說，因為不夠專業，在走廊上不能喝可樂。我只聽說這裡非常艱苦。過去五年裡，每隔一晚就得值班，而且沒有休假，（第一波訪談）

> 一般外科的工作環境簡直有毒，有些科別不只讓你工作好幾個小時、完全睡不到覺，[而且] 還把你當垃圾一樣對待，（第一波訪談）

其次，實體物件——尤其是白色制服長褲——對於區分外科住院醫師第一年與其他所有實習醫師，以及建立他們的專業認同，都很重要。正如 James（1890）在一百多年前指出的，衣著和其他個人物品往往是認同不可或缺的一部分。服裝是醫師和其他醫療專業人員認同中特別有力的組成要素（Becker et al., 1961; Pratt & Rafaeli, 1997）。過去，所有住院醫師在住院醫師訓練第一年穿白褲和白袍是很常見的。許多研究所醫學教育中心雖然保留了白袍，但「白色制服」的下半身大多已被淘汰。不過，Boot Camp 外科部門仍保留這項傳統，並提醒住院醫師 Boot Camp 有何不同：

> 我不但不介意穿上這套白制服，反而引以為傲……我很清楚，歷來有誰穿過這套制服……這是一項偉大的傳統，所以我的意思是，這些傑出人物可不是隨便什麼人都能比得上的，我敢肯定。這也是為什麼我有點傳統派，因為我認為我們逐漸變得過於隨便，某種程度上是朝這個方向走得太遠了。我的意思是，在思想、個人紀律以及各種這類事情上，（第一波訪談）

穿著白色長褲進一步強化了訓練營的「突擊隊員」形象，這有助於

形塑受訪者的訓練營實習生身分：

> 新兵訓練營式的「突擊隊員」態度……對，現在還是這樣……我得重新穿上白袍，才能走出手術室 [operating room]，待 10 分鐘再回去……（第一波訪談）

放射科：客製化類型。為了處理工作內容（例如研讀；見表 1）與專業身分之間缺乏一致性的問題，放射科住院醫師經歷了兩階段的身分客製化過程。在訓練計畫開始時，由於放射科住院醫師實際上尚未從事放射科工作，工作與身分之間缺乏一致性。雖然放射科醫師應該為真實病人判讀 X 光片及其他影像，但這些第一年住院醫師把時間花在閱讀、研讀，以及參加每日的教學會議上。由於無法將工作與成為放射科醫師的意義連結起來，住院醫師採用了先前的身分：學生。以下兩段引自兩位放射科住院醫師期中訪談的話，便是很好的例子：「你真的會覺得自己又回到醫學院了」以及「今年在訓練中 i

ng it's almos一如e bei同一個tudent again."

過去的學生身分強化了尚未明確成形的放射科醫師身分，為住院醫師提供一個暫時的身分，讓他們在放射科醫師身分逐漸發展並變得更加穩固之前得以依憑。我們將這種身分調整的過程稱為「身分夾板」。就像受損的骨骼一樣，放射科住院醫師在年度訓練開始時的自我認同無法支撐他們；這種認同缺乏足夠的解釋力，無法讓他們理解自己所做的工作。因此，住院醫師暫時以過去的學生身分作為夾板，保護他們在學生身分之外仍保留著的脆弱放射科身分。此外，隨著放射科醫師身分逐漸穩固，學生身分也像真正的

P夾板一樣被丟到一旁。隨著住院醫師的工作內容改變、知識基礎日益深厚，放射科醫師身分也隨之強化。這種強化始於訓練的第八個月，當時住院醫師開始被安排值「初階班」（如前所述，此時住院醫師開始在晚間和週末協助資深住院醫師）：

> 你就是還不夠懂，無法做出太多貢獻；開始值班後，看片的經驗會豐富許多，也會看到更多影像，但你也會覺得自己是計畫的一分子。你有參與其中。（第 3 波）

隨著這些住院醫師開始執行專科工作，他們也逐漸具體描繪出自己對放射科醫師意義的看法。也就是說，他們

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從120.126.194.245 下載 所有使用均須遵守 https://about.jstor.org/terms

moved beyond understanding the job as involving "thinking and a small degree of patient contact." They began to more fully understand their identity as a "doctor's doctor" or a doctor's consultant, marking a shift away from identity splinting. By wave 3, we saw these physicians adopting a professional identity centered on this idea of being a consultant instead of a professional identity centered on being a student. Unlike the responses from wave 1, the wave 3 responses have few or no qualifiers. One wave 3 radiological resident simply said, "I feel like a consultant." Another explained:

> I think we are consultants to every other specialty and that's what it means to me to be a radiologist—the fact that we get to consult with neurosurgery, with general surgery, with medicine and family practice. . . . To be a radiologist, I think that we are diagnosticians number 1—we don't treat patients per se for the most part—and [we are] able to add a diagnosis or two.

This conceptualization of radiologist as consultant deepens over time. Thus, during their exit interviews, radiology residents were exhibiting *identity enrichment*. Note how easily the following quotation distinguishes radiologists from doctors who "make people feel better":

> For me, the benefit of being a physician as a radiologist is that is what we do all day long—we are not actually making anyone feel better. We are actually trying to figure out what is wrong with them, and then other people actually do the work to make them feel better.

Still another resident discussed in the exit interview how a radiologist must be able to take others' perspectives and measure them against his/her own knowledge.

> No matter how much the referring doctors think they know about radiology, you know more than them. You try to be helpful in seeing their point of view and realize how or what you say is going to affect that doctor and his/her patient. If you are able to put all that together and keep it all in mind, you will be a good radiologist. A radiologist is not just someone who can read films, but be a valuable consultant to the referring physicians.

It is also interesting to note that radiologists do not revive their student identities during their last year of residency, even though this year is marked by intensive study for radiology boards (the professional certification examination; see Table 1). Thus, it is clear that by this point the radiology identity is strong enough that residents no longer need the splint provided by this student identity.

Radiology: Identity set. As noted, radiology residents explicitly referred to medical school and their identities as students during their initial attempts to create an identity splint to protect their fragile and underdeveloped radiologist identities. This student identity was reinforced by the language used by the chair of the department (who referred to them as students during their orientation), and it was reinforced by the work assigned: reading, attending lectures, taking written examinations. In a similar way, the consultant identity label was also learned in medical school, but reinforced in how radiology residents referred to each other on the unit. As the chair of radiology told them on their first day, "One of the things about this program that may be different from some other programs . . . is that this is a very clinically oriented program and we work a lot with the clinicians. . . . We like to see ourselves as part of the team."

What is perhaps as interesting as the identity set for radiology residents is the fact that they rejected many of the identity sources used by other physicians. For example, unlike the primary care and surgical residents, the radiologists did not see the general medical identity learned in medical school as relevant for explaining the type of work they did or for defining professional identity. As one radiology resident explained during his first year:

> The whole approach to things seems different in a sense that you go into radiology and take a step back—you are not directly involved with patient care and I knew that going into it but it feels different to finally get there and sort of give up that whole aspect that most of medical school gears you towards. (wave 2)

Another resident, in the exit interview, was more pointed about the disconnect between being a medical doctor and being a radiologist: "I realize how much they mislead you into thinking that radiology is—that it is a lot of medicine."

It also appeared that Boot Camp had relatively little impact on radiology residents' identity customization. Before entering, radiology residents (like primary care and surgical residents) saw Boot Camp as very tough and demanding. As one radiology resident said, "This is a very competitive place, they work their residents to death." This identity, however, was not in agreement with how these residents viewed their workload. That is, unlike the surgical residents, the radiological residents did not need Boot Camp to explain the intensity of work. Statements such as "I am actually working less than I thought I would be," and "I

This content downloaded from
120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

不再只把這份工作理解為涉及
「思考以及少量的病人接觸。 "
他們開始更充分地理解自己作為「醫師的醫師」或醫師顧問的身分，
m標誌著他們逐漸擺脫身分分裂。B y
w在第 3 波中，我們看到這些醫師採納了一種以成為專 o
業身分核心的理念，即成為 a
顧問，而非以學生身分為核心的專業認同。與來自

w第一波的回應不同，第三波的回應很少帶有修飾語，甚至完全沒有 l
詞 說÷位第三波的放射科住院醫師簡潔地
"我覺得自己像個顧問。 " 另一位則解釋道：
我認為我們是其他所有專科的顧問，而這對我來說就是放射科醫師的意義。 ?
我們能與神經外科、一般外科、內科和家醫科進行會診，這點很重要。……我認為，放射科醫師首先是診斷專家？我們大多不會直接治療病人？而且 [我們能] 多提供一兩項診斷。

這種將放射科醫師視為顧 l
問的觀念會隨時間逐漸深化。因此，在離職訪談中，
放射科住院醫師展現出 g
身分認同的豐富化。請注意，以下引述多麼輕易地將放射科醫師與「讓人感覺好一點」的醫師區分開來：

對我來說，身為放射科醫師而成為醫師的好處是 l
放射科醫師的工作就是整天做這些事嗎？我們其實沒有讓任何人感覺好一點。我們實際上是在找出他們哪裡出了問題，接著由其他人負責讓他們感覺好一點。

另一位住院醫師在離職訪談中談到，放射科醫師必須能夠接納他人的 ,
觀點，並以自己的知識加以衡量。 r
自己的知識。

不論轉診醫師認為自己對放射學了解多少，你都比他們懂得更多。你會試著理解他們的觀點，並明白你的言行將如何影響那位醫師及其病人。若你能把這些都融會貫通並牢記在心，就會成為一位出色的放射科醫師。放射科醫師不只是能判讀影像的人，也應該是轉診醫師值得信賴的顧問。

另外值得注意的是，放射科醫師在住院醫師訓練的最後一年並未重新拾回學生身分，儘管這一年以……為標誌 d
透過密集準備放射科專科醫師考試（專業認證考試；見表 1）。因此，很明顯到了這個階段，放射科身分已經足夠穩固，住院醫師不再需要學生身分所提供的支撐。

放射科：身分組合。如前所述，放射科住院醫師明確提到醫學院 d
他們在最初嘗試建立身分夾板、保護脆弱且尚未發展成熟的放射科醫師身分時，仍以學生自居。系主任使用的語言也強化了這種學生身分（他在迎新時稱他們為學生

a
），指派給他們的工作也強化了這種身分， :
例如閱讀、出席講座、參加筆試。類似地，「顧問」這個身分標籤也是在醫學院習得的，但放射科住院醫師在院內彼此稱呼的方式，則進一步強化了這個身分。放射科主任在他們報到第一天告訴他們：「這個訓練計畫有一點可能和其他計畫不同，那就是我們非常重視臨床， t
而且我們和臨床醫師有很多合作…… i
我們喜歡把自己視為團隊的一員。」

或許，和放射科住院醫師所建立的身分同樣值得探討的是，他們拒絕了其他醫師所採用的許多身分來源。舉例來說，與基層照護及外科住院醫師不同，放射科醫師並不認為在醫學院習得的一般醫學身分，與說明他們所從事的工作類型或界定其專業身分有關。正如一位放射科住院醫師在第一年時所解釋：

整體而言，處理事情的方式似乎有所不同，因為你進入放射科後會退後一步？你不再直接參與病人照護，而我在選擇這條路時就知道這一點，但真正到達那裡、某種程度上放棄醫學院大多數訓練引導你走向的那個面向時，感覺還是不同。（第 2 波）

另一位住院醫師在離職訪談中，更直接地指出成為內科醫師與成為放射科醫師之間的脫節：「我意識到，他們讓你誤以為放射科就是？放射科包含大量醫學。」

此外，Boot Camp 似乎對放射科住院醫師的身分認同調適影響不大。入訓前，放射科住院醫師（如同基層照護科與外科住院醫師）都認為 Boot Camp 非常艱辛、要求很高。一位放射科住院醫師說：「這裡競爭非常激烈，他們把住院醫師操得精疲力盡。」然而，這種認知與這些住院醫師對自身工作量的看法並不一致。也就是說，不同於外科住院醫師，放射科住院醫師不需要透過 Boot Camp 來解釋工作的繁重程度。他們常說：「我實際上的工作量比原先想的少。」以及「我

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從120.126.194.245 下載 所有使用均受 https://about.jstor.org/terms 約束

really think we're spoon-fed" were common (see Table 1).

To this point, we have discussed how each of the three resident groups constructed their identities. However, common to most theories of identity is the notion that identities are socially validated (see Ashforth [2001] for a review). In the next section, we go past the initial construction processes and discuss how identities were validated through both work feedback and role models.

Validating Professional Identities

In our examination of identity customization among residents, we found that social validation took one of two forms. The first form was largely initiated by senior physicians and peers and involved validating how well a particular resident was performing as a resident. The second form of validation appeared to be initiated by the residents and involved identifying specific role models. Unlike identity customization processes, the validation processes were highly similar across the three groups of residents.

Validating via work feedback. At Boot Camp, the overriding sentiment was, "No news is good news." In general, all residents noted that that they received relatively little instruction in how to perform their work. Specifically, they noted that they were given a large number of tasks to perform during their rotations and were told to figure out how to get them done. Residents described this aspect of their training with comments such as:

> I think Boot Camp does this more than some other places but you are sort of put in a position and if you're going to [do] well, you have to figure it out for yourself because they don't tell you exactly what to do. (primary care resident, wave 3)

> You learn by just simply being immersed in it. (surgical resident, wave 3)

> Nobody trains you. They tell you what they want. They tell you what their expectations are but they don't really guide you. (radiology resident, wave 3)

When feedback was given, it often took one of two forms. First, all the residents reported getting yelled at when they made mistakes. One surgical resident referred to this as "a bite."

> They say you get a bite so that if something goes wrong, you did it wrong or they would have done it another way, they tell you about it and then you don't do it that way again. (wave 3)

Bites were common for both surgery and primary care residents. In a wave 2 interview, a primary care resident recalled a colleague's run-in with a senior physician:

> Dr. Glenn sat down next to the intern and put his arm around him and said, "You know Mrs. X's [hemato-] crit dropped three points and I hope you noticed that today and what are you going to do about it? I just wanted to let you know that if her crit drops or if she bleeds out and you don't catch it, I'll fire you. I want you to tell your [senior] resident that if that happens, I'll fire her too." He didn't laugh.

Residents were also able to assess their performance compared to their colleagues' through an active grapevine. Residents often found this informal discussion of performance by residents and attendings more useful than formal evaluations. Not only was the grapevine fast and ubiquitous, but also, it was often seen as more relevant than their performance "write-ups."[8] As one surgical resident noted in the exit interview:

> The grapevine is pretty thick. Yeah, everything spreads like wildfire. You know who is being reprimanded and who is sort of in trouble very quickly. I've never really seen such a rapid flow of news. Everyone just loves to talk about things. I think a lot of it is we're put in a situation where it may be a little stressful at times and maybe this is how people deal with it.

This information was spread primarily through stories. As one primary care resident said at wave 3, "You hear stories, stories about attendings yelling at interns and interns screwing things up." Another resident (in radiology, at a wave 3 interview) noted the importance of the grapevine in disseminating information about class rankings: "The people in the bottom third of the class, everyone knows who that is because the walls speak around here. . . . These people are talked about behind their backs all the time whether it's by me or not. You hear it."

We argue that feedback, through "bites" and

[8] None of the surgical residents reported reading their formal evaluations, and some did not even know that they had access to them. And, although residents in primary care were required to read these formal evaluations during semiannual meetings with the program director, they reported discounting them because (1) they spent very little time working directly with the evaluator or (2) because they were unable to interpret the numerical evaluations in a meaningful manner, given that they did not know the numbers given to others. Finally, radiology residents found the evaluations to be somewhat useless as they were provided after a rotation and were therefore of little use in addressing any performance deficiencies they may have occurred in that rotation.

This content downloaded from 120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

真的覺得我們根本是被餵到嘴邊的」（見表 1）

。到目前為止，我們已討論三個住院醫師群體各自如何建構其身分。然而，大多數身分理論的共同觀點是，身分會受到社會驗證（參見 Ashforth [2001] 的評論）。在下一節中，我們將超越最初的建構過程，討論身分如何透過工作回饋與角色模範獲得驗證。

驗證專業身分

在檢視住院醫師的身分客製化時，我們發現社會驗證採取兩種形式。第一種形式主要由資深醫師與同儕發起，著重於驗證特定住院醫師以住院醫師身分執行職務的表現。第二種驗證形式似乎由住院醫師發起，涉及辨識特定的角色模範。不同於身分客製化過程，這些驗證過程在三個住院醫師群體之間高度相似

group住民的身份。

透過工作回饋來驗證。在 Boot Camp，最普遍的看法是：「沒有消息就是好消息。」整體而言，所有住院醫師都表示，他們很少接受如何執行工作的指導。具體來說，他們提到，輪訓期間會被交付大量任務，並被告知要自己想辦法完成。住院醫師以以下說法描述訓練中的這個部分：

> 我覺得 Boot Camp 比其他一些地方更常這樣，但你就是會被放在某個位置上；如果你想要[做] 得好，就得自己摸索，因為他們不會明確告訴你該怎麼做。（基層醫療住院醫師，第 3 波）

> 你只是全心投入其中，自然而然就學會了。（外科住院醫師，第 3 波）

> 沒有人會訓練你。他們會告訴你想要什麼，也會說明對你的期待，但不會真正引導你。（放射科住院醫師，第 3 波）

提供回饋時，通常有兩種方式。首先，所有住院醫師都表示，犯錯時會被大聲斥責。一位外科住院醫師稱之為「挨一下」。

> 他們會說，你會挨一頓罵，這樣要是出了什麼差錯，就是你做錯了，或者他們本來會用別的方法做；他們會告訴你，之後你就不會再那樣做了。（第 3 波）

外科與基層醫療住院醫師都常遭遇咬傷。在第二波訪談中，一名基層醫療

住院醫師回憶起一名同事與資深醫師發生衝突的經歷：

> Glenn 醫師坐到實習醫師旁邊，摟著他的肩膀說：「你知道 X 太太的[血比容] 下降了三個百分點，我希望你今天有注意到。你打算怎麼處理？我只是想讓你知道，要是她的比容下降，或是她大量出血而你沒有發現，我就開除你。我希望你告訴你的[資深] 住院醫師，要是發生這種事，
>
> ’我也會開除她。」他沒有笑。

住院醫師也能透過活躍的人際傳聞網絡，評估自己與同事相比的表現。住院醫師常覺得，這種住院醫師與主治醫師之間非正式的表現討論，比正式評量更有用。傳聞網絡不僅傳得快、無所不在，也常被認為比他們的表現「書面評語」更切合實際。[8] 一位外科住院醫師在離職訪談中指出：

> 小道消息傳得相當快。沒錯，所有事情都像野火一樣迅速傳開。你很快就會知道誰受到斥責、誰似乎惹上了麻煩。我從沒見過消息流通得這麼快。大家就是喜歡談論各種事情。我想，這其中很大一部分原因是，我們有時會被置於有點令人感到壓力的情境中，而這或許就是人們應對壓力的方式。

這些資訊主要透過傳聞散播。正如一名基層醫療住院醫師在第 3 波所說：「你會聽到各種傳聞，像是主治醫師對實習醫師大吼，以及實習醫師把事情搞砸的傳聞。」另一名住院醫師（任職於放射科，在第 3 波訪談中）指出了小道消息在傳播班級排名資訊方面的重要性：「班上排名倒數三分之一的人，大家都知道是誰，因為這裡的牆壁會說話……這些人一直在背後被議論，不管是不是我在談論他們。你就是會聽到。」

我們主張，回饋透過「耳語」和

8 沒有任何外科住院醫師表示自己讀過正式評量，有些人甚至不知道自己可以查看這些評量。此外，儘管基層醫療住院醫師必須在每半年與科主任會面時閱讀這些正式評量，他們卻表示不太把評量當一回事，原因是：（1）他們與評量者直接共事的時間非常少，或（2）由於不知道其他人獲得的分數，因此無法有意義地解讀這些數字評量。最後，放射科住院醫師認為這些評量多少有些無用，因為評量是在輪訓結束後才提供，因此對於處理他們在該輪訓期間可能出現的任何表現缺失，幫助不大。

此內容下載自 120.126.194.245，時間為 at, 31 ct 2026 06:54:44 UTC
所有使用均受 https://about.jstor.org/terms 條款規範

through the grapevine, helped shape physicians' identities by shaping their behavior. By learning what they, and others, were doing wrong and consequently how the work should be performed, they changed how they viewed themselves as physicians. As one surgical resident summarized in a wave 3 interview:

> This place is really a machine that takes you in, chews you up, spins you around and either spits you out or you come out top notch and I think that is true in everything. . . . That's just the Boot Camp way and I think that's what has maintained its reputation.

Validating via role models. We also found that residents received validation for their professional identities through role models. Residents clearly felt that a large portion of the medical staff at Boot Camp was highly competent; when asked about whom they personally saw as role models, they chose those physicians whose characteristics matched the perceptions of their professional identities. For example, the professional identity of a primary care physician involved taking care of both medical and nonmedical aspects of care. And a commonly mentioned role model was described as having both these characteristics:

> [Dr.] Kline is very in tune with treating the patient as a person and not just as a commodity or a case and that's really important when you realize that these patients are [not] just any sort of disease process but they are a person as a whole. Again, that also goes into the fact that being a primary care physician, you're treating them as just the whole person and not just certain organs or diseases separately (wave 3 interview).

Surgical residents came to view themselves as the most complete doctors and as Boot Camp surgeons. Not surprisingly, these characteristics were common in the role models they chose. In this wave 3 quotation, Drs. Ian and Ivan were viewed as being both surgeons *and* excellent providers of patient care:

> They [Dr. Ian and Dr. Ivan] are excellent surgeons and they are so attentive to their patients. They talk and they communicate with their patients all the time. They always give them the plan and they explain everything. They see them at least once a day, no matter what time in the night.

Similarly, some role models were chosen because they seemed to reflect the Boot Camp way of doing things. One popular role model is described below in another wave 3 interview as both highly competent and also epitomizing the commando attitude of the organization:

> He is a little bit outspoken, little arrogant, very friendly. I've never worked with him in the OR [operating room] but I hear he's just a total asshole. I hear that you just shake. You screw up. . . . You cut wrong or nothing that's going to hurt the patient but just little technique things. I hear that he's just a total asshole. It's like Jekyll and Hyde—because every time I have interacted with him, he's incredible. He's just been a great teacher, very friendly and he's also a team player and protects his residents.

And some role models appeared to be both "complete" and "Boot Camp" doctors:

> He is an intern/resident/chief resident/fellow and attending wrapped into one. He comes in in the morning and rounds with us and basically his actions . . . he doesn't say this but his actions are saying . . . "I don't need any of you, I can do it all myself. I can take care of my own patients myself. I don't need [you] to pre-op my patients. I don't need you to post-op my patients. I don't need you to do any of the scut work. You're more than welcome to come along with me and I will teach you everything I know but I don't need you." (wave 3 interview)

Finally, radiology residents viewed themselves as consultants, as "doctors' doctors" who provide information to physicians, unlike other physicians, who only provide information to patients. These characteristics were also central in the role models they chose. A quotation from an exit interview illustrates:

> Dr. Smith, is somebody who I believe does a good job, you know, kind of, first being a radiologist and second knowing how to communicate well with other physicians and technologists.

Correlates of Identity Customization and Validation

We end our description of identity customization by assessing the impact of identity changes on other outcomes. Although our research design was not well suited for assessing causality, we were interested in understanding what other changes in physicians covaried with their identity customization and validation. Our data suggest at least two additional changes in resident attitudes: (1) they communicated a higher degree of perceived competence and (2) they exhibited varying patterns of identification with their profession and their organization.

Perceived competence. We noticed that just as work and identity changed over time, so also did members' perceptions of their own competence. Table 2 presents data illustrating changes in perceived competence at two different points in the course of residency. For each group, the second

This content downloaded from 120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

道聽塗說，藉由形塑醫師的行為，幫助塑造了他們的身分認同。透過了解自己和他人做錯了什麼，進而明白工作應如何進行，他們改變了看待自己作為醫師的方式。正如一位外科住院醫師所總結的

a

第三波訪談：

> 這個地方真的就像一台機器，把你吸進來、嚼碎、轉來轉去，最後不是把你吐出去，就是讓你成為頂尖人才，而我認為一切事情都是如此。……那就是訓練營的方式，我想這正是它能維持聲譽的原因。

透過角色楷模獲得確認。我們也發現，住院醫師透過角色楷模，為自己的專業身分獲得了確認。住院醫師明確感受到，訓練營中有很大一部分醫療人員都具備高度專業能力；當被問到他們個人視誰為角色楷模時，他們選擇的是那些特質符合其專業身分認知的醫師。例如，初級照護醫師的專業身分包含同時照顧病患醫療與非醫療層面的需求。而一位經常被提及的角色楷模，便被描述為同時具備以下兩項特質：

> [Kline 醫師非常能體會要把病人當作一個人來治療，而不只是把他們當成商品或病例；當你意識到這些病人[不] 只是某種疾病過程，而是完整的一個人時，這一點真的很重要。再者，這也與身為初級照護醫師的事實有關：你是把他們當作完整的人來治療，而不是分別只處理某些器官或疾病（第三波訪談）。

外科住院醫師逐漸認為自己是最全面的醫師，也是訓練營式外科醫師。他們選擇的典範人物也普遍具備這些特質，這並不令人意外。在這段第三波訪談引言中，Ian 醫師和 Ivan 醫師被視為兼具外科醫師的專業，以及提供優質病患照護的能力：

> 他們 [Ian 醫師和 Ivan 醫師] 是很優秀的外科醫師，也非常關心病患。他們總是不斷與病患交談、溝通，隨時告訴病患治療計畫，並解釋所有事情。無論是什麼時候，他們每天至少都會去看病患一次。

同樣地，有些榜樣之所以被選中，是因為他們似乎展現了訓練營的行事方式。以下是第三波訪談中對一位廣受歡迎的榜樣的描述：他不僅能力高超，也充分體現了該組織的突擊隊員態度：

> 他有一點直言不諱，也有一點傲慢，但非常友善。我從沒和他一起在手術室[operating room] 工作過，但聽說他簡直是個十足的混蛋。聽說你只要一出錯……你切錯地方，或者做了些不會傷害病人、只是技術上的小失誤，我聽說他簡直是個十足的混蛋。這就像傑基爾與海德？因為每次我和他互動時，他都令人難以置信。他一直是位很棒的老師，非常友善，而且也是團隊成員，會保護他的住院醫師。

有些典範人物似乎同時是「全面型」和「訓練營式」醫師：

> 他就像把實習醫師、住院醫師、總住院醫師、研究醫師和主治醫師集於一身。早上他會來跟我們一起查房，基本上，他的行動……他不會明說，但他的行動彷彿在說……「我不需要你們任何人，我自己什麼都能做。我可以自己照顧自己的病患。我不需要[你們] 替我的病患做術前準備，也不需要你們照顧我的病患術後復原。我不需要你們做任何繁瑣的雜事。你們很歡迎跟著我，我也會把我知道的一切都教給你們，但我不需要你們。」（第三波訪談）

最後，放射科住院醫師將自己視為顧問，也就是向醫師提供資訊的「醫師的醫師」；相較之下，其他醫師只向病人提供資訊。這些特質也是他們所選擇的榜樣中不可或缺的部分。以下是一段離職訪談的引述：

> Smith 醫師是我認為表現很好的人，你知道，他首先是一名放射科醫師，其次也很懂得如何與其他醫師及技術人員良好溝通。

身分客製化與驗證的相關因素

我們在評估身分變化對其他結果的影響後，結束對身分客製化的描述。雖然我們的研究設計並不適合評估因果關係，但我們有興趣了解，醫師身上還有哪些變化會與其身分客製化及驗證共同變動。我們的資料至少顯示住院醫師的態度還有兩項變化：(1) 他們表達出更高程度的自我感知能力，以及 (2) 他們對自身專業及其組

g組織。

感知到的能力。我們注意到，正如工作與身分認同會隨時間改變，成員對自身能力的看法也同樣會改變。表 2 呈現了住院醫師訓練過程中兩個不同時間點的感知能力變化資料。對每個群組而言，第二個

此內容下載自120126.194.245，時間為 2026 年 31 ct 06:54:44 UTC
所有使用均受 https://about.jstor.org/terms 的條款約束

TABLE 2
Changes in Perceived Competence

Program	Early Perceptions	Later Perceptions
Primary care	"That's one of the things as a physician that you need to be aware of or how people are going to take care of themselves not just when you give out their drugs or fix their knee. In a way it's certain anticipation and it's a certain awareness that you need of the whole dynamics of their family—of their background, of their education—and that is something I think that in the internship year it is hard to do sometimes . . . I think the internship year is tough when it comes to that." (wave 2 interview)	"[I] kind of know what to do without actually having to think about it. I know what to do in most situations. I know how to take care of sick people." (wave 3 interview)
Surgery	"As much as we learned in medical school, we really don't know anything." (wave 2 interview)	"Well you go from knowing nothing hopefully to having confidence in approaching mastery in certain areas." (exit interview)
Radiology	"It's obviously a little bit unnerving because you will be acting more independently [in the future], . . . I think that is the main problem. We really have no application yet for our knowledge." (wave 3 interview)	"You are using your knowledge to sort of bridge a gap between an internist, a surgeon, an obstetrician, and the patient, so you are sort of being the problem solver in many cases." (exit interview)

comparison point was determined by when they first discussed feeling competent as primary care, surgery, and radiology residents.

Interestingly, the timing of competence assessments demonstrated in Table 2 overlaps with the onset of identity enrichment strategies. This parallel suggests feelings of competence occur when one's identity beliefs are relatively stable and are undergoing nuanced changes. Our data further suggest that perceived competence is also related to the learning of work. This quote from a surgery resident's exit interview illustrates:

> You learn the most about surgery by doing surgery, there's no replacement for it. You can watch. Once you have achieved a certain level of confidence and you are approaching mastery, seeing stuff becomes more useful. . . . You know exactly what you're seeing, you can visualize that as being a deviation from what you might usually do. You've already got your way, now you're seeing somebody else's way that you might want to incorporate.

As we discuss below, we believe that learning about work and identity are interrelated.

Patterns of identification. At their exit interviews, we asked residents to rank-order items in terms of how important they were to their identities, with 1 being the most important and higher numbers denoting less importance. These items included being a "physician," being a "primary care physician/surgeon/radiologist," being a "Boot Camp graduate," being a "Boot Camp physician," and being "a Boot Camp primary care physician/surgeon/radiologist." Both the primary care and radiology residents saw being a physician/radiologist as being most important to their identities; the average rank given was 1.2 for primary care residents and 1.8 for radiology residents. Boot Camp was seen as relatively unimportant. Two statements illustrate:

> I enjoy my time at Boot Camp and I'm glad I came here. If I carry anything with me I'd say that I'm a [medical school] grad. I'm not really faithful to Boot Camp per se. (primary care resident, exit interview)

> It definitely means something . . . there are obviously people that trained at Boot Camp, that are Boot Camp–trained physicians. . . . I wouldn't necessarily be glad to group myself in with them. (radiology resident, exit interview)

Surgical residents, however, ranked being a Boot Camp surgeon the highest of all the work identities, assigning it an average rank of 1.5. We suggest that this difference between this group and the other two resident groups stemmed directly from the surgeons' identity customization process. Being a member of Boot Camp helped surgical residents make sense of how work was done, and thus the organization become incorporated into professional identity (see Table 1). As noted earlier, the primary care and radiology residents did not need such explanations to make sense of their work.

This content downloaded from
[illegible]20.126.194.245 on at, 31 ct 2026 06:54:44 UTC [illegible]
All use subject to https://about.jstor.org/terms

表2 感知能力的變化

計畫	早期認知	後期認知
初級照護	「這是身為醫師需要 注意的事情之一，也就是人們將如何照顧 自己，而不只是當你開藥給他們 或治好他們的膝蓋。某種程度上，這是一種特定的 預期，也是一種特定的覺察，你 了解他們整個家庭的動態？ 了解他們的背景、教育程度？而這是 我認為在實習年很難做到的事 有時候很難做到……我認為實習 年在這方面確實很辛苦。」（第二波 訪談）	「[I] 我大概知道該怎麼做，不必真的 仔細思考。我知道大多數情況下該怎麼做， 也知道如何照顧生病的人。」 （第三波訪談）
Surgery	「我們在醫學院學到的東西再多，實際上還是 什麼都不懂。」（第二波訪談）	「嗯，你會從一無所知，希望逐漸變成 對精進之路抱持信心 的狀態。」（離職訪談）
Radiology	「這顯然會讓人有點忐忑不安，因為你將會 在未來[更加獨立地]行動……我 我認為那就是主要問題。我們確實還沒有 將我們的知識應用於實務。』（第 3 波 訪談）	「你運用自己的知識，某種程度上彌合了 內科醫師、外科醫師、 產科醫師與病人之間的差距，所以你某種程度上 在許多情況下成為解決問題的人。」（離開 面談）

比較點是依據他們首次談到自己作為基層照護醫師感到勝任的時間所決定，

gery，以及放射科 gy 住院醫師。

有趣的是，表 2 所呈現的能力評估時機，與身分豐富化策略的開始時間有所重疊。這種平行關係顯示，能力感會在人們的身分信念相對穩定、同時正經歷細微變化時出現。我們的資料進一步顯示，感知到的能力也與工作學習有關。以下這段外科住院醫師離開面談的引述說明了這一點：

> 你透過實際做手術，能學到最多關於手術的知識，沒有任何東西可以取代它。你可以觀摩。當你達到一定程度的信心、接近精通境界時，觀看就會變得更有用……你很清楚自己看到的是什麼，也能將其視為偏離自己通常做法的地方。你已經有了自己的方式，現在看到的是別人的方式，而你可能會想把它納入自己的做法。

如以下所述，我們認為，了解工作與身分認同

У 彼此相互關聯。

認同模式。在結業訪談中，我們請住院醫師依照這些項目對其身分認同的重要程度排序，1 代表最重要，數字越大則表示重要性越低。這些項目包括成為「醫師」、成為「基層醫療醫師／外科醫師／放射科醫師」、成為「訓練營畢業生」、成為「訓練營醫師」，

以及成為「訓練營基層醫療醫師／外科醫師／放射科醫師」。基層醫療與放射科住院醫師都認為，身為醫師／放射科醫師對其身分認同最為重要；基層醫療住院醫師給予的平均排名為 1.2，放射科住院醫師則為 1.8。訓練營被視為相對不重要。以下兩段話可以說明這一點：

> 我很享受在訓練營的時光，也很高興自己來到這裡。如果要說我帶走了什麼，我會說，我是一名[醫學院] 畢業生。我並不算真正忠於訓練營本身。（初級照護住院醫師，離站訪談）

> 這確實代表某種意義……顯然有些人曾在訓練營受訓，是接受過訓練營訓練的醫師。……我不一定會很樂意把自己歸入他們之中。（放射科住院醫師，離站訪談）

然而，外科住院醫師將成為 Boot Camp 外科醫師評為所有工作身分中最高，平均排名為 1.5。我們認為，這一群體與另外兩組住院醫師之間的差異，直接源自外科醫師的身分客製化過程。成為 Boot Camp 的成員有助於外科住院醫師理解工作的執行方式，因此該組織也融入了他們的專業身分之中（見表 1）。如前所述，基層醫療與放射科住院醫師不需要這類解釋來理解自己的工作。

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從[illegible]20.126.194.245 下
載 [illegible]所有使用均受 https://about.jstor.org/terms 的條款約束

TOWARD A MODEL OF IDENTITY CUSTOMIZATION: THE INTERPLAY OF WORK AND IDENTITY LEARNING CYCLES

To help make sense of the various concepts and their relationships in our data, we constructed Figure 2, which both summarizes and generalizes our main findings. This figure illustrates our view that work results in work-identity integrity assessments but that such assessments actually result in two related learning cycles.

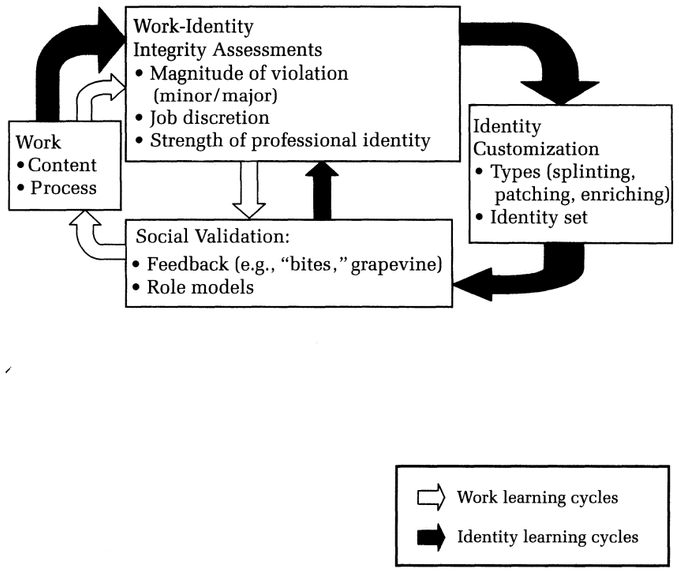
The first cycle, indicated by the white arrows in the figure, involves learning about the work itself. Here, individuals work, experience work-identity integrity violation, and receive social validation for their performance. We have argued that informal feedback—especially in the form of grapevine assessments and bites—works to remind students about their work performance. When one receives (mostly negative) feedback, one is expected to improve performance. As illustrated in Figure 2, learning about work serves as the inner core of the identity learning cycle.

Also shown in Figure 2, work and resulting work-identity integrity violations can also lead to a second cycle, indicated by the black arrows, that involves learning about one's professional identity. When who you are does not match what you do, another possible outcome is change in your sense of who you are. We have called this process identity customization.

The two cycles are inextricably linked. Both cycles begin with *work* and *assessments of work-identity integrity*. When who you are does not match what you do, you can improve your performance and/or how you view yourself. For example, challenges to become better at patient care are wrapped up in becoming "a more complete doctor." Similarly, Hall (1968, 1971, 1995) suggested that acquiring a sense of self-esteem and agency (i.e., work competency) is critical to stepping into new identities throughout people's careers. In our data, the time required to gain competence appeared to be related to the severity of the work-integrity violation. To illustrate: the primary care residents, who experienced the fewest work-identity integrity violations, reported feelings of mastery and competence at the end of their first year. But the surgical and radiology residents, who experienced much more profound integrity violations, did not make statements about perceived competence until the end of their residencies (see Table 2).

Although the two cycles are linked, as integrity violations lead to identity changes, the two cycles begin to diverge. Specifically, our data suggest that job discretion plays a major role in *how* work-iden-

FIGURE 2
Interplay of Work and Identity Learning Cycles in Professional Identity Customization



This content downloaded from 120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

邁向身分客製化模型：工作與身分學習循環的交互作用

為了協助理解資料中的各種概念及其彼此關係，我們建構了圖 2，該圖同時概括並一般化了我們的主要發現。此圖呈現了我們的觀點，即

t

工作會導致工作身分一致性評估，但這類評估實際上會導致兩種

o

相關學習 g cy循環。

第一個循環以圖中的白色箭頭表示，涉及對工作本身的學習。在此過程中，個人從事工作、經歷工作與身分完整性受到侵犯，並因其表現獲得社會認可。我們曾指出，非正式

l

回饋？尤其是透過小道消息的評價與耳語？會提醒學生

s

注意自己的工作表現。收到（大多是負面的）回饋時，個人通常被期待改善表現。如圖 2 所示，對工作的學習構成身分

y 學習 g cycle.

圖 2 也顯示，工作及其所造成的工作與身分完整性侵犯，也可能導致

a

第二個循環由黑色箭頭表示，涉及了解自己的專業身分

.

當你是誰與你所做的事不相符時，另一個可能的結果是你對自我認同的感受發生變化

。我們將這個過程稱為身分認同

y 客製化。

兩個循環密不可分。兩個循環都始於工作，以及對工作身分完整性的評估。當你是誰與你所做的事情不

t

相符時，你可以改善自己的表現，以及／或改善看待自己的方式。例如

,

面對成為更優秀病患照護者的挑戰

e

都融入成為「更完整的醫師」之中。同樣地，Hall（1968、1971、1995）指出，建立自尊與能動性的感受（即工作能力）對於踏入

o

在職涯中持續形成新的身分。在我們的資料中，獲得勝任能力所需的時間，似乎與工作完整性違規的嚴重程度有關。舉例來說：主要照顧者

e

住院醫師經歷的工作身分完整性違規最少，他們在第一年結束時表示感到已能駕馭工作並具備勝任能力。然而，外科與放射科住院醫師經歷的完整性違規更為嚴重，他們直到住院醫師訓練結束前，都沒有表示自己感受到

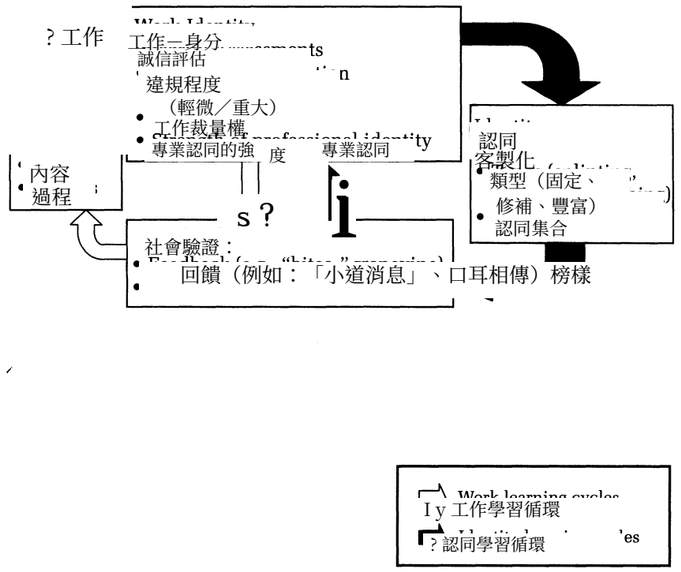
d

勝任能力（見表 2）。

雖然這兩個循環彼此相互連結，因為誠信違反會導致身分變化，但兩個循環開始出現分歧。具體而言，我們的資料顯示，工作自主裁量權在工作身分誠

圖 2 專業身分客製化中工作與身分學習循環的交互作用

1



此內容下載自120.126.194.245，時間為 at, 31 ct 2026 06:54:44 UTC
所有使用均須遵守 https://about.jstor.org/terms

tity integrity violations are resolved and that the magnitude of the violation and strength of identity play a major role in predicting *what* type of identity customization will take place.

According to Nicholson and colleagues (Nicholson, 1984; West et al., 1987), one key predictor of how integrity violations will be resolved is *job discretion*. As physicians-in-training, the residents in our study had relatively little discretion over how their work was done. As the chair of the radiology program noted, "We'll try it our way for four years. . . . You'll find a few things you don't like you can change after you're gone. While you're here, it's our way." As a result, identity is more likely to change to fit the work than vice versa.

We argue that the *magnitude* of the violation largely predicts the *type* of identity change that occurs when integrity is violated. In sensemaking terms, the lack of integrity induces "unfreezing" of an individual's current sense of self (Lewin, 1958; Schein, 1987), and a "seeking" of identity-related meaning (Lofland & Stark, 1965; Pratt, 2000). When the violations to work-identity integrity are minor, sensemaking results in identity enrichment, or a more nuanced understanding of identity. By the end of residency, all our residents experienced identity enrichment. This uniformity suggests that identity enrichment is a customization strategy common to most professional identity construction processes.

However, when violations are more major, two results are possible: identity patching and identity splinting. What appeared to account for which type of customization occurred had to do with the current *strength* of the professional identity. As noted in Table 1, surgical residents had a fairly clear (even if inaccurate) sense of what a surgeon was. As a result, they were able to use this identity as a lens for interpreting their work (Weick, 1995). Starting with this strong foundation, the surgical residents were able to add to, or patch, their existing identity. Specifically, to explain the broader range of tasks, surgical residents essentially annexed the physician identity to create the most complete doctor identity. To explain how the work was done (e.g., long hours, presence of bites), surgical residents annexed identity-related beliefs about Boot Camp.

Radiology residents, however, were initially much less sure of their identities. In Hall's (1968) terms, their sense of professional identity was somewhat crude, or "embryonic," compared with those of their surgery and primary care peers. Hall described embryonic identities as "rudimentary and ineffective, analogous to the small clublike arms and legs of a young fetus" (1968: 465). Because of the weakness in the identity, radiology residents adopted a previously learned identity to serve as a splint to protect the growing professional identity. Taken together, these observations suggest that professionals with well-developed identities who face identity-incongruent work are likely to "patch." By contrast, professionals with less-developed identities are, under the same conditions, more likely to "splint."

The differences among these resident groups suggest that the identity customization types that we have offered may be temporally ordered. Professionals who are unsure of their tasks and their identities, such as radiology residents, may begin customization through splinting. We propose that splinting therefore might occur either at the beginning of professional development, or when professionals encounter a major "boundary crossing" (Van Maanen & Schein, 1979) in their careers. This suggests that professional identity may not begin as a tabula rasa, but rather might from the beginning involve importing previous identities that serve as a temporary means of making sense of work. As professional identities begin to mature, major misalignments with work may be resolved via patching. Finally, as we saw here, enrichment may occur later in professional identity development than either splinting or patching. Thus, future research could look for a splinting-patching-enriching pattern among professionals.[9]

In addition to differing in customization types, residents also differed in their *identity sets*. In our data, the surgical residents drew upon the widest set of identity beliefs. Some were internal and were associated with prior socialization (Nicholson, 1984). Others were external and appeared to be associated with the organization's Boot Camp identity; these were central, enduring, and distinctive beliefs about the organization (Albert & Whetten, 1985). It appeared that the misalignment of the content of work and professional identity for all residents caused them to look back at prior socialization (e.g., medical school). However, when there

[9] This sequencing begs the question of why we did not find patching for our radiology residents. Patching occurs when professional identity is strong but tasks are new. We have argued that radiology residents did not have strong professional identities until after their first year. Extrapolating from our data, patching would have therefore been the most likely to occur during the radiology residents' second year, when they transitioned away from student activities to shift work and were taking full call. During this time, tasks were changing, but professional identity was fairly strong. However, as noted in our methodology, this year was not a focus of our data collection.

This content downloaded from
20.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

信違反如何獲得解決方面扮演重要角色，而違反的程度與身分認同的強度，則在預測將採取何種類型的身分客製化方面扮演重要角色

P位置。

根據 Nicholson 及其同事的研究（Nicho l son，1984；West 等人，1987），預測誠信違規將如何解決的一項關鍵因素 f 是工作裁量權。身為受訓中的醫師，住院醫師 n

我們研究中的參與者對工作方式的自主裁量權相對有限。正如放射學計畫主任所指出的：「我們會照自己的方式做四年……你會發現有些事情不喜歡，可以等你離開後再改。在你還在這裡時，就得照我們的方式。」 n

你離開後再改。在你還在這裡時，就得照我們的方式。」因此，身分更可能 e 配合工作，而非工作配合身分。

我們主張，違反的程度大致上能預測在完整性遭到侵犯時會發生何種身分變化 t 。從意義建構的角度來看，完整性缺失會使個人對自我的既有認知「解凍」（Lewin, 1958 ; ；Schein, 1987），並促使其「尋求」與身分相關的意義（Lofland & Stark, 1965; Pratt, 2000）。當工作身分完整性受到的侵犯較輕微時，意義建構會帶來身分豐富化，也就是對身分有更細緻的理解。住院醫師訓練結束時，所有受訪住院醫師都經歷了身分豐富化。這種一致性顯示，身分豐富化是多數專業身分建構過程中常見的客製化策略

n

P過程。然而，當違規情況較為嚴重時，可能產生兩種結果：身分修補與身分夾板化。究竟發生哪一種類型的身分客製化，似乎取決於當時專業身分的強度。如表 1 所示，外科住院醫師對於外科醫師是什麼樣的人，已有相當清晰的認知（即使這種認知並不準確）。因此，他們能夠運用這個身分作為理解工作的透鏡（Weick, 1995）。以這個穩固基礎為起點，外科住院醫師得以增補或修補既有身分。具體而言，為了解釋更廣泛的任務範圍，外科住院醫師基本上將醫師身分併入其中，以建立最完整的醫師身分。為了解釋工作是如何完成的（例如，長時間工作、有人遭咬傷），外科住院醫師將身分

y 與 Boot Cam 相關的信念 p.

然而，放射科住院醫師起初對自己的身分確定得多。用 Hall（1968）的話來說，相較於外科和初級照護領域的同儕，他們對專業身分的認知相當粗略，或可說仍處於「胚胎期」。Hall 將胚胎期身分描述為「粗略且無效，類似於幼胎小小的棍棒狀手臂和腿」（1968: 465）。由於身分的薄弱

，放射學

住院醫師採用先前習得的身分，作為保護其逐漸成形之專業身分的支撐。綜合來看，這些觀察顯示，專業身分發展成熟、卻面臨與身分不相符工作的專業人士，可能會採取「修補」策略。相較之下，在相同情況下，專業身分發展較不成熟的專業人士則更有可能

y 用來「夾板」p。」

這些住院醫師群體之間的差異顯示，我們提出的身分調整類型或許具有時間上的先後順序。對自己的工作任務與身分感到不確定的專業人士，例如放射科住院醫師，可能會從以「夾板」方式開始調整身分。因此，我們認為，夾板式調整可能發生在專業發展初期，或是專業人士在職涯中遭遇重大「跨界」（Van Maanen & Schein, 1979）之時。這表示，專業身分或許並非從一張白紙開始，而可能從一開始就納入既有身分，作為暫時理解工作的方式。隨著專業身分逐漸成熟，工作上的重大不一致可能透過「修補」來解決。最後，正如我們在此所見，與夾板或修補相比，「豐富化」可能發生在專業身分發展的較後期。因此，未來研究可以探討是否存在一種「夾板—修補—豐富化」的模式

g P專業人士。9

除了客製化類型不同之外，住院醫師的身分認同組合也各不相同。在我們的資料中，外科住院醫師運用的身分認同信念範圍最廣。有些信念是內在的，與過往的社會化過程相關（Nicholson, 1984）。另一些則是外在的，似乎與組織的訓練營身分認同有關；這些信念是對組織而言核心、持久且獨特的信念（Albert & Whetten, 1985）。所有住院醫師的工作內容與專業身分認同之間似乎都存在落差，因而促使他們回溯過往的社會化經驗（例如

，醫學院）。然而 ，當

9 這樣的排序引出一個問題：為什麼我們沒有在放射科住院醫師身上發現修補？當專業身分認同強烈、但工作任務尚新時，就會發生修補。我們主張，放射科住院醫師直到第一年結束後才建立起強烈的專業身分認同。根據我們的資料推論，修補最可能發生在放射科住院醫師的第二年；此時他們從學生的工作轉為輪班工作，並開始承擔完整的值班責任。在此期間，工作任務有所變化，但專業身分認同已相當穩固。然而，如研究方法一節所述，這一年並非我們資料蒐集的重點。

此內容下載自20.126.194.245，時間為 2026 年 10 月 31 日
06:54:44 UTC，所有使用均受 https://about.jstor.org/terms 條款約束

was a violation in how the work was done, or *work process,* the scope of sensemaking expanded. Thus, surgical residents complemented their prior socialization with notions of the Boot Camp commando.

Finally, we found that the work and identity learning cycles intertwined in the area of social validation. Feedback about work progress from "bites" and from the informal grapevine was common to learning about work and professional identity among the residents. In the same vein, like Ibarra (1999), we found that role models played a critical role in identity learning. However, unlike Ibarra, we did not find that junior physicians tried on and experimented with various "possible selves" based on multiple role models. Rather, it appeared that role model choice was based more on a justification or validation of an existing or emerging identity. Furthermore, in addition to low job discretion, the life-and-death nature of their work may have left these physicians less willing or able to play with their identities. In this situation, the residents may have been more satisfied with somewhat haphazard attempts (e.g., identity patching) that worked in the short run but would likely be modified in the future.

DISCUSSION

We began our article by discussing how construction of identities among professionals is often implied but rarely explicated. Our research contributes to our understanding of identity construction by describing the importance of the relationship between "doing" and "being" among professionals. When faced with a violation between their work and professional identities, physicians rectified this violation by customizing who they were to match what they did. We further identified some conditions that favor these specific identity changes; these include work discretion, violation strength, and strength of identity. In doing so, we not only build new theory, but also connect disparate pieces of the identity construction puzzle that other theories have offered and in turn offer new insights for practitioners.

Theoretical Implications

An overview of three sets of theories that touch upon the notion of identity construction introduced this article. We return to those three sets here and discuss how our study builds or extends theory in these areas as they related to identity construction.

Identity work. Our work extends and bridges various studies that comprise the literature on identity work. To our knowledge, although others have begun to outline the factors important to identity work, we are the first to detail how this process occurs among professionals. That is, we are the first to discuss specific types of customization (e.g., identity enriching, patching, and splinting); sources of customization (identity sets); and sources of validation of these identities (e.g., grapevine feedback and role models). Nevertheless, there are important similarities between this study and other treatments of identity work. For example, we see "stories" as important to identity construction, as they are the raw materials of a wide "identity set" that individuals can draw upon to construct identity (e.g., Alvesson, 1994; Sveningsson & Alvesson, 2003). Similarly, we see role models (Ibarra, 1999) as critical to the social validation of newly constructed identities (Ashforth, 2001).

More generally, we add to this work by turning scholars' attention toward the motivation for identity customization (see also Baumeister, 1998; Deaux, 1991; Epstein, 2002; Erez & Earley, 1993). In doing so, we take lessons from research on "meaningful work," according to which work is meaningful to an individual to the extent that it reflects who the individual is (Gardner, Csikszentmihalyi, & Damon, 2001; Gini, 2000; Pratt & Ashforth, 2003). And as Kahn (1990) noted, work becomes meaningful when one's "preferred self" can be expressed through one's work and through one's membership in an organization. Building on this research, we argue that achieving alignment between identity and work is a fundamental motivator in identity construction.

Careers/role transitions. Our research builds and extends research in the area of careers and role transitions in two ways. First, in discussing the correlates of the identity customization process, we complement the work of Hall (1968, 1971, 1995) by showing that work learning cycles are embedded in identity learning cycles. This is perhaps not surprising given that, during transitions in people's professional careers, learning about work is intimately associated with learning about identity. This interplay may explain the confusion that often surrounds the concept of role (Ashforth, 2001; Biddle, 1986). In his review of role theory, Biddle argued that roles may refer to behaviors, parts that people act out (social positions), or behavioral scripts (role expectations). However, by treating work behaviors and professional identities separately—rather than confounding them as roles—we could see that these various conceptualizations of role were related in the identity construction process. Our model suggests that what one does (work behaviors) is often compared with expectations

This content downloaded from
120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

工作方式有違規之處，或工作 k
過程中，意義建構的範圍擴大了。因此 ，
外科住院醫師將先前的社會化 l
與訓練營最後，我們發現，工作與認同 p 突擊隊

y

在社會認可的範疇中，工作與身分學習循環彼此交織。住院醫師從「小片段」及非正式的耳語網絡中獲得工作進度回饋，這是他們學習工作與專業身分的常見方式。同樣地，和 Ibarra（1999）一樣，我們發現楷模在身分學習中扮演關鍵角色。然而，與 Ibarra 不同的是，我們並未發現年輕醫師會嘗試

d

扮演並嘗試各種以多位楷模為基礎的「可能自我」。相反地，楷模的選擇似乎更多是為了合理化或確認既有或正在形成的身分。此外，除了工作自主權低之外

b

他們工作攸關生死的特性 k
或許也使這些醫師較不願意或無法 e
嘗試不同的身分。在這種情況下，這些 e
住院醫師或許更能接受一些多少帶有偶然性的嘗試（例如身分拼補）；這些嘗試短期內奏效，但未來很可能還會有所調整。

討論

我們在文章開頭探討了專業人士的身分建構往往隱含其中，卻鮮少明確說明。我們的研究透過描述專業人士之間「做」與「成為」兩者關係的重要性，增進了我們對身分建構的理解。當他們的工作與專業身分出現

k

不一致時，醫師會修正這種不一致 d
，調整自我認知，使之符合自己的所作所為。我們進一步找出了一些 e
有利於這類特定身分轉變的條件，包括工作自主權、不一致的強度，以及身分認同的強度。如此一來，我們 e
不僅建立了新的理論，也將其他理論所提出、彼此分散的身分建構拼圖片段串連起來，並進一步為實務工作者帶來新的洞見。

理論意涵

本文首先概述了三組探討身分建構概念的理論。在此，我們重新檢視這三組理論，並討論本研究如何在這些領域建構或延伸理論，以及這些理論如何與身分建構相關。

身分工作。我們的研究延伸並連結 s
構成身分認同工作相關文獻的各項研究

。據我們所知，儘管已有其他研究開始概述身分認同工作的重要因素，我們是第一個詳細說明這項過程如何在專業人士之間發生的研究。也就是說，我們是第一個討論特定客製化類型的研究（例如 ，
身分的充實、修補與支撐）；客製化的來源（身分組合）；以及 d
這些身分的認可來源（例如，同儕 e
回饋與角色楷模）。然而，仍有 e
本研究與其他身分工作研究之間有重要的相似之處 d
。例如，我們認為，「故事」對身分建構至關重要，因為它們是構成廣泛「身分集合」的原始素材，個人可據此建構身分（例如，Alvesson, 1994；Sveningsson &
Alvesson, 2003）。同樣地，我們認為，角色典範（Ibarra, 1999）對新
y 建構的身分認同（Ashforth ，2001）。
更廣泛而言，我們藉由引導學者關注身分客製化的動機，進一步拓展這項研究（另見 Baumeister, 1998 ;
Deaux，1991；Epstein，2002；Erez 與 Earley，1993）。I n
這麼做時，我們借鑑了「有意義的工作」相關研究。這類研究指出，工作對個人而言之所以有意義，是因為工作反映了這個人的自我（Gardner、Csikszentmihalyi，&
Damon，2001；Gini，2000；Pratt 與 Ashforth，2003）。Kahn（1990）也指出，當一個人能透過工作及其在組織中的成員身分表達「理想中的自我」時，工作便會變得有意義
p
。基於這些研究，我們主張，讓身分認同與工作相契合，是建構身分認同的重要動力。

職涯／角色轉換。我們的研究拓展了職涯與角色轉
e
換領域的研究，並有兩方面的貢獻。首先，在探討身分認同調整過程的相關因素時，我們補充了 Hall（1968、1971、1995）的研究，指出工作學習循環嵌入於
n
身分學習循環。鑑於在人們職涯轉換期間，學習工作與學習身分密切相關，這或許並不令人意外。這種相互作用或許可以解釋角色概念經常引發的混淆（Ashforth, 2001
;
Biddle, 1986）。Biddle 在回顧角色理論時，Biddl e
指出，角色可能指涉行為、人們所扮演的部分（社會位置），或行為腳本（角色期待）。然而，若將工作行為與專業身分分開看待？而非將兩者混同為角色？w
e
可以看出，這些對角色的不同概念化方式都與身分建構過程有關。我們的模型指出，個人的所作所為（工作行為）常會與對其身分的期待（身分評估）相比較，以促使

此內容下載自120.126.194.245，時間為 2026 年 31 ct
06:54:44 UTC 所有使用均受 https://about.jstor.org/terms 規範

about who one is (identity assessments) to motivate the construction process. Newly constructed identities are in turn validated by others with similar social positions. Perhaps different conceptualizations of roles stem from focusing on different "parts" of this identity construction process.

Second, we build upon this research by describing the job characteristics that might influence whether individuals change themselves or change their work. Nicholson and colleagues (Nicholson, 1984; West et al, 1987) argued that job discretion and job novelty are two critical factors. In our study, we also found that a lack of job discretion favored identity change over changes in work. We further expect that when work becomes more discretionary, as occurs when someone gains autonomy through experience or expertise, the relative emphases on work and identity learning cycles will change, and work customization, or "job crafting," will occur (Wrzesniewski & Dutton, 2001). Further research that tracks individuals for even longer spans in their work careers, however, is needed to validate this prediction.

We found the novelty of work tasks to be a less helpful predictor. To illustrate, both our surgery and primary care residents had similar work to do, and they had relatively similar training in medical school. Thus, the work that they did should have been comparable in terms of novelty. However, identity change was greater for the surgical residents. Our data suggest that it was work-identity integrity violations, rather than novelty, that motivated identity customization. Novel situations may evoke such violations, but even familiar work may lead to identity change if it does not fit an individual's conceptualization of who he/she is. This possibility suggests a need to look not just at the characteristics of work, but also at the characteristics of the worker, when predicting identity change.

We also extend research in this area by going beyond predicting when identity change might occur, to predicting *how* it might occur. Thus, we predicted that individuals with low job discretion and well-developed identities—who experience minor integrity violations—are likely to experience identity enrichment. Alternatively, individuals with low job discretion and major violations will either patch or splint their identities, depending on how well developed their identities are. As far as we know, no other research has been able to match individual (e.g., identity development) and job characteristics (e.g., job discretion) with specific change outcomes (e.g., identity enrichment). And certainly, more research is needed to explore other possibilities (e.g., high discretion/weak identity/minor violation). In sum, we believe that research on predictors of job versus identity change in careers needs to focus on a wider set of factors.

Socialization. Finally, our research adds to the often overlooked relationship between socialization and identity change (Ibarra, 1999). In some ways, the question of whether individuals change to fit their jobs or change their jobs to fit themselves is also evidenced in the literature on socialization tactics (Ashforth & Saks, 1996; Van Maanen & Schein, 1979), which has asked what predicts when an individual will adopt a role (become a role custodian) or change it (become a role innovator). Thus, as noted above, our research sheds new light on this issue by going beyond what an organization does (socializes people in groups) to address what individuals bring to the organization (e.g., prior socialization; see also Nicholson [1984]) and working conditions (e.g., lack of job discretion).

At a more general level, compared with those studies that have focused primarily on how organizations convert or transform the identity of members, the present study offers a much more subtle and nuanced perspective on identity change. Work on psychological (e.g., brainwashing [Schein, 1962]) and sociological conversion (Greil & Rudy, 1983; Lofland & Stark, 1965), for example, suggests that individuals adopt whole identities and discard old ones. We argue that identity change can be more incremental. Instead of adopting a whole new identity, an individual can enrich an existing identity, patch together two (or more) identities, or use another identity as a temporary splint.

Though we detail less extreme forms of identity change, we resonate with the socialization focus of looking at how organizations influence the identity construction process. Although clearly "people are not just milquetoasts who passively sit back as targets of others' treatment and absorb any identity-relevant information they encounter" (Polzer, Milton, & Swann, 2002: 299), our research reveals a fair amount of intragroup conformity within the three different residency groups regarding how identities were constructed. Thus, there was some consensus around what it meant to be a primary care physician, radiologist, or surgeon in all waves of data (see Table 1). How was such consensus achieved? We believe that this consensus came about because the organization both created the need for sensemaking and limited members' identity sets.

To begin, we note that, by having high discretion over the content and process of work, the medical center's administrators were able to trigger work-identity violations. Medical directors explicitly told us that over the course of residency, residents would feel "stupid" and "frustrated by doing 'scut work.'" Thus, they propagated the conditions that

This content downloaded from
20.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

關於一個人是誰（身分認同評估）以激勵 e
建構過程。新建構的身分接著會由具有相似社會地位的他人加以驗證。或許，對角色的不同概念化源自於聚焦於不同

a
角色的不同詮釋源自於聚焦於不同的面向 t
"parts" of thi s identity cons tructi在過程中 .
其次，我們透過描述可能影響個人是否改變自己或改變工作的工作特徵，進一步建立在這項研究之上。Nicholson 及其同事（Nicholson, 1984；West et al., 1987）認為，工作裁量權與工作的新穎性是兩項關鍵因素。在我們的研究中，我們也發現，缺乏工作裁量權更有利於身分認同的改變，而非工作本身的改變。W

e
我們進一步預期，當工作變得更具裁量性時——例如個人透過經驗或專業取得自主權——工作與身分認同學習循環的相對重點將會

l
改變，而工作客製化，或稱「工作塑造」，也將隨之發生（Wrzesniewski & Dutton, 2001）。然而，仍需進一步研究，追蹤個人在職涯中更長時間的發展，以驗證這項

P項預測。
我們發現，工作任務的新穎性是一項較少 s
有助於預測的因素。舉例來說，外科住院醫師和基層醫療住院醫師都面臨相似的工作 ,
而且他們接受的醫學 l
院訓練也相對相似。因此，他們所從事的工作在新穎性方面應該具有可比性。然而 ,
外科住院醫師的身分變化幅度較大。我們的資料顯示，促使身分客製化的是工作與身分完整性之間的違背，而非新穎性。新穎情境可能引發這類違背，但即使是熟悉的工作，只要不符合個人對自身身分的概念，也可能導致身分變化。這種可能性顯示，我們不應只關注工作的特徵，也應關注工作者的特徵

, when p預測 g 身分 y change.
我們也拓展了此領域的研究，不再只預測身分何時可能發生變化，也預測這種變化可能如何發生。因此，我們預測，工作自主性低且身分發展完善的個人——經歷輕微誠信違規者——很可能會經歷身分豐富化。相較之下，工作自主性低且遭遇重大違規的個人 s
將根據具體情況修補或加固其身分， n
其身分認同發展得多麼完善。據我們所知，尚無其他研究能夠將個人特徵（例如身分認同發展）與工作特徵（例如工作裁量權），配對至特定的變化結果（例如身分認同豐富化）。當然，我們仍需要更多研究來探討其他可能性（例如高裁量權／薄弱身分認同／輕微違規）。總而言之，我們認為，研究

職涯中工作變動與身分認同變動之預測因素時，需要聚焦於更廣泛的因素集合。

社會化。最後，我們的研究也增進了對 e
經常遭到忽視的社會化與身分變化 a
之間關係的理解（Ibarra，1999）。在某些 e
方面，個人究竟是改變自己以適應工作，還是改變工作以適應自己，這個問題 s
也可從有關社會化策略的文獻中得到佐證（Ashforth & Saks, 1996；Van Maanen & Schein, 1979）；這些研究探討了哪些因素能預測 s
個人何時會採納某種角色（成為角色 e
守成者）或改變該角色（成為角色創新者）。因此，如上所述，我們的研究超越了組織所做的事情，為這個議題帶來新的見解 n
（以群體方式使人社會化），轉而探討個人帶給組織的因素（例如，先前的社會化經驗；另見 Nicholson [1984]）以及在
g 工作條件下（例如g.，缺乏工作自主權）。
從更廣泛的層面來看，與那些 e
主要探討組織如何轉換或改變成員身分認同的研究相比，本研究提出了更為細緻 e
且更具層次的身分認同變化觀點。關於心理轉變的研究（例如，洗腦 [Schein, 1962] ）以及社會學轉變的研究（Greil & Rudy, 1983 ;
Lofland & Stark, 1965），例如，指出個人會採納完整的新身分認同，並拋棄舊有的身分認同。我們認為，身分認同的變化可以更為漸進。個人不一定要採納一套全新的身分認同，也可以豐富既有的身分認同 ,
拼湊兩種（或更多種）身分認同，或運用另一種 r
identit作為一 t暫時性專 lint.
雖然我們詳細探討的是較不極端的身分轉變形式，但我們的研究呼應了社會化研究對於 f
探討組織如何影響身分 y
建構過程。儘管顯然「人們並非只是些唯唯諾諾的人，被動地坐著，任由他人對待自己，並吸收所遇到的任何與身分相關資訊」（Polzer、Milton 與 Swann，2002：299），我們的研究顯示，三個 e
種不同住院醫師群體在身分建構方式上呈現相當程度的群體內從眾。因此，在所有資料蒐集階段，大家對於成為基層照護醫師、放射科醫師或外科醫師的意義都有一定共識 a
（見表 1）。這樣的共識是如何達成的？ ?
我們認為，這種共識之所以形成，是因為該組織既創造了進行意義建構的需求
g 也限制了成員的身分 y 集合。
首先，我們指出，藉由對工作內容與流程擁有高度自主權 n
對工作內容與流程，醫療 l
中心的行政人員能夠引發工作身分認同的違背。醫療主任明確告訴我們，在住院醫師訓練期間，住院醫師 s
會覺得自己「很笨」，並且「因為做『scu t
工作。' 因此，他們傳播了促成

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從20.126.194.245 下載 所有使用均受 https://about.jstor.org/terms 條款約束

motivated identity construction. Administrators also controlled what raw materials the physicians would use to construct their professional identities. As noted in Table 1, first-year residents spent anywhere from 45 to 70 percent of their waking hours at the hospital and spent most of their time with hospital personnel. This picture suggests that residents were well encapsulated, cut off from people other than hospital personnel (Greil & Rudy, 1984). Pratt (2000) suggested that organizations that encapsulate members have a high degree of control over member sensemaking. In the language of our study, by limiting contact with non–hospital members, the medical organization limited the potential identity set, the raw materials that members drew upon to make sense of their work. Given the residents' lack of significant outside contact, it is perhaps not surprising that they used medical school experiences as one of their first sources for making sense of their role as residents. However, when these experiences did not suffice, as was the case for surgical residents, the residents turned to organizational sources for meaning (e.g., organizational stories and organizational dress codes).

This is not to say that there was no variation in how identity was constructed. Some surgical residents, for example, gravitated more toward the "most complete doctor" identity than to the "Boot Camp surgeon" identity. Similarly, at the end of their first year, some primary care residents viewed themselves as "quarterbacks," or coordinators for entire medical teams working with patients, while others saw themselves more actively, as "advocates" or even "parents" (see Table 1). Therefore, the customization process described herein appears more akin to *mass customization,* a process whereby goods are produced on an assembly line but tailored to an individual's needs. Here, members can be said to have been able to pick and choose from various identity elements—as if choosing from an identity "menu"—to make sense of their work, but these choices were highly constrained by the organizational context.

Practical Implications: Health Care and Residency Training

We believe that our study has practical implications for the professional training of medical residents. For example, the number of hours that residents spend at work, long assumed to be natural and inherent to excellent training, has come under harsh criticism by those who argue that it improves neither patient care nor the ability of residents to learn (e.g., McCall, 1988). In response to such criticism, the Accreditation Council of Graduate Medical Education has established strict guidelines mandating that residents spend no more than 80 hours per week in their hospitals. Programs that violate these rules can face a variety of sanctions, up to and including loss of accreditation. In our study (which preceded these legal changes), many physicians worked more than 80 hours a week. To the extent that such physical encapsulation aids an organization's ability to control how physicians' construct their identities, one may wonder whether and how fewer hours might influence identity customization and perceived competence. We suggest that medical organizations concerned about these potential outcomes should look to alternative ways of encapsulating their members (Greil & Rudy, 1983), such as establishing strong interpersonal bonds (social encapsulation) and fostering belief systems that discourage members from seeking advice from nonmembers (ideological encapsulation).

Perhaps more importantly, our research sheds light on how the training of physicians might influence the performance of those in whose care we place our lives when sick or otherwise disabled. It was clear in this context that physicians, who at the time had significant (although not sole) responsibility as care providers, felt somewhat incompetent as they attempted to figure out who they were in light of the work they were assigned. As we have noted, program administrators expect such feelings. We did not directly measure levels of stress, but it was clear that violations to professional identity were distressing to residents. Although the training tactics of this organization were ultimately successful—for example, in the case of surgical residents, the identity customization process did work to create "most complete doctors" and "Boot Camp surgeons"—one wonders if both the *length of training* and *integrity violations* are essential for producing good doctors.

Can the work and identity learning cycles be completed more quickly, so that feelings of integrity come more rapidly than we observed them to in Boot Camp? Do surgeons really need two years of "scut work" to achieve these ends? Our data, while not conclusive, hint that giving physicians more professionally relevant tasks earlier in their residency might speed up this process. What is less clear, however, is whether there is any proxy for work-identity integrity violations. Might another motivator for identity construction be found? Given that professional identity is so bound up in what residents do, these violations may be necessary to spark work and identity learning cycles. However, if such stresses are inherent to becoming a professional, perhaps health care organizations should

This content downloaded from
20.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

身分建構的條件。管理者也控制了醫師所需的原材料

s

會用來建構其專業身分。表 1 顯示，第一年住院醫師清醒時段中，有 45% 至 70% 的時間

s

待在醫院，且大部分時間都與醫院人員相處。這幅圖景顯示，住院醫師受到良好隔離，與醫院人員以外的人隔絕（Greil & Rudy, 1984）

.

Pratt（2000）指出，將成員封閉其中的組織，對成員如何建構意義具有高度控制力。以本研究的用語來說，透過限制成員與非醫院人員接觸，醫療組織限制了身分

l

集合，也就是成員用來 w
理解自身工作的原始材料。鑑於這些研究結果 i

由於住院醫師與外界接觸不多，他們將醫學院經驗作為理解自己住院醫師角色的最初來源之一，或許並不令人意外。然而，當

n

這些經驗不足以提供充分理解時，外科住院醫師便轉而從組織來源尋找意義（例如組織

l

故事和組 g織服裝規範）。

這並不是說身分建構的方式沒有差異。例如，一些外科住院醫師比起「訓練營外科醫師」身分，更傾向於「最完整的醫師」身分。同樣地，在第一年結束時，一些基層醫療住院醫師將自己視為負責協調整個醫療團隊的「四分衛」，

r

這些團隊與病人共同工作；另一些人則更積極地將自己視為「倡議者」，甚至是「父母」（見表 1）。因此，本文所描述的客製化過程，似乎更接近大量客製化：產品在生產線上製造，再依個人需求調整。在此，成員可以從各種身分元素中挑選，彷彿從身分「選單」中選擇，以理解自

f

他們的工作，但這些選擇受到組織環境的嚴重限制。

實務意涵：醫療照護與 d
住院醫師訓練

我們相信，本研究對醫學住院醫師專業訓練具有實務上的意涵。 i

師。例如，住院醫師投入工作的時數，一直被視為優良訓練中自然且不可或缺的一環，如今已受到

r

嚴厲批評，批評者認為這既無法改善病人照護，也無法提升住院醫師 o
學習（例如，McCall，1988）。為回應這類批 t
批判，美國研究所醫學教育認證委員會

已制定嚴格規範，要求住院醫師每週在醫院工作的時間不得超過 80 小時。違反這些規定的培訓計畫可能面臨各種制裁，最嚴重甚至會失去認證。在我

r

研究（早於這些法律變革），醫 y
醫師每週工作超過 80 小時 .
若這種身體上的封閉 n

有助於組織掌控醫師如何建構自己的身分認同，人們或許會想知道，工時減少是否以及如何影響身分認同的調整與感知能力。我們認為，關注這些潛在結果的醫療組織

s

應尋求其他封閉成員的方式（Greil & Rudy，1983），例如 s

建立緊密的人際連結（社會封閉），並形塑

s

一套阻止成員向非成員尋求建議的信念體系（意識形態 g封 p閉）。或許更重要的是，我們的研究揭示了醫師的訓練可能如何影

e t

響那些在我們生病或失能時照護我們的人之表現。我在這個脈絡下很明顯，當時身為照護提供者、承擔重大（但並非全部）責任的醫師，在試圖根據分派給他們的工作釐清自我定位時，感到有些無能為力。正如我們所

e

指出的，計畫管理者預期會出現這類感受。我們並未直接測量壓力程度 ,

但很明顯，對專業身分的侵犯令住院醫師感到痛苦。儘管這個組織的訓練策略最終取得了成功？例如，就外科住院醫師而言，身分塑造過程確實造就了「最完整的醫師」與「訓練營外科醫師」？人們仍不禁要問，訓練時間的長短與身分完整性的侵犯是否都是培養

g g優秀的醫師。

工作與身分學習的循環能否更快完成，使完整感比我們在訓練營中觀察到的更早出現？外科醫師真的需要兩年的「雜務工作」才能達成這些目標嗎？我們的資料，雖

e

雖然尚無定論，但這暗示著，讓醫師在住院醫師訓練期間較早承擔更多與專業相關的任務，或許能加快這個過程。較不

s

清楚的是，是否存在可作為工作身分完整性受損的指標。身分建構是否還有其他動力？既然專業身分如此緊密地繫於

t

住院醫師的這些違規行為或許是促使工作與身分學習循環展開的必要因素。然而，如果這類壓力本就伴隨著成為專業人士的過程，醫療機構或許應該 d

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從 20.126.194.245 下
載 所有使用均須遵守 https://about.jstor.org/terms

discourage using tactics that exacerbate an already stressful identity transition—for instance, they might discourage validating professional identity via biting criticism.

Practical Implications: Professional Training and Management

We believe that our findings hold not only for socializing physicians, but also for training most other (if not all) professionals. With regard to managing the identity construction process of professionals during their training, we argue that decision makers see this management as a *system of choices* that continue from selection through socialization. Given the importance of prior socialization for how members construct identities, more emphasis should be given to understanding employees' prior socialization experiences and not just their prior memberships. As Louis (1980) noted, to understand what individuals are "changing into" at work, it is critical to know what they are "changing from."[10] In addition, organizational leaders need to remember their role in three interrelated steps of the identification construction process: (1) creating the need for construction, (2) providing the raw materials for such construction, and (3) providing opportunities for validating newly constructed identities.

First, with regard to motivation, we feel that most professional programs, such as academic graduate programs, create work-identity integrity violations in the course of training (e.g., is being a doctoral student all about copying papers from the library?). As noted, given that these violations are likely to be stressful, those managing professional training programs may seek to minimize stress in other parts of the system (e.g., helping students validate identities). Second, regarding the provision of raw materials, program leaders need to be cognizant of the values and beliefs that they may be unintentionally communicating to students. As research has shown, even small changes in the dress codes of established professionals can have large implications for how they construe their professional identities (Pratt & Rafaeli, 1997). Moreover, being aware of both the values and beliefs that a profession holds, and *how they are transmitted,* will help professional educators adjust to environmental jolts, such as legal challenges to how socialization is done. Third, professional educators need to be cognizant of the power of role models, even role models who are not mentors. *All* members of a training faculty are potential sources of professional identity validation (Ibarra, 1999). This observation suggests that professional educators periodically assess faculty not only for their skills, but also for their fit with professional values and beliefs. Finally, our research suggests that informal feedback and grapevines are especially powerful. This observation implies that educators should be aware not only of what is being taught in their curricula, but also of what is being taught informally in the "hidden curricula" (e.g., Margolis & Romero, 1998).

More generally, our study sheds some new light on the relationship between professional and organizational attachment by noting the relationship between identity construction and patterns of identification. Goulder (1957), for example, viewed differences in degrees of loyalty to an organization and a profession as critical in delineating "locals" and "cosmopolitans." These distinctions, in turn, had ramifications for organizational rule following, organizational influence and participation, and the like. This line of inquiry regarding attachment to organization versus profession remains a vital line of inquiry. In fact, Wallace (1995) argued that how these affiliation questions play out are increasingly important to organizations, as professionals become a greater part of nonprofessional organizations. Of the three groups we examined, only surgeons appeared to incorporate their organizational affiliations into their self-conceptualizations.

Research suggests that identities are the lenses through which people make sense of the world (Weick, 1995) and that identification with a social group may be motivated by the need to reduce uncertainty about one's place in that group (Hogg & Abrams, 1993; Hogg & Mullin, 1999; Pratt, 2000). We take these arguments a step further by emphasizing that such sensemaking is *pragmatic.* Residents engaged in certain types of identity customization because it helped them to better understand their work. For example, surgical residents adopted the Boot Camp surgeon identity to make sense of work intensity. This observation suggests that traditional drivers of organizational identification, such as prestige and distinctiveness (Ashforth & Mael, 1989; Dutton et al., 1994; Pratt, 1998) may not be sufficient for professionals, who may fulfill such needs via their professional affiliation. Rather, adopting an organizationally relevant identity must have some pragmatic value.

[10] This need to understand existing identities before one can understand identity change is echoed in Erikson's (1983) classic work on stages of identity development and in Cote and Levine's (2002) notion of "identity capital," or investment in existing identities.

This content downloaded from 120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

避免採用會加劇身分轉變壓力的手段？例如，

y

可能會抑制人們透過尖銳批評來確認專業認同。

實務意涵：專業訓練與管理

我們認為，研究結果不僅適用於醫師的社會化過程，也適用於多數（甚至所有）其他專業人士的訓練。至於如何管理專業人士在受訓期間的認同建構過程，我們主張，決策者將這種管理視為一套選擇系統

s

在受訓期間的專業人士，我們主張，決策者將這種管理視為一套選擇系統

s

從甄選一路延續到社會化。鑑於先前社會化經驗對於如何

w

成員建構身分認同的重要性，應更加重視瞭解員工過去的社會化經驗，而不只是他們過去的組織成員身分。正如 Louis（1980）所指出，若要瞭解個人在工作中「正在轉變成什麼」，關鍵在於知道他們「原本是什麼」。此外，組織領導者需要

o

記住他們在三個彼此相關的步驟中的角色 o

f

身分認同建構的過程：（1）創造建構的需求；（2）提供建構所需的原始素材；以及（3）提供驗證新建構身分認同的機會。

首先，就動機而言，我們認為大多數專業培訓課程，例如學術研究所課程，會在培訓過程中造成工作身分完整性的違反（例如，成為博士生就只是從圖書館複印論文嗎？）。如前所述，鑑於這些違反很可能造成壓力，負責管理專業培訓課程的人員可能會設法降低系統其他部分的壓力（例如，協助學生驗證身分）。其次，就原材料的提供而言，課程領導者需要意識到他們可能無意間向學生傳達的價值觀與信念。研究顯示，即使只是稍微改變既有專業人士的服裝規範，也可能對他們如何理解自身的專業身分產生重大影響（Pratt & Rafaeli, 1997）。此外，了解某一專業所持有的價值觀與信念，以及這些價值觀與信念如何被傳遞，將有助於專業教育者調整以因應環境衝擊，

10 在理解身分認同如何改變之前，必須先瞭解既有身分認同；這一點也見於 Erikson（1983）關於身分認同發展階段的經典著作，以及 Cote 與 Levine（2002）提出的「身分資本」概念，亦即對既有身分認同的投入。

例如對社會化進行方式提出的法律挑戰。第三，專業教育工作者需要認知到角色模範的力量，即使這些角色模範並非導師。培訓教職員的所有成員都可能成為驗證專業認同的來源（Ibarra，1999）。這項觀察顯示，專業教育工作者應定期評估教職員，不僅評估他們的技能，也評估他們是否符合專業價值觀與信念。最後，我們的研究顯示，非正式回饋與私下流傳的消息尤其具有影響力。這項觀察意味著，教育工作者不僅應注意課程中教授的內容，也應注意「隱藏課程」中非正式教授的內容（例如。

g., Margolis 與 Romero ，1998）。

更廣泛而言，本研究透過指出身分建構與認同模式之間的關係，為專業依附與組織依附之間的關係提供了一些新的見解。例如，Goulder（1957）認為，對組織與專業的忠誠程度差異，是界定「地方派」與「世界派」的關鍵。這些區分進而對組織規則的遵循、組織中的影響力與參與程度等產生了影響。關於依附於組織相較於依附於專業的研究方向，至今仍十分重要。事實上，Wallace（1995）主張，這些歸屬問題如何發展，對組織而言日益重要，因為專業人士在非專業組織中所占的比例逐漸增加。在我們研究的三個群體中，只有外科醫師似乎將其組織歸屬納入自我概念

P之中。

研究指出，身分認同是人們理解世界的透鏡（Weick, 1995），而認同某個社會群體，可能是為了減少對自己在該群體中定位的不確定感（Hogg & Abrams, 1993; Hogg & Mullin, 1999; Pratt, 2000）。我們進一步指出，這種理解世界的過程具有務實目的。住院醫師之所以進行某些身分認同的調整，是因為這有助於他們更了解自己的工作。例如，外科住院醫師採納「訓練營外科醫師」的身分認同，以理解工作強度。這項觀察顯示，聲望與獨特性等傳統的組織認同驅動因素（Ashforth & Mael, 1989; Dutton et al., 1994; Pratt, 1998），對專業人士而言可能並不足夠，因為他們可能透過所屬專業滿足這些需求。相反地，採納與組織相關的身分認同，必須具有某種務實價值。

此內容下載自120.126.194.245，時間為 2026 年 10 月 31 日 06:54:44 UTC
所有使用均受 https://about.jstor.org/terms 的條款約束

Limitations and Future Research

As with all attempts to build and elaborate theory from a limited sample, one must be careful when generalizing these findings to other organizations. For example, our focus has been on professional identity as it undergoes change. Changes in other work-related identities, or even nonwork identities, may be qualitatively different. However, although as a qualitative study our research lacks statistical generalizability, we do feel that it can be used for "naturalistic generalization," whereby one recognizes similarities based on experiences with similar "cases" without any statistical inference (Stake, 1995). For example, as a prototypical profession (Hughes, 1956), medicine offers fairly easy-to-see parallels with other professions. Doctoral students, for example, undergo cycles of identity and work learning resembling the cycles these physicians experienced (see Hall, 1968). Similarly, treatments of law students have often focused on their encapsulation during the first year of training (e.g., Turow, 1977). In addition, we see our study as having "analytical generalizability" in that its purpose is to "expand and generalize theories . . . not to enumerate frequencies" (Yin, 2003: 10).

We must also be careful about the unique nature of the setting. As noted in the introduction, our work focuses on professionals. Because professions are delineated by their unique sets of work knowledge and skills, work-identity integrity is highly likely to be a motivator for professionals, as people who derive a large part of their self-meaning from the work that they do. By contrast, work-identity violations may be minimally motivating to those who find meaning in the culture of their workplaces (see Pratt and Ashforth's [2003] distinction between meaningfulness "in" and "at" work), and such violations may be minimally motivating to those who find meaning primarily outside of the work arena. In future work, researchers may wish to more explicitly compare identity construction dynamics among professionals and nonprofessionals, and among those who look at work as a primary source of meaning in their lives and those who do not.

In addition, our emphasis on identity construction as opposed to job crafting or role innovation largely stems from the fact that we were examining professionals-in-training. As noted, we believe that emphasis might shift to job crafting or role innovation as people gain experience in their work. This idea suggests the need to view the identity customization of people at various stages in their career processes.

Conclusion

Despite increasing interest in identity, and the importance of professionals in organizations, there is little research on how professionals construct professional identities. The goal of the current research was to build and enrich theory in this area. We believe we contribute to theory in several important ways.

First, our research shows how each of several existing bodies of knowledge (including socialization, career/role transitions, and identity work) illuminates separate aspects of identity construction. Specifically, our findings advance theory in each of these areas as they pertain to how professionals build identity over time. In doing so, we provide new insights into extant conclusions (e.g., why perceived competence accompanies identity construction), refine current ways of thinking (e.g., the importance of the magnitude of integrity violations rather than task newness as a trigger for identity change), and show how findings from these separate literatures are linked in the identity construction process.

Additionally, in our findings and in our construction of a theoretical model, we make several advances in understanding professional identity construction. First, our research points to the importance of looking at work-identity *integrity* as motivating identity construction. Thus, we argue for the need to examine how work and identity reinforce each other rather than concentrating on work-identity fit, which can be conceptualized in a myriad of ways (Kristof, 1996). Second, our model focuses attention on different types of identity customization, and our findings suggest not only the conditions that might lead to each type of customization, but also how and why these customization types may change over time. Third, we discuss some correlates of identity construction: perceived competence and different patterns of identification. The last is especially useful to draw on when predicting when professionals might also identify with their organizations—a key concern in the study of professionals (e.g., Gouldner, 1957; Wallace, 1995). Finally, we reiterate the importance of understanding "what they do" in order to gain a richer appreciation of "who they are" in the identity construction of professionals. As such, we show how organizational practices, such as giving feedback and providing role models, serve a dual role in improving the understanding of both work and professional identity. In making these advances, we hope that this research might serve as a foundation for further research on identity construction in organizations.

This content downloaded from
20.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

研究限制與未來研究

如同所有試圖根據有限樣本建構並闡述理論的研究，都必須謹慎看待

將這些研究發現推廣至其他組織。例如，我們關注的是專業認同在經歷變化時的情況。其他工作相關認同，甚至非工作認同的變化，可能在性質上有所不同。然而，儘管

我們的研究是一項質性研究，缺乏統計上的一般化能力，但我們認為它可以用於「自然式一般化」；在此過程中，人們根據對相似「個案」的經驗，在不進行任何統計推論的情況下辨識其相似之處（Stake

1995）。例如，作為典型的專業

（Hughes，1956），醫學提供了與其他專業相當容易看出的相似之處。博士生

例如，經歷類似於這些醫師所經歷之循環的身分與工作學習週期（見 Hall，1968）。同樣地，治療方

法律學生在訓練的第一年往往專注於自身的封閉化（例如，Turow

1977）。此外，我們認為本研究具有「分析性概括性」，因為其目的在於「擴展並概括理論……而非枚舉

ate f頻率 ies」(Yi n, 2003：10）。

我們也必須注意這一情境的獨特性。如引言所述，我們的研究聚焦於專業人士。由於各種專業是以其獨特的工作知

識與技能組合來界定，工作認同完整性很可能是專業人士的動機來源，因為他們很大一部分的自我意義都來自所從事的工作。相較之下，對於那些從工作場所的文化中尋找意義的人而言，工作認同受到侵犯可能僅具有極低的激勵作用（參見 Pratt 與 Ashforth 對工作「中的」意義與工作「上的」意義之區分，即 [2003]），而對於主要在工作領域之外尋找意義的人而言，這類侵犯可能同樣僅具有極低的激勵作用。未來的研究可以更明確地比較專業人士與非專業人士之間的認同建構動態，以及那些將工作視為生活

中主要意義來源的人與不將工作視為主要意義來源的人之間的差異。

此外，我們之所以強調認同建構，而非工作塑造或角色創新，主要是因為我們研究的是正在接受專業訓練的人員。如前所述，我們相信

隨著人們在工作中累積經驗，重點可能會轉向工作塑造或角色創新。這個觀點指出，有必要探討人們在職涯歷程不同階段如何調整自己的認同。

結論

儘管人們對認同的興趣日益增加，且專業人士在組織中的重要性不容忽視，但目前探討專業人士如何建構專業認同的研究仍相當有限。本研究旨在建構並深化此領域的理論。我們相信，本研究在幾個方面對理論有所貢獻

重要的方式 ys.

首先，我們的研究顯示，現有的幾個知識體系（包括社會化、職涯／角色轉換與身分認同建構）如何分別闡明身分建構的不同面向。具體而言，我們的研究發現推進了這些領域的理論，說明專業人士如何隨時間建立身分認同。在此過程中，我們為既有結論提供了新的洞見（例如，為何知覺到的能力會伴隨身分認同建構），細化了目前的思考方式（例如，觸發身分認同改變的是誠信違反的程度，而非任務的新穎性），並展示這些分立文獻中的研究發現如何在身分建構過程中彼此連結

的過程。

此外，透過研究結果與理論模型的建構，我們在理解專業工作者的身分認同建構方面取得了幾項進展。首先，我們的研究指出，探討工作與身分認同的一致性如何促使身分認同建構，十分重要。因此，我們主張應檢視工作與身分認同如何相互強化，而非將焦點放在工作與身分認同的契合度上；契合度可以有許多不同的概念化方式（Kristof, 1996）。其次，我們的模型聚焦於不同類型的身分認同調整；研究結果不僅指出可能促成各類調整的條件，也說明這些調整類型可能如何以及為何隨時間改變。第三，我們探討了身分認同建構的一些相關因素：知覺到的能力，以及不同的認同模式。後者在預測專業工作者何時也可能認同其組織時，尤其值得參考；這是專業工作者研究中的一項重要議題（例如 Gouldner, 1957; Wallace, 1995）。最後，我們再次強調，理解「他們做什麼」，有助於更深入體會專業工作者在身分認同建構中「他們是誰」。因此，我們說明了組織實務（例如提供回饋與角色模範）如何發揮雙重作用，同時增進對工作與專業身分認同的理解。我們期望，這些進展能使本研究成為組織中身分認同建構後續研究的基礎。

此內容下載自20.126.194.245，時間為 2026 年 31 月
06:54:44 UTC 所有使用均受 https://about.jstor.org/terms 約束

REFERENCES

Abbott, A. 1992. Professional work. In Y. Hasenfeld (Ed.), ***Human services as complex organizations:*** 145–162. Chicago: University of Chicago Press.

Albert, S., & Whetten, D. A. 1985. Organizational identity. In L. L. Cummings & B. M. Staw (Eds.), ***Research in organizational behavior,*** vol. 7: 263–295. Greenwich, CT: JAI Press.

Alvesson, M. 1994. Talking in organizations: Managing identity and impressions in an advertising agency. ***Organization Studies,*** 15: 535–563.

Ashforth, B. E. 2001. ***Role transitions in organizational life: An identity-based perspective.*** Mahwah, NJ: Erlbaum.

Ashforth, B. E., & Mael, F. 1989. Social identity theory and the organization. ***Academy of Management Review,*** 14: 20–39.

Ashforth, B. E., & Saks, A.M. 1996. Socialization tactics: Longitudinal effects on newcomer adjustment. ***Academy of Management Journal,*** 39: 149–178.

Baumeister, R. F. 1998. The self. In G. Lindzey (Ed.), ***The handbook of social psychology,*** (4th ed.): 680–740. New York: Mc-Graw Hill.

Becker, H. S. 1970. ***Sociological work: Method and substance.*** Chicago: Aldine.

Becker, H., Geer, B., Hughes, E., & Strauss, A. 1961. ***Boys in white: Student culture in medical school.*** Chicago: University of Chicago Press.

Biddle, B. J. 1986. Recent developments in role theory. In R. Turner & J. F. Short (Eds.), ***Annual review of sociology,*** vol. 12: 67–92. Palo Alto, CA: Annual Reviews.

Blackwell, B., Gutmann, M. C., & Jewell, K. E. 1984. Role adoption in residency training. ***General Hospital Psychiatry,*** 6: 280–288.

Carr-Saunders, A. M., & Wilson, P.A. 1933. ***The professions.*** Oxford, U.K.: Clarendon Press.

Conrad, P. 1988. Learning to doctor: Reflections on recent accounts of the medical school years. ***Journal of Health and Social Behavior,*** 29: 323–332.

Cote, J. E., & Levine, C. G. 2002. ***Identity formation, agency, and culture: A social psychological synthesis.*** Mahwah, NJ: Erlbaum.

Deaux, K. 1991. Social identities: Thoughts on structure and change. In R. C. Curtis (Ed.), ***The relational self: Theoretical convergences in psychoanalysis and social psychology:*** 77–93. New York: Guilford.

Dickie, V. A. 2003. Establishing worker identity: A study of people in craft work. ***American Journal of Occupational Therapy,*** 57: 250–261.

Dutton, J. E., Dukerich, J. M., & Harquail, C. V. 1994. Organizational images and member identification. ***Administrative Science Quarterly,*** 39: 239–263.

Eisenhardt, K. M. 1989. Building theories from case study research. ***Academy of Management Review,*** 14: 532–550.

Epstein, S. 2002. Cognitive-experiential self-theory of personality. In M. J. Lerner (Ed.), ***Comprehensive handbook of psychology, vol. 5—Personality and social psychology:*** 159–184. Hoboken, NJ: Wiley.

Erez, M., & Earley, C. 1993. ***Culture, self-identity, and work.*** New York: Oxford University Press.

Erikson, E. 1982. ***The life cycle completed: A review.*** New York: Norton.

Freidson, E. 1970. ***Profession of medicine: A study of the sociology of applied knowledge.*** New York: Dodd, Mead.

Freidson, E. 2001. ***Professionalism: The third logic.*** Chicago: University of Chicago Press.

Gardner, H., Csikszentmihalyi, M., & Damon, W. 2001. ***Good work: When excellence and ethics meet.*** New York: Basic Books.

Gini, A. 2000. ***My job, my self: Work and the creation of the modern individual.*** New York: Routledge.

Glaser, B. G., & Strauss, A. 1967. ***The discovery of grounded theory: Strategies for qualitative research.*** Chicago: Aldine.

Gouldner, A. 1957. Cosmopolitans and locals: Towards an analysis of latent social roles–I. ***Administrative Science Quarterly,*** 2: 281–306.

Greil, A. L., & Rudy, D. R. 1983. Conversion to the world view of alcoholics anonymous: A refinement of conversion theory. ***Qualitative Sociology,*** 6: 5–28.

Greil, A. L., & Rudy, D. R. 1984. Social cocoons: Encapsulation and identity transformation in organizations. ***Sociological Inquiry,*** 54: 260–278.

Hall, D. 1968. Identity changes during the transition from student to professor. ***School Review,*** 76: 445–469.

Hall, D. 1971. A theoretical model of career subidentity development in organizational settings. ***Organizational Behavior and Human Performance,*** 6: 50–76.

Hall, D. 1995. Unplanned executive transitions and the dance of subidentities. ***Human Resource*** **Management,** 34: 71–92.

Hogg, M. A., & Abrams, D. 1993. Towards a single-process uncertainty-reduction model of social motivation in groups. In M. A. Hogg & D. Abrams (Eds.), ***Motivation—Social psychological perspectives:*** 173–190. New York: Harvester-Wheatsheaf.

Hogg, M. A., & Mullin, B. A. 1999. Joining groups to reduce uncertainty: Subjective uncertainty reduction and group identification. In D. Abrams & M. A. Hogg (Eds.), ***Social identity and social cognition:*** 249–279. Oxford, U.K.: Blackwell.

Hughes, E. C. 1956. The making of a physician—General statement of ideas and problems. ***Human Organization,*** 14(4): 21–25.

This content downloaded from
120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

參考文獻

Abbott, A. 1992. 專業工作。收錄於 Y. Hasenfeld（編），《作為複雜組織的人類服務》：145–162。芝加哥：芝加哥大學出版社。Albert, S., & Whetten, D. A. 1985. 組織認同。收錄於 L. L. Cummings & B. M. Staw（編），《組織行為研究》第 7 卷：263–295。康乃狄克州格林威治：JAI 出版社。Alvesson, M. 1994. 組織中的對話：廣告公司的身分認同與印象管理。《組織研究》，15：535–563。Ashforth, B. E. 2001. 組織生活中的角色轉換：以身分認同為基礎的觀點。紐澤西州馬瓦：Erlbaum。Ashforth, B. E., & Mael, F. 1989. 社會認同理論與組織。《管理學會評論》，14：20–39。Ashforth, B. E., & Saks, A.M. 1996. 社會化策略：對新進人員適應的縱向影響。《管理學會期刊》，39：149–178。Baumeister, R. F. 1998. 自我。收錄於 G. Lindzey（編），《社會心理學手冊》（第 4 版）：680–740。紐約：McGraw Hill。Becker, H. S. 1970. 社會學工作：方法與實質。芝加哥：Aldine。Becker, H., Geer, B., Hughes, E., & Strauss, A. 1961. 白袍男孩：醫學院的學生文化。芝加哥：芝加哥大學出版社。Biddle, B. J. 1986. 角色理論的近期發展。收錄於 R. Turner & J. F. Short（編），《社會學年度評論》第 12 卷：67–92。加州帕洛阿爾托：Annual Reviews。Blackwell, B., Gutmann, M. C, & Jewell, K. E. 1984. 住院醫師訓練中的角色採納。《綜合醫院精神醫學》，6：280–288。Carr-Saunders, A. M., & Wilson, P.A. 1933. 專業職業。英國牛津：Clarendon Press。Conrad, P. 1988. 學習行醫：對近期醫學院歲月敘述的省思。《健康與社會行為期刊》，29：323–332。Cote, J. E., & Levine, C. G. 2002. 身分認同形成、能動性與文化：社會心理學綜論。紐澤西州馬瓦：Erlbaum。Deaux, K. 1991. 社會身分認同：對結構與變遷的思考。收錄於 R. C. Curtis（編），《關係自我：精神分析與社會心理學的理論交會》：77–93。紐約：Guilford。Dickie, V. A. 2003. 建立工作者身分認同：手工藝工作者研究。《美國職能治療期刊》，57：250–261。Dutton, J. E., Dukerich, J. M., & Harquail, C. V. 1994. 組織形象與成員認同。《行政科學季刊》，39：239–263。Eisenhardt, K. M. 1989. 從個案建立理論研究研究。《管理學院評論》，14：532-550。Epstein, S. 2002. 人格的認知－經驗自我理論。收錄於 M. J. Lerner（編），《心理學綜合手冊》，第 5 卷：人格與社會心理學：159-184。新澤西州霍博肯：Wiley。Erez, M., & Earley, C. 1993. 文化、自我認同與工作。紐約：Oxford University Press。Erikson, E. 1982. 《生命週期的完成：回顧》。紐約：Norton。Freidson, E. 1970. 《醫學專業：應用知識社會學研究》。紐約：Dodd, Mead。Freidson, E. 2001. 《專業主義：第三種邏輯》。芝加哥：University of Chicago Press。Gardner, H., Csikszentmihalyi, M., & Damon, W. 2001. 《優質工作：卓越與倫理相遇之時》。紐約：Basic Books。Gini, A. 2000. 《我的工作，我自己：工作與現代個體的創造》。紐約：Routledge。Glaser, B. G, & Strauss, A. 1967. 《紮根理論的發現：質性研究策略》。芝加哥：Aldine。Gouldner, A. 1957. 世界主義者與地方主義者：邁向潛在社會角色分析（一）。《行政科學季刊》，2：281-306。Greil, A. L., & Rudy, D. R. 1983. 歸信匿名戒酒會的世界觀：對歸信理論的修訂。《質性社會學》，6：5-28。Greil, A. L., & Rudy, D. R. 1984. 社會繭：組織中的封閉與認同轉變。《社會學探究》，54：260-278。Hall, D. 1968. 從學生到教授的過渡期間的認同變化。《學校評論》，76：445-469。Hall, D. 1971. 組織情境中職涯次級認同發展的理論模型。《組織行為與人類績效》，650 76：。Hall, D. 1995. 未預期的主管職涯轉換與次級認同的交替。《人力資源管理》，34：71-92。Hogg, M. A., & Abrams, D. 1993. 邁向群體社會動機的單一歷程不確定性降低模型。收錄於 M. A. Hogg & D. Abrams（編），《動機：社會心理學觀點》：173-190。紐約：Harvester-Wheatsheaf。Hogg, M. A., & Mullin, B. A. 1999. 加入群體以降低不確定性：主觀不確定性的降低與群體認同。收錄於 D. Abrams & M. A. Hogg（編），《社會認同與社會認知》：249-279。英國牛津：Blackwell。Hughes, E. C. 1956. 醫師的養成：觀念與問題概述。《人類組織》，14(4)：21-25。

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從 120.126.194.245 下載。所有使用均須遵守 https://about.jstor.org/terms

Ibarra, H. 1999. Provisional selves: Experimenting with image and identity in professional adaptation. ***Administrative Science Quarterly,*** 44: 764–791.

James, W. 1890. ***Principles of psychology*** (vols. 1 & 2). New York: Holt.

Kahn, W. A. 1990. Psychological conditions of personal engagement and disengagement at work. ***Academy of Management Journal,*** 33: 692–724.

Kanter, R. M. 1972. ***Commitment and community: Communes and utopias in sociological perspective.*** Cambridge, MA: Harvard University Press.

Kaufmann, J., & Pratt, M. G. 2005. Re-examining the link between organizational image and member attraction: On the positive uses of negative organizational stories in recruiting medical residents. In K. Elsbach (Ed.), ***Qualitative organizational research:*** 75–112. Greenwich, CT: Information Age.

Knight, J. A. 1973. ***Medical student: Doctor in the making.*** New York: Appleton-Century-Crofts.

Kristof, A. 1996. Person-organization fit: An integrative review of its conceptualizations, measurement, and implications. ***Personnel Psychology,*** 49: 1–49.

Larson, M. S. 1977. ***The rise of professionalism: A sociological analysis***. Berkeley: University of California Press.

Lee, T. W., Mitchell, T. R., & Sablynski, C. 1999. Qualitative research in organizational and vocational psychology, 1970–1999. ***Journal of Vocational Behavior,*** 55: 161–187.

Lewin, K. 1958. Group decision and social change. In T. M. Newcomb & E. L. Hartley (Eds.), ***Readings in social psychology:*** 201–216. New York: Harper & Row.

Locke, K. 2001. ***Grounded theory in management research.*** Thousand Oaks, CA: Sage.

Lofland, J., & Stark, R. 1965. Becoming a world-saver: A theory of conversion to a deviant perspective. ***American Sociological Review,*** 30: 862–874.

Louis, M. L. 1980. Surprise and sense making: What newcomers experience in entering unfamiliar organizational settings. ***Administrative Science Quarterly,*** 25: 226–251.

Macdonald, K. M. 1995. ***The sociology of the professions.*** London: Sage.

Margolis, E., & Romero, M. 1998. "The department is very male, very white, very old, and very conservative": The functioning of the hidden curriculum in graduate sociology departments. ***Harvard Educational Review,*** 68(1): 1–32.

Marion, R. 1989. ***Intern blues.*** NewYork: Fawcett Crest.

Marion, R. 1991. ***Learning to play God.*** Reading, MA: Addison-Wesley.

McCall, T. B. 1988. The impact of long working hours on resident physicians. ***New England Journal of Medicine,*** 318: 775–778.

Miles, M. B., & Huberman, A. M. 1994. ***Qualitative data analysis.*** Beverly Hills, CA: Sage.

Nicholson, N. 1984. A theory of work role transitions. ***Administrative Science Quarterly,*** 29: 172–191.

Pettigrew, A. 1990. Longitudinal field research on change: Theory and practice. ***Organization Science,*** 1: 267–292.

Polzer, J. T., Milton, L. P., & Swann, W. B. J. 2002. Capitalizing on diversity: Interpersonal congruence in small work groups. ***Administrative Science Quarterly,*** 47: 296–324.

Pratt, M. G. 1998. To be or not to be: Central questions in organizational identification. In P. Godfrey (Ed.), ***Identity in organizations: Developing theory through conversations:*** 171–207. Thousand Oaks, CA: Sage.

Pratt, M. G. 2000. The good, the bad, and the ambivalent: Managing identification among Amway distributors. ***Administrative Science Quarterly,*** 45: 456–493.

Pratt, M. G., & Ashforth, B. E. 2003. Fostering meaningfulness in working and at work. In R. Quinn (Ed.), ***Positive organizational scholarship:*** 309–327. San Francisco: Berrett-Koehler.

Pratt, M. G., & Rafaeli, A. 1997. Organizational dress as a symbol of multilayered social identities. ***Academy of Management Journal,*** 40: 862–898.

Schein, E. 1962. Man against man: Brainwashing. ***Corrective Psychiatry & Journal of Social Therapy,*** 8(2): 90–97.

Schein, E. 1987. ***Process consultation, volume 2: Lessons for managers.*** Reading, MA: Addison-Wesley.

Shulman, R., Wilkerson, L., & Goldman, D. A. 1992. Multiple realities: Teaching rounds in an inpatient pediatric service. ***American Journal of Diseases of Children,*** 146: 55–60.

Snow, D. A., & Anderson, L. 1987. Identity work among the homeless: The verbal construction and avowal of personal identities. ***American Journal of Sociology,*** 92: 1336–1371.

Snow, D. A., & Machalek, R. 1984. The sociology of conversion. In R. H. Turner & J. F. Short (Eds.), ***Annual review of sociology,*** vol. 10: 167–190. Palo Alto, CA: Annual Reviews.

Snow, D. A., & Phillips, C. L. 1980. The Lofland-Stark conversion model: A critical reassessment. ***Social Problems,*** 27: 430–447.

Spradley, J. 1979. ***The ethnographic interview.*** New York: Holt, Rinehart & Winston.

Stake, R. 1995. ***The art of case study research.*** Thousand Oaks, CA: Sage.

Strauss, A., & Corbin, J. 1990. ***Basics of qualitative re-***

This content downloaded from
120.126.194.245 on at, 31 ct 2026 06:54:44 UTC ▫
All use subject to https://about.jstor.org/terms

Ibarra, H. 1999. 暫時自我：在專業適應中試驗形象與認同。《行政科學季刊》，44：764-791。James, W. 1890. 《心理學原理》（第 1、2 卷）。紐約：Holt。Kahn, W. A. 1990. 工作中個人投入與抽離的心理條件。《管理學院期刊》，33：692-724。Kanter, R. M. 1972. 《承諾與社群：社會學觀點下的公社與烏托邦》。麻薩諸塞州劍橋：Harvard University Press。Kaufmann, J., & Pratt, M. G. 2005. 重新檢視組織形象與成員吸引力之間的關聯：招募住院醫師時，負面組織故事的正面用途。收錄於 K. Elsbach（編），《質性組織研究》：75-112。康乃狄克州格林尼治：Information Age。Knight, J. A. 1973. 《醫學生：未來的醫師》。紐約：Appleton-Century-Crofts。Kristof, A. 1996. 人格－組織契合：概念化、衡量方式及其意涵的整合性回顧。《人事心理學》，49：1-49。Larson, M. S. 1977. 《專業主義的興起：社會學分析》。柏克萊：University of California Press。Lee, T. W., Mitchell, T. R., & Sablynski, C. 1999. 1970 至 1999 年組織與職業心理學的質性研究。《職業行為期刊》，55：161-187。Lewin, K. 1958. 群體決策與社會變遷。收錄於 T. M. Newcomb & E. L. Hartley（編），《社會心理學讀本》：201-216。紐約：Harper & Row。Locke, K. 2001. 《管理研究中的紮根理論》。加州千橡市：Sage。Lofland, J., & Stark, R. 1965. 成為拯救世界的人：偏差觀點歸信理論。《美國社會學評論》，30：862-874。Louis, M. L. 1980. 驚訝與意義建構：新進人員進入陌生組織情境時的經驗。《行政科學季刊》，25：226-251。Macdonald, K. M. 1995. 《專業的社會學》。倫敦：Sage。Margolis, E., & Romero, M. 1998. 「這個系非常男性、非常白人、非常年長，而且非常保守」：社會學研究所中隱性課程的運作。《哈佛教育評論》，68(1)：1-32。Marion, R. 1989. 《實習醫師的憂鬱》。紐約：Fawcett Crest。Marion, R. 1991. 《學習扮演上帝》。麻薩諸塞州雷丁：Addison-Wesley。McCall, T. B. 1988. 長工時的影響住院醫師。New England Journal of Medicine, 318: 775-778。Miles, M. B., & Huberman, A. M. 1994。質性資料分析。Beverly Hills, CA: Sage。Nicholson, N. 1984。工作角色轉換理論。Administrative Science Quarterly, 29: 172-191。Pettigrew, A. 1990。變革的縱向田野研究：理論與實務。Organization Science, 1: 267-292。Polzer, J. T., Milton, L. P., & Swann, W. B. J. 2002。善用多樣性：小型工作團體中的人際一致性。Administrative Science Quarterly, 47: 296-324。Pratt, M. G. 1998。成為或不成為：組織認同的核心問題。收錄於 P. Godfrey（編），《組織中的認同：透過對話發展理論》：171-207。Thousand Oaks, CA: Sage。Pratt, M. G. 2000。好的、壞的與矛盾的：管理安利經銷商的認同。Administrative Science Quarterly, 45: 456-493。Pratt, M. G, & Ashforth, B. E. 2003。在工作中及工作本身培養意義感。收錄於 R. Quinn（編），《正向組織學術》：309-327。San Francisco: Berrett-Koehler。Pratt, M. G, & Rafaeli, A. 1997。組織服飾作為多層次社會認同的象徵。Academy of Management Journal, 40: 862-898。Schein, E. 1962。人與人對抗：洗腦。Corrective Psychiatry & Journal of Social Therapy, 8(2): 90-97。Schein, E. 1987。《過程諮詢》第二卷：給管理者的課程。Reading, MA: Addison-Wesley。Shulman, R., Wilkerson, L., & Goldman, D. A. 1992。多重現實：住院兒科服務中的教學查房。American Journal of Diseases of Children, 146: 55-60。Snow, D. A., & Anderson, L. 1987。無家者之間的認同工作：個人認同的言語建構與宣稱。American Journal of Sociology, 92: 1336-1371。Snow, D. A., & Machalek, R. 1984。皈依社會學。收錄於 R. H. Turner & J. F. Short（編），《社會學年度評論》第10卷：167-190。Palo Alto, CA: Annual Reviews。Snow, D. A., & Phillips, C. L. 1980。Lofland-Stark 皈依模型：批判性再評估。Social Problems, 27: 430-447。Spradley, J. 1979。《民族誌訪談》。New York: Holt, Rinehart & Winston。Stake, R. 1995。《個案研究的藝術》。Thousand Oaks, CA: Sage。Strauss, A., & Corbin, J. 1990。《質性研究基礎

此內容於 2026 年 10 月 31 日 06:54:44 UTC 從120.126.194.245 下載 ▫所有使用均須遵守 https://about.jstor.org/terms

search: Grounded theory procedures and techniques. Newbury Park, CA: Sage.

Strauss, A., & Corbin, J. 1998. ***Basics of qualitative research: Techniques and procedures for developing grounded theory*** (2nd ed.). Newbury Park, CA: Sage.

Sveningsson, S., & Alvesson, M. 2003. Managing managerial identities: Organizational fragmentation, discourse and identity struggle. ***Human Relations,*** 56: 1163–1193.

Turow, S. 1977. ***1 L: The turbulent true story of a first year at Harvard Law School.*** New York: Farrar Straus & Giroux.

Van Maanen, J. 1997. ***Identity work: Notes on the personal identity of police officers.*** Working paper, Massachusetts Institute of Technology, Cambridge.

Van Maanen, J., & Barley, S. R. 1984. Occupational communities: Culture and control in organizations. In B. M. Staw & L. L. Cummings (Eds.), ***Research in organizational behavior,*** vol. 6: 287–365. Greenwich, CT: JAI Press.

Van Maanen, J., & Schein, E. 1979. Towards a theory of organizational socialization. In B. M. Staw (Ed.), ***Research in organizational behavior,*** vol. 1: 209–264. Greenwich, CT: JAI Press.

Wallace, J. E. 1995. Organizational and professional commitment in professional and nonprofessional organizations. ***Administrative Science Quarterly,*** 40: 228–255.

Weick, K. E. 1995. ***Sensemaking in organizations.*** Thousand Oaks, CA: Sage.

Weinholtz, D. 1991. The socialization of physicians during attending rounds: A study of team learning among medical students. ***Qualitative Health Research,*** 1(2): 152–177.

West, M., Nicholson, N., & Arnold, J. 1987. Identity changes as outcomes of work-role transitions. In T. Honess & K. Yardley (Eds.), ***Self and identity: Perspectives across the lifespan:*** 287–303. New York: Routledge & Kegan Paul.

Wrzesniewski, A., & Dutton, J. E. 2001. Crafting a job: Revisioning employees as active crafters of their work. ***Academy of Management Review,*** 26: 179–201.

Wyatt, J. F. 1978. Sociological perspectives on socialization into a profession: A study of student nurses and their definition of learning. ***British Journal of Educational Studies,*** 26: 263–276.

Yin, R. K. 2003. ***Case study research: Design and methods*** (3rd ed.). Thousand Oaks, CA: Sage.

Michael G. Pratt *(mpratt@uiuc.edu)* earned his Ph.D. at the University of Michigan; he is a James F. Towey Fellow and an associate professor of organizational behavior at the University of Illinois at Urbana-Champaign. His current research examines issues of organizational attachment (e.g., identification and commitment), identity and meaning, and intuition among professionals in both traditional and dispersed work arrangements.

Kevin W. Rockmann *(krockman@gmu.edu)* is an assistant professor of management at George Mason University. He earned his Ph.D. in organizational behavior from the University of Illinois at Urbana-Champaign. His research centers on various types of distributed work, identity and identification, socialization, and faultlines in teams.

Jeffrey B. Kaufmann *(jkaufmnn@iastate.edu)* is an assistant professor in management at Iowa State University. Jeffrey earned his Ph.D. at the University of North Carolina. His research centers on issues of identity and socialization in professional organizations and the intersection between law and management.

This content downloaded from
120.126.194.245 on at, 31 ct 2026 06:54:44 UTC
All use subject to https://about.jstor.org/terms

：紮根理論程序與技術》。Newbury Park, CA: Sage。Strauss, A., & Corbin, J. 1998。《質性研究基礎：發展紮根理論的技術與程序》（第2版）。Newbury Park, CA: Sage。Sveningsson, S., & Alvesson, M. 2003。管理管理者認同：組織碎片化、認同鬥爭。Human Relations, 56: 1163-1193。Turow, S. 1977。《1L：哈佛法學院一年級的動盪真實故事》。New York: Farrar Straus & Giroux。Van Maanen, J. 1997。《身份工作：警察個人認同札記。工作論文，麻省理工學院，劍橋。Van Maanen, J., & Barley, S. R. 1984。職業社群：組織中的文化與控制。收錄於 B. M. Staw & L. L. Cummings（編），《組織行為研究》第6卷：287-365。Greenwich, CT: JAI Press。Van Maanen, J., & Schein, E. 1979。邁向組織社會化理論。收錄於 B. M. Staw（編），《組織行為研究》第1卷：209-264。Greenwich, CT: JAI Press。Wallace, J. E. 1995。專業與非專業組織中的組織承諾與專業承諾。Administrative Science Quarterly, 40: 228-255。Weick, K. E. 1995。《組織中的意義建構》。Thousand Oaks, CA: Sage。Weinholtz, D. 1991。主治醫師查房期間的醫師社會化：醫學生團隊學習研究。Qualitative Health Research, 1(2): 152-177。West, M., Nicholson, N., & Arnold, J. 1987。工作角色轉換的結果：認同變化。收錄於 T. Honess & K. Yardley（編），《自我與認同》：生命歷程中的觀點：287-303。紐約：Routledge & Kegan Paul。Wrzesniewski, A., & Dutton, J. E. 2001。打造工作：重新構想員工如何成為積極塑造自身工作的創作者。組織研究院，26: 179-201。Wyatt, J. F. 1978。職業社會化的社會學觀點：探討護理系學生及其對學習的界定。《英國教育研究期刊》，26：263-276。Yin, R. K. 2003。《個案研究研究：設計與方法》（第3版）。加州千橡市：Sage。

Michael G. Pratt（mpratt@uiuc.edu）是伊利諾大學厄巴納－香檳分校的組織行為學教授。他在密西根大學取得博士學位，主修組織依附相關議題。他目前的研究探討組織成員與承諾、意義建構以及工作中的身分與認同。

Kevin W. Rockmann（krockman@gmu.edu）是喬治梅森大學管理學系的教授，任職於管理學系，喬治梅森大學。他取得博士學位，主修組織行為學。他的研究聚焦於各種分散式工作的類型、身分與認同、以及團隊中的斷層。

Jeffrey B. Kaufmann（jkaufmnn@iastate.edu）是愛荷華州立大學管理學系的助理教授。Jeffrey 於北卡羅來納大學取得博士學位。他的研究聚焦於專業組織中的身分與社會化議題，以及法律與管理之間的交集。

此內容下載自120。126.194.245，時間為 2026 年 31 ct 06:54:44 UTC。所有使用均受 https://about.jstor.org/terms 條款約束